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Title Page

A PHASE 3, RANDOMIZED, OPEN-LABEL STUDY OF PF-06821497 (MEVROMETOSTAT) IN COMBINATION WITH ENZALUTAMIDE COMPARED WITH ENZALUTAMIDE OR DOCETAXEL IN PARTICIPANTS WITH METASTATIC CASTRATION RESISTANT PROSTATE CANCER PREVIOUSLY TREATED WITH ABIRATERONE ACETATE (MEVPRO-1)

Study Intervention Number:	PF-06821497
Study Intervention Name:	Mevrometostat
US IND Number:	134,135
EU CT Number:	2024-511650-50-00
ClinicalTrials.gov ID:	NCT06551324
Pediatric Investigational Plan Number	NA
Protocol Number:	C2321014
Phase:	3
Sponsor Legal Address:	Pfizer Inc. 66 Hudson Boulevard East New York, NY 10001
Brief Title: A Study to Learn About the Investigational Medicine Called PF-06821497 (mevrometostat) in Men with mCRPC Who Were Previously Treated with Abiraterone Acetate for Prostate Cancer (MEVPRO-1).	

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Document History

Document	**Version Date**
Original Protocol	03 May 2024
Amendment #1	11 June 2024
Amendment #2	13 November 2024
Amendment #3	29 January 2025
Amendment #4	27 January 2026

This amendment incorporates all revisions to date, including amendments made at the request of country health authorities and IRBs/ECs and any protocol administrative change letter(s).

Protocol Amendment Summary of Changes Table

Amendment **4 (27 January 2026**)

Overall Rationale for the Amendment

The main purpose of Protocol Amendment 4 is to incorporate on-treatment ECG assessments, and to update the statistical plan for the overall survival Interim Analysis 2.

Description of Change	**Brief Rationale**	**Section # and Name**
	Substantial Modification(s)	
Added postdose ECG assessments at C1 D1 and C2 D1	Added per FDA Request in order to address the uncertainty in the potential for QTc prolongation.	Section 1.3 Schedule of Activities, Tables 1 and 2
Changed Interim Analysis 2 for OS from time dependent to event based	Interim Analysis 2 for OS will be event-driven rather than Calendar based, in accordance with recent FDA guidance "Approaches to Assessment of Overall Survival in Oncology Clinical Trials – *Guidance for Industry*".	Section 9.4 Interim Analyses
Removed Sun exposure risk and Photosensitivity guidance	Based on supportive preclinical data in the mevrometostat IB. Per IB v10.0, there is no potential risk for phototoxicity.	Section 2.3.1 Risk Assessment, Section 5.3 Lifestyle Considerations
Added 5-alpha reductase inhibitors as an exclusion	Updated for consistency with Section 6.9.1 and clarified	Section 5.2 Exclusion Criteria

Description of Change	Brief Rationale	Section # and Name
and clarified the exceptions to the list of prohibited prior medications in EC 5a and 5c.	exceptions to the list of prohibited medications as they remain in use in certain countries.	
Clarified timeframes related to suicidal ideation and behavior in EC 1	To provide clarity on exclusion criteria	Section 5.2 Exclusion Criteria
Clarified that a left anterior hemiblock and a bifascicular block are both an exclusion for exclusion criteria #2.	To provide clarity on exclusion criteria	Section 5.2 Exclusion Criteria
Updated guidance for potential cases of AKI	To clarify guidance for investigators and to align with the current Pfizer protocol template	Section 7.1.4 Potential Cases of Acute Kidney Injury Section 10.7 Appendix 7: Kidney Safety: Monitoring Guidelines
Added Long Term Follow-up (LTFU) Visit	Added LTFU to begin after Safety Follow-Up	Section 1.3 Schedule of Assessments, Table 1
Non-Substantial Modification(s)		
Clarified primary estimand censoring rules for rPFS	Clarifies these requirements.	Section 9.1.1.1 Primary Estimand
Updated reference date for start of Safety Follow-Up period from “permanent treatment discontinuation” to “last dose of study intervention”	Updated for consistency throughout the protocol and with SAP.	Section 1.1 Synopsis; Section 4.1 Overall Design
Added +/- 7 day window for End of Treatment (EOT) visit for participants receiving mevrometostat + enzalutamide or enzalutamide alone	To allow for flexibility in performing assessments at EOT visit.	Section 1.3 Schedule of Activities, Table 1 and Table 2; Section 7.1 Discontinuation of study intervention
Added the EOT visit for participants completing 10 cycles of docetaxel, the EOT visit is scheduled 21 days (+/-7 days) after C10 D1	To allow for flexibility in performing assessments at EOT visit.	Section 1.3 Schedule of Activities, Table 2

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Description of Change	Brief Rationale	Section # and Name
Added information on the timing of BPI-SF Question 3 and updated the window for PRO-CTCAE and FACT-GP5 from +/-1 to +/-3 days	Clarifies these requirements.	Section 1.3 Schedule of Activities, Table 2
Updated the timeline for scheduled PRO-CTCAE and FACT-GP5 assessments to the first 5 weeks (Wk 6).	Clarifies these requirements	Section 1.3 Schedule of Activities, Table 1
Updated text for Secondary malignancies	Clarifies these requirements.	Section 2.3.1 Risk Assessment PF-06821497
Updated text for assessments conducted at different local laboratory	Clarifies these requirements.	Section 2.3.1 Risk Assessment Study Procedures
Updated text related to participants being dosed at the site by the investigator or designee.	Clarifies these requirements.	Section 6.5 Study Intervention Compliance
Updated number of OS events for final OS analysis	Align with the change from calendar-based to event-driven for OS Interim Analysis 2.	Section 9.4 Interim Analyses, Section 9.5 Sample Size Determination
Updated estimate for number of events at the time of OS Interim Analysis 1	Align with the timing of the primary rPFS analysis based on updated accrual rates.	Section 9.4 Interim Analyses
Clarified that dabigatran and edoxaban are allowable while on treatment, and that any concurrent hormonal treatments are prohibited while on treatment	Clarifies these requirements.	Section 6.9.1 Prohibited During the Study
Added language about starting loperamide to treat Grade 2 and 3 diarrhea	Clarifies these requirements.	Section 6.6.5 Dose Reductions for PF-06821497, Table 6

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Description of Change	Brief Rationale	Section # and Name
Updated language about the imaging modalities not considered evaluable in IC 3	Clarifies these requirements.	Section 5.1 Inclusion Criteria
Updated language pertaining to administration of study intervention with food	Clarify these requirements.	Section 5.3.2 Meals and Dietary Restrictions, Section 6.1.1.1 PF-06821497 or Placebo
Added language about recording PSMA PET scans on CRFs	To capture the data when relevant scan is done although it is not an evaluable imaging modality for this study.	Section 8.2.2 Assessment of Primary Endpoint: rPFS
Added updated language to the ECG procedures section	Clarifies the requirements.	Section 8.3.2 (12-Lead) Electrocardiograms
Added text about absence of symptoms and signs indicating clinically significant progression of disease and about exposure of participants to unreasonable risks	Ensure participants are not exposed to unreasonable risks.	Sections 7.1 Discontinuation of Study Intervention,
Added language when the EOT visit coincides with a regular study visit.	Clarifies the requirements.	Section 7.1 Discontinuation of Study Intervention
Clarified meaning of “new anticancer” language by replacing with “new antineoplastic or investigational therapy”	Clarifies these requirements.	Section 7.1.1 28 Day Safety Follow-Up
Updated the start of LTFU data collection from “last dose of study intervention” to “Safety Follow-up”	Clarified when data collection should begin.	Section 1.1 Synopsis; Section 4.1 Overall Design; Section 7.1.2 Long Term Follow-Up; Section 7.1.3 Survival Follow-Up
Moved survival data collection references from Long Term Follow-up section to new section for Survival Follow-up	Clarifies these requirements.	Section 7.1.2 Section 7.1.3 Survival Follow-up

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Description of Change	Brief Rationale	Section # and Name
Added information about telehealth and unscheduled visits	Clarifies these requirements.	Section 8.1.1 Telehealth Visits
Removed references to PSSA throughout and updated with protocol mandatory text	To align with the current Pfizer protocol template	Section 8.4.1 Time Period and Frequency for Collecting AE and SAE Information, 8.4.5 Environmental Exposure, Exposure During Pregnancy or Breastfeeding, and Occupational Exposure, 8.4.8 Protocol Specified Adverse Events of Special Interest, 8.4.10 Medication Errors, 10.3.3 Recording/Reporting and Follow-up of AEs and/or SAEs During the Active Collection Period
Deleted the Lack of Efficacy section	To align with the current Pfizer protocol template.	Formerly Section 8.4.8.1 Lack of Efficacy
Clarified that investigator or qualified designee and not a local healthcare professional, is responsible for obtaining consent	Clarifies this requirement.	Section 8.1 Administrative and Baseline Procedures, Section 10.1.3 Informed Consent Process
Added language about outcome assessment for reporting/recording AEs and SAEs	Clarifies the instructions and aligns with the current Pfizer protocol template.	Section 10.3.3 Recording/Reporting and Follow-Up of AEs and/or SAEs During the Active Collection Period
Specified that imaging assessments should utilize the date of randomization as the reference date for all time points, and clarified the conditions for acceptance of Screening/baseline imaging assessments	Ensures visibility of these requirements within the SoA.	Section 1.3 Schedule of Activities, Tables 1 and 2

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Description of Change	Brief Rationale	Section # and Name
Updated Risk Assessment section for enzalutamide to add PRES and Dysphagia risks for enzalutamide and updated clinical data for other enzalutamide risks	Updated for alignment with the most recent product labeling.	Section 2.3.1 Risk Assessment - Enzalutamide
Included notation that hepatic toxicity is an important potential risk in Risk Assessment Section for PF-06821497.	Updated to note that this is an important risk and to align with the mitigation strategy with the mevrometostat IB	Section 2.3.1 Risk Assessment – PF-06821497
Added text related to AE due to PF-06821497.	Added reference to guidance if the relationship of AEs due to PF-06821497 cannot be excluded.	Section 6.6.1 PF-06821497 Dose Modifications
Clarification of participant reproductive inclusion criteria	Updated to align with protocol template	Section 10.4.1 Male Participant Reproductive Inclusion Criteria
Clarification of effective methods of contraception updated	Updated to align with protocol template	Section 10.4.2 Contraceptive Methods
Changes added to the publication policy section per updated template language	Updated to align with current protocol template	Section 10.1.11 Publication Policy
Clarification added to Liver Safety section	Updated to align with current protocol template	Section 10.6 Appendix 6 Liver Safety
Updated the AE Grading table for decline in kidney function	Corrected the table to align with CTCAE V5.0	Section 10.7.4 Adverse Event Grading for Kidney Safety Laboratory Abnormalities
General editorial changes	To correct grammatical errors, to maintain consistency, and/or to increase clarity.	Throughout protocol

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1. PROTOCOL SUMMARY

1.1. Synopsis

Protocol Title:

A Phase 3, Randomized, Open-Label Study of PF-06821497 (mevrometostat) in Combination with Enzalutamide Compared with Enzalutamide or Docetaxel in Participants with Metastatic Castration Resistant Prostate Cancer Previously Treated with Abiraterone Acetate (MEVPRO-1)

Brief Title:

A Study to Learn About the Investigational Medicine Called PF-06821497 (mevrometostat) in Men with mCRPC Who Were Previously Treated with Abiraterone Acetate for Prostate Cancer. (MEVPRO-1)

Regulatory Agency Identification Number(s):

US IND Number:	134,135
EU CT Number:	2024-511650-50-00
ClinicalTrials.gov ID:	NCT06551324
Pediatric Investigational Plan Number:	Number is not available
Protocol Number:	C2321014
Phase:	3

Rationale:

The rationale of the study is to evaluate whether the addition of EZH2 (enhancer of zeste homologue-2) inhibition to enzalutamide can reverse anti-androgen resistance, thereby increasing the duration of clinical benefit of enzalutamide in participants who have previously been treated with abiraterone. This Phase 3 randomized study (C2321014) is designed to demonstrate that PF-06821497 plus enzalutamide provides superior clinical benefit compared to physician’s choice of treatment (enzalutamide or docetaxel) in participants with mCRPC (metastatic castration resistant prostate cancer) who have previously been treated with abiraterone acetate.

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Objectives, and Endpoints:

Objectives	Endpoints
Primary:	**Primary:**
• To demonstrate that PF-06821497 in combination with enzalutamide is superior to PCT (physician's choice of treatment) of enzalutamide or docetaxel in prolonging rPFS (radiographic progression free survival).	• BICR (blinded independent central review) assessed rPFS per RECIST (Response Evaluation Criteria in Solid Tumors) v1.1 (soft tissue disease) and PCWG3 (Prostate Cancer Clinical Trials Working Group 3) (bone disease).
Key Secondary:	**Key Secondary:**
• To demonstrate that PF-06821497 in combination with enzalutamide is superior to PCT (enzalutamide or docetaxel) in prolonging OS (overall survival).	• OS (alpha protected).
Other Secondary:	**Other Secondary:**
To evaluate anti-tumor activity	• Proportion of participants with measurable soft tissue disease at baseline with an objective response per RECIST v1.1 (assessed by BICR).
	• Duration of response (DoR) in soft tissue disease per RECIST v1.1 (assessed by BICR).
	• Proportion of participants with PSA (prostate specific antigen) response ≥50% in participants with detectable PSA values at baseline.
	• Time to PSA progression.
	• Time to initiation of new antineoplastic therapy.
	• Time to first symptomatic skeletal event.
	• PFS2 (progression free survival on next line of therapy) based on investigator assessment.
• To compare safety and tolerability between the treatment arm and the control arm	• Type, incidence, severity (as graded by NCI CTCAE [National Cancer Institute Common Terminology Criteria for Adverse Events] v5.0), seriousness and relationship to study medications of AEs (adverse events).

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Objectives	Endpoints
• To compare patient reported outcomes (PROs) between the treatment arm and the control arm	• Change from baseline in patient reported pain symptoms per Brief Pain Inventory – Short Form (BPI-SF); • Change from baseline in health-related quality of life (HRQoL), functioning and symptoms per Functional Assessment of Cancer Therapy – Prostate (FACT-P); • Change from baseline in patient reported health status per European Quality of Life 5-Dimension 5 Level (EQ-5D-5L); • Symptomatic toxicity and the overall side effect burden as measured by items from the Patient-Reported Outcome CTCAE (PRO-CTCAE) and FACT- GP5; • Time to confirmatory deterioration in patient-reported pain symptoms per BPI-SF Item 3 "worst pain in 24 hours" • Time to definitive deterioration in patient-reported HRQoL and physical well-being per FACT-P.
• To evaluate the PK (pharmacokinetics) of PF-06821497 when dosed with enzalutamide	• PK characterized by pre-dose trough and post-dose plasma concentrations of PF-06821497 at selected visits.
• To assess the relationship between ctDNA (circulating tumor DNA) burden and outcome.	• ctDNA burden at baseline and on study

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Overall Design

This is a global, multicenter, randomized, open-label Phase 3 study evaluating PF-06821497 plus enzalutamide versus physician's choice of either enzalutamide or docetaxel for participants with mCRPC after progression on prior abiraterone acetate. Participants may have received prior chemotherapy for mCSPC but are required to be chemotherapy naïve in the castration-resistant setting. Participants who received prior treatment with 2nd generation ARSi's (androgen receptor signaling inhibition) in any setting (eg, enzalutamide, apalutamide or darolutamide) will be excluded. This study consists of a Screening Phase, Randomization, Treatment Phase, Safety Follow-up, and Long-Term Follow-up. Participants will be randomized on a 1:1 basis to receive (Arm A) PF-06821497 875 mg BID (twice daily) in combination with enzalutamide, 160 mg QD (once daily) or (Arm B) physician's choice of comparator therapy of either enzalutamide, 160 mg QD, or docetaxel (75 mg/m^2 administered IV (intravenous) every 21 days, for up to a maximum of 10 cycles).

The physician's choice of agent in the control arm must be prespecified prior to randomization. Docetaxel is given in combination with prednisone, prednisolone, and dexamethasone (premedication). Randomized participants will receive protocol-assigned treatment until confirmation of radiographic progression by BICR, participant no longer benefits in the opinion of the investigator, unacceptable toxicity, initiation of a new antineoplastic, global deterioration of health status requiring discontinuation, significant protocol violation, lost to follow-up, participant refused further treatment, or death. Participants on the docetaxel physician's choice treatment arm will continue to undergo study assessments following completion of up to 10 cycles of study intervention and will be able to continue to receive ADT (androgen deprivation therapy) as per SOC (standard of care). An independent E-DMC (external data monitoring committee) will monitor the safety of the

participants on a periodic basis. Participants discontinuing the Treatment Phase will continue to the End of Treatment Visit and a Safety Follow-up (approximately 28 to 35 days after last dose of study intervention or before initiation of a new antineoplastic or investigational therapy).

After Safety Follow-up, participants enter the LTFU (Long-term Follow-up) period. Participants who discontinue treatment and do not have radiologic disease progression will continue radiographic imaging and PSA assessments until they progress radiographically as determined by BICR (regardless of the initiation of a new antineoplastic therapy) or death. For all participants in the LTFU period, survival status, initiation of new antineoplastic therapy, diagnosis of myelodysplastic syndrome (MDS), T-cell lymphoblastic lymphoma (T-LBL), and acute myeloid leukemia (AML) or second primary malignancy will be monitored and collected every 12 weeks from Safety Follow-up until the participant dies or withdraws consent for follow-up, or the study is terminated by the sponsor. The LTFU period will conclude at the time of the final OS analysis. Crossover will not be allowed in the trial.

Number of Participants

Approximately 600 participants will be enrolled in the study and randomized 1:1 to the 2 treatments arms (ie, 300 participants per arm).

Note: "Enrolled" means a participant's agreement to participate in a clinical study following completion of the informed consent process and randomization to study intervention.

Study Population

Eligible participants include adult men (age ≥18 years) with histologically or cytologically confirmed adenocarcinoma of the prostate, surgical or medical castration, and documented progression of metastatic disease following treatment with abiraterone acetate in the mCSPC setting or first line mCRPC setting. Participants must have an Eastern Cooperative Oncology Group (ECOG) performance status of 0 - 2 with no medical or psychiatric condition or laboratory abnormality that may increase the risk of study participation. Prior prostate cancer treatments with any cytotoxic chemotherapy, radioligand therapy, ARSi treatment, PARP (poly adenosine diphosphate-ribose polymerase) monotherapy, or other systemic anticancer treatments are exclusionary with some exceptions.

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Study Arms and Duration:

Study Arm(s)		
Arm Title	Arm A	Arm B
Arm Description	Participants will receive PF-06821497 875 mg BID (2 times a day) + enzalutamide 160 mg QD. Cycles are 28 days, and treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, lost to follow-up, or study termination.	Physician's choice: enzalutamide: Participants will receive enzalutamide 160 mg QD. Cycles are 28 days; and treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, lost to follow-up, or study termination. Physician's choice docetaxel: Participants will receive docetaxel at a dose of 75 mg/m^2 IV every 21 days, for up to a maximum of 10 cycles. Docetaxel is given in combination with prednisone or prednisolone, 5 mg BID for 21 days, and premedication of oral dexamethasone 8 mg at 12 hours, 3 hours, and 1 hour (or IV dexamethasone per standard of care) before the docetaxel infusion. Treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, or lost to follow-up or study termination.

Ethical Considerations:

The results of an ongoing Phase 1 study of PF-06821497, C2321001, either as monotherapy in participants with solid tumors or in combination with enzalutamide in mCRPC, support this investigation of PF-06821497 plus enzalutamide for participants with mCRPC after treatment with abiraterone acetate. Taking into account the measures to minimize risk to participants, the potential risks associated with PF-06821497 plus enzalutamide are justified by the anticipated benefits that may be afforded to patients with mCRPC following prior abiraterone acetate treatment. Therefore, there is a favorable benefit-risk profile to support the study rationale.

Participants may experience delayed progression of disease and clinical benefit and improved quality of life from the combination of PF-06821497 plus enzalutamide. Study participants may also benefit from more intense monitoring and more frequent assessments.

Based on the Phase 1 clinical data for participants with mCRPC, the most frequently reported all-causality adverse events (≥10%) occurring with the combination of PF-06821497 plus enzalutamide include diarrhea, decreased appetite, nausea, fatigue, dysgeusia, asthenic conditions, edema peripheral, vomiting, weight decreased, arthralgia, alopecia, hypokalemia, hypocalcemia, anemia, thrombocytopenia, and neutropenia. Study safety data will be monitored on a regular basis by the sponsor, as well as by an independent E-DMC (external data monitoring committee). Male participants will be notified to seek advice regarding sperm cryopreservation due to the possibility of reduced sperm counts during treatment.

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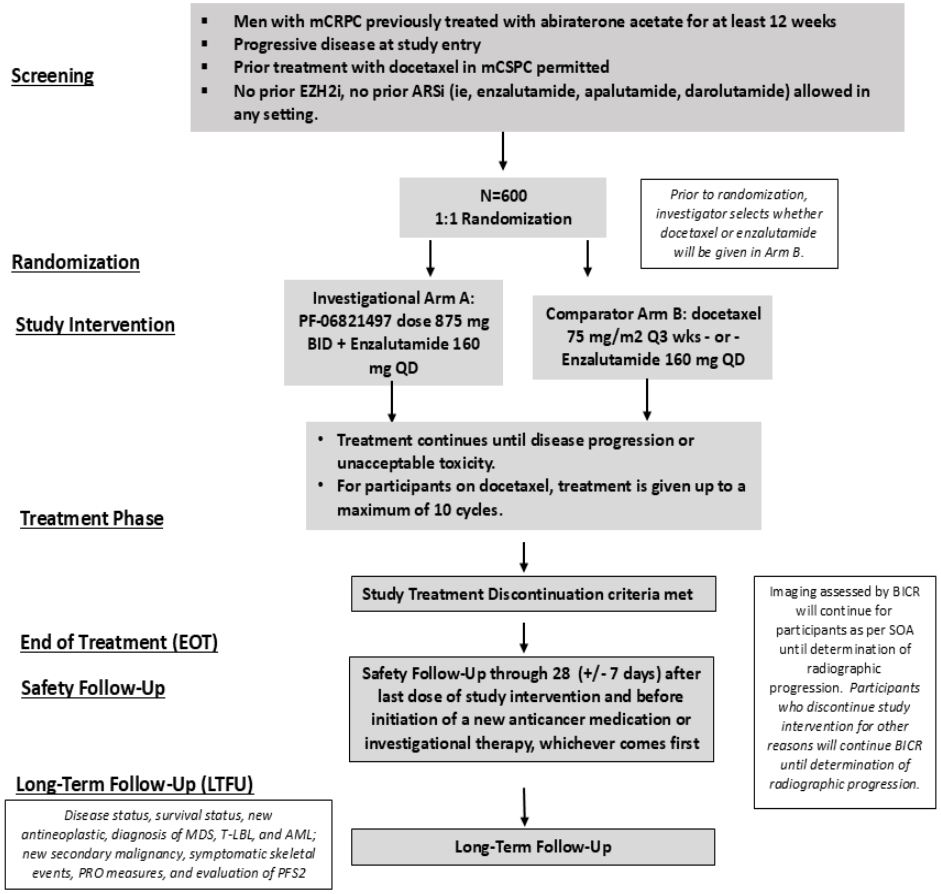
1.2. Schema



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1.3. Schedule of Activities

The SoA table provides an overview of the protocol visits and procedures. Refer to the STUDY ASSESSMENTS AND PROCEDURES section of the protocol for detailed information on each assessment required for compliance with the protocol.

The investigator may schedule visits (unplanned visits) in addition to those listed in the SoA table, in order to conduct evaluations or assessments required to protect the well-being of the participant.

For Screening and Randomization activities, refer to either Table 1 or Table 2; these activities are the same on both tables.

Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Informed Consent	X									See Section 10.1.3. Informed consent must be obtained prior to undergoing any study specific procedures. Ensure consent is on the current version of the form approved by the ethics committee and sponsor.
Screen for inclusion/exclusion criteria	X									See Section 5.1 and Section 5.2.
Randomization		X								After participant is deemed eligible for the study; the participant enrollment

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									Notes
	Screening	Treatment						Follow-up		
		W1	W3	W5	W7	W9	EOT	28 day Safety Follow-up	LTFU every 12 weeks	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
										number and Treatment Arm are assigned at randomization. Participants should start treatment within 3 days after randomization.
Investigator reason for treatment choice of docetaxel or enzalutamide		X								Select primary reason for investigator's choice of treatment prior to randomization.
Demographics	X									
Medical History	X									Relevant medical and surgical history and current illnesses. Includes histology, TNM classification, Gleason score,
Physical Exam	X	X	X	X	X	X	X	X		See Section 8.3.1. Physical examinations to be completed before administration of study intervention.
Vital signs, height, weight	X	X	X	X	X	X	X	X		See Section 8.3.1. Measure blood pressure, heart rate, temperature (ºC), height (cm) (screening only) and weight (kg).
ECOG Performance Status	X	X	X	X	X	X	X	X		See Section 10.11, Appendix 11.
12 Lead ECG	X	X		X			X	X		ECG at C1 D1 and C2 D1 should be collected at 2-hours (+2 hr window)

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
										post dose. When coinciding with blood sample draws, ECG assessments must be performed prior to blood sample collection, such that the blood sample is collected at the nominal time. Management of signs or symptoms of ischemic heart disease is to be per general clinical practice and local guidelines, including QTcF interval (machine reported or manually). See Section 8.3.2, Appendix 8.
Contraception check	X	X		X		X	X	X		See Section 5.3.1 and Section 10.4, Appendix 4.
Contraceptive counseling to seek advice about donation and cryopreservation of sperm cells prior to start of study	X									See Section 5.3.1
Prior/concomitant treatment(s)	X									See Section 6.9.
Screening Tumor Biopsy	X									Only required if no prior histological or cytological confirmation of prostrate adenocarcinoma is available.

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Serious/nonserious AE monitoring	X									Report serious and non-serious adverse event information from time informed consent form is signed through screen failure or through a minimum of 28 days after the last dose of study intervention. See Section 8.4**.**
Symptomatic Skeletal Events		X								See Section 8.3.4
Laboratory Assessments – Central/Local Lab										
Hematology, Blood chemistry, CD4 count* (pre dose)	X	X	X	X	X	X	X	X		Tests will not need to be repeated at C1 D1 if Screening tests were performed within 7 days of C1 D1. See Section 8.3.3 and Section 10.2, Appendix 2. Samples may be collected up to 3 days prior to the visit to guide decision making. *Consider obtaining CD4 count if lymphocyte count is <500/µL
Serum Testosterone	X									
Tumor markers - PSA (pre dose)	X	X		X		X	X	X	X	PSA collected during LTFU only if participant did not discontinue treatment due to BICR confirmed rPFS

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Radiographic Assessments	Imaging assessments are to be scheduled using the randomization date as the reference date for all time points. See Section 8.2.2. All radiographic assessments must be forwarded to the independent core imaging laboratory; the same method of assessment and the same technique should be used to characterize each identified and reported lesion at baseline and during follow-up.									
CT Scan of chest; CT or MRI of abdomen and pelvis	X -42 days accepted					X[a]	X[b]		X[c]	**Screening/baseline scans:** may use scans obtained as part of standard of care before consent was signed and within 42 days prior to Day 1 (Randomization) **if scans were performed per the specific study requirements (per imaging manual).** a Scans will be performed every 8 weeks (at Week 9, 17, 25) then every 12 weeks (at Week 37, 49, 61, etc.). Radiographic assessments continue for all participants until blinded independent central confirmation of radiographic progression. b Tumor assessments are to be performed during the EOT visit if the reason was other than central radiographically confirmed disease progression and it has been ≥ 8 weeks
Whole body bone scan: Tc-99m Bone Scintigraphy	X -42 days accepted					X[a]	X[b]		X[c]	

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									Notes
	Screening	Treatment						Follow-up		
		W1	W3	W5	W7	W9	EOT	28 day Safety Follow-up	LTFU every 12 weeks	
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
										since the last assessment. c Not required during follow-up if disease progression has been confirmed by BICR.
PRO Assessments										
BPI-SF, FACT-P, EQ-5D-5L	X	Every 6 weeks (starting Cycle 1 Day 1)					X	X	X	See Section 8.2.4. Participants will self-administer assessments via an electronic device. On C1 D1, complete questionnaires after randomization, but before the first dose of study intervention. At all subsequent scheduled PRO assessments, participant is to complete the questionnaires +/- 3 days of the scheduled assessment. In cases which the PRO assessment timepoint overlaps with a study visit, all PROs must be completed before other study activities.
PRO-CTCAE and FACT-GP5	X	Every week for the first 6 weeks (starting Cycle 1 Day 1) then every 6 weeks until Week 25					X			See Section 8.2.4. Participants will self-administer assessments via an electronic

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									Notes
	Screening	Treatment						Follow-up		
		W1	W3	W5	W7	W9	EOT	28 day Safety Follow-up	LTFU every 12 weeks	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				**Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
										device. On C1 D1, complete questionnaires after randomization, but before the first dose of study intervention. For the scheduled PRO-CTCAE and FACT-GP5 assessments during the first 5 weeks (Wk 6), participant is to complete the questionnaires +/- 1 days of the scheduled assessment. Then for all subsequent PRO-CTCAE and FACT-GP5 assessments starting at Week 7, the patient is to complete the questionnaires +/- 3 days of the scheduled assessment. In cases in which the PRO assessment timepoint overlaps with a study visit, all PROs must be completed before other study activities.
Biomarker Assessments										
Tumor tissue (archival or de novo)		X								See Section 8.8.1; Designation of available pretreatment tumor tissue for shipment to sponsor for retrospective molecular profiling analysis.

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									Notes
	Screening	Treatment						Follow-up		
		W1	W3	W5	W7	W9	EOT	28 day Safety Follow-up	LTFU every 12 weeks	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Blood sample for cf nucleic acids analyses (pre-dose)		X				X (Cycle 3 and Cycle 7 Day 1 only)	X			See Section 8.8.2, 8.8.4, and Laboratory Manual.
Blood sample for serum biomarkers analysis (pre-dose)		X				X (Cycle 3 and Cycle 7 Day 1 only)	X			See Section 8.8.4 and Laboratory Manual.
Blood sample for matched germline control (pre-dose)		X								See Section 8.8.3 and Laboratory Manual;
Retained research sample for biomarkers (Prep D1) (pre-dose)		X								See Section 8.7.2 If not collected at C1 D1, collect at the next available time point when biospecimens are being collected in conjunction with a participant visit.
Study Intervention										
Study intervention dispensation (PF-06821497 and/or enzalutamide)		X		X		X				See Section 6.2.1.
Study intervention accountability				X		X	X			See Section 6.2.

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Pharmacokinetics **(Arm A only) blood sampling**			X	X		X (last PK draw at Cycle 6 Day 1)				See Section 8.5. A blood sample for PK analysis must be collected predose (within 60 minutes prior to morning dose of mevrometostat) and 2 hours post dose (+2 hr window) on Weeks 3 (C1 D15), 5 (C2 D1), and 9 (C3 D1); and predose (within 60 minutes prior to morning dose of mevrometostat) on Weeks 13 (C4 D1), 17 (C5 D1), and 21 (C6 D1). The morning dose must be taken in clinic on PK days (not at home). Record the dose and time of treatment administration on both the day of, and the day before PK sampling. Food is to be consumed within 1 hour prior to dosing on PK days where post dose sample is collected (ie, Weeks 3, 5, and 9).
Long Term Follow-up (LTFU)	Long-term follow-up begins after Safety Follow-Up and may be conducted by telephone unless imaging or a PSA level is required.									

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Collection of new Antineoplastic/Investigational Therapy (may be completed via phone)		X								Record all subsequent antineoplastic treatments systemic or localized (ie radiotherapy, surgery) for participants starting a new antineoplastic or investigational therapy.
Diagnosis of Second Primary Malignancy	X	X								Record any second primary malignancy including MDS, T-LBL, and AML and report as an SAE. See Section 4.1.
Survival Status									X	See Section 7.1.2 Obtain survival status by telephone, email, clinic visit, chart review, or by communicating with an individual (eg, family, friend, or referring health care provider) who is knowledgeable of the participant's survival status. Publicly available information may be used to determine vital status only as appropriately directed in accordance with local law.

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Table 1. Schedule of Activities: Participants Receiving Enzalutamide - Randomized to PF-06821497 + enzalutamide (Arm A) or enzalutamide alone (Arm B)

	Visit Identifier									**Notes**
	Screening	**Treatment**						**Follow-up**		
		W1	**W3**	**W5**	**W7**	**W9**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Treatment Cycle defined as every 28 days for participants randomized to Arm A or assigned to physician's choice of enzalutamide in Arm B** **Treatment visits should be fixed according to C1 D1** **See Section 7.1.1 for 28 day Safety Follow-up, and Section 7.1.2 for Long Term Follow-up details**
Abbreviations used in this table are found in Appendix 13	**Screening**	**C1 D1**	**C1 D15**	**C2 D1**	**C2 D15**	**C3 D1 and every 4 weeks thereafter**				
Visit Window	**(-28 days to -1 day)**	**± 3 days (± 7 for scans)**					**± 7 days**	**+ 7 days**	**± 7 days**	**Study drug supplies must be taken into account when scheduling visits during the visit windows.**
Health Resource Assessments										
Healthcare Resource Use for Hospitalizations		X (Starting Cycle 2)					X	X		See Section 8.2.6. Healthcare resource use for hospitalizations will be collected once every cycle from cycle 2.

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Table 2. Schedule of Activities: Participants Receiving Docetaxel – Randomized to Arm B and Assigned to Physician's Choice of docetaxel

	Visit Identifier									Notes
	Screening	TREATMENT						Follow-up		
Abbreviations used in this table are found in Appendix 13.	**SCR**	**C1D1**	**C2D1**	**C3D1**	**Cycle 3 Day 15 (Week 9) and every 8 weeks thereafter**	**Cycle 4 Day 1 (and D1 of every subsequent cycle)**	**EOT**	**28 day Safety Follow-up**	**LTFU every 12 weeks**	**Docetaxel treatment Cycle defined as every 21 days (maximum of 10 cycles)** **Treatment visits should be fixed according to the prior visit date.**
Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
Informed Consent	X									See Section 10.1.3. Informed consent must be obtained prior to undergoing any study specific procedures. Ensure consent is on the current version of the form approved by the ethics committee and sponsor
Screen for inclusion/exclusion criteria	X									See Section 5.1 and Section 5.2
Randomization		X								After participant is deemed eligible for the study; the participant enrollment number and Treatment Arm are assigned at randomization. Participants should start treatment within 3 days after randomization.
Investigator reason for treatment choice of docetaxel or enzalutamide		X								Select primary reason for investigator's choice of treatment prior to randomization.
Demographics	X									
Medical History	X									Relevant medical and surgical history and current illnesses. Includes histology, TNM classification, Gleason score.

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Table 2. Schedule of Activities: Participants Receiving Docetaxel – Randomized to Arm B and Assigned to Physician's Choice of docetaxel

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Physical Exam	X	X	X	X		X	X	X		See Section 8.3.1. Physical examinations to be completed before administration of study intervention
Vital signs, height, weight	X	X	X	X		X	X	X		See Section 8.3.1. Measure blood pressure, heart rate, temperature (ºC). Height (cm) and weight (kg); height only required at screening.
ECOG Performance Status	X	X	X	X		X	X	X		See Section 10.11, Appendix 11.
12 Lead ECG	X	X	X				X	X		ECG at C1 D1 and C2 D1 should be collected at 2-hours (+2 hr window) postdose. Management of signs or symptoms of ischemic heart disease is to be per general clinical practice and local guidelines, including QTcF. interval (machine reported or manually). See Section 8.3.2., Appendix 8.
Contraception check	X	X	X	X		X	X	X		See Section 5.3.1 and Section 10.4, Appendix 4.

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Contraceptive counseling to seek advice about donation and cryopreservation of sperm cells prior to start of study	X									See Section 5.3.1.
Prior/concomitant treatment(s)	X									See Section 6.9.
Screening Tumor Biopsy	X									Only required if no prior histological or cytological confirmation of prostrate adenocarcinoma is available.
Serious/nonserious AE monitoring	X									Report serious and non-serious adverse event information from time informed consent form is signed through screen failure or through a minimum of 28 days after the last dose of study intervention; See Section 8.4 for additional information.
Symptomatic Skeletal Events		X								See Section 8.3.4.
Laboratory Assessments – Central / Local Lab										
Hematology, Blood Chemistry, CD4 count* (pre-dose)	X	X	X	X		X	X	X		Tests will not need to be repeated at C1 D1 if Screening tests were performed within 7 days of C1 D1. See Section 8.3.3 and Section 10.2, Appendix 2. Samples may

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Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
										be collected up to 3 days prior to the visit to guide decision making. *Consider obtaining CD4 count if lymphocyte count is <500/µL
Serum Testosterone	X									
Tumor markers – PSA (pre dose)	X	X	X	X		X	X	X	X	collect during LTFU only if participant did not end study treatment due to BICR confirmed rPFS
Radiographic Assessments	Imaging assessments are to be scheduled using the randomization date as the reference date for all time points. See Section 8.2.2; All radiographic assessments must be forwarded to the independent core imaging laboratory; the same method of assessment and the same technique should be used to characterize each identified and reported lesion at baseline and during follow-up									
CT Scan of chest; CT or MRI of abdomen and pelvis	X (-42 days accepted)				X[a]		X[b]		X[c]	Screening/baseline scans: may use scans obtained as part of standard of care before consent was signed and within 42 days prior to Day 1 (Randomization) **if scans were performed per the specific study requirements (per imaging manual).** a Scans will be performed every 8 weeks (at Week 9, 17, 25) then every 12 weeks (at Week 37, 49, 61, etc.); Radiographic assessments should continue for
Whole body bone scan: Tc-99m Bone Scintigraphy	X (-42 days accepted)				X[a]		X[b]		X[c]	

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Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
										all participants until blinded independent central confirmation of radiographic progression. b Tumor assessments are to be performed during the EOT visit if the reason was other than central radiographically confirmed disease progression and it has been ≥ 8 weeks since the last assessment. c Not required during follow-up if disease progression has been confirmed by BICR.

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Table 2. Schedule of Activities: Participants Receiving Docetaxel – Randomized to Arm B and Assigned to Physician's Choice of docetaxel

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Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
PRO Assessments										
BPI-SF, FACT-P, EQ-5D-5L	X	Every 6 weeks (starting Cycle 1 Day 1)					X	X	X	See Section 8.2.4. Participants will self-administer assessments via an electronic device. On C1 D1, complete questionnaires after randomization, but before the first dose of study intervention. At all subsequent scheduled PRO assessments, participant is to complete the questionnaires +/- 3 days of the scheduled assessment. In cases in which the PRO assessment timepoint overlaps with a study visit, all PROs must be completed before other study activities
PRO-CTCAE and FACT-GP5	X	Every week for the first 6 weeks then every 6 weeks (starting Cycle 1 Day 1) until Week 25					X			See Section 8.2.4. Participants will self-administer assessments via an electronic device. On C1 D1, complete questionnaires after randomization, but before the first dose of study intervention. For the scheduled PRO-CTCAE and FACT-GP5 assessments during the first 6 weeks, participant is to

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Table 2. Schedule of Activities: Participants Receiving Docetaxel – Randomized to Arm B and Assigned to Physician's Choice of docetaxel

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Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
										complete the questionnaires +/- 1 days of the scheduled assessment. Then for all subsequent PRO-CTCAE and FACT-GP5 assessments after Week 6, the patient is to complete the questionnaires +/- 3 days of the scheduled assessment. In cases in which the PRO assessment timepoint overlaps with a study visit, all PROs must be completed before other study activities.
Biomarker Assessments										
Tumor tissue (archival or de novo)		X								See Section 8.8.1; Designation of available pretreatment tumor tissue for shipment to sponsor for retrospective molecular profiling analysis.
Blood sample for cf nucleic acids analyses (pre-dose)		X			X [C3 D15 (Week 9) and C9 D1 (Week 25) only]		X			See Section 8.8.2, 8.8.4, and Laboratory Manual;
Blood sample for serum biomarker analysis (pre-dose)		X			X [C3 D15 (Week 9) and C9 D1 (Week 25) only]		X			See Section 8.8.4 and Laboratory Manual;

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Blood sample for matched germline control (pre-dose)		X								See Section 8.8.3 and Laboratory Manual;
Retained research sample for biomarkers (Prep D1) (pre-dose)		X								See Section 8.7.2. If not collected on C1 D1, collect at the next available time point when biospecimens are being collected in conjunction with a participant visit.
Study Intervention										
Study intervention administration (docetaxel)		X	X	X		X				Docetaxel administration will continue up to a maximum of 10 cycles (whereupon no study intervention shall be given, but assessments continue until PD).
Corticosteroid administration		X	X	X		X				Pre-med and BID; See Table 3.
Collection of new antineoplastic/Investigational Therapy (may be completed via phone)		X								Record all subsequent antineoplastic treatments systemic or localized (ie radiotherapy, surgery) for participant starting a new antineoplastic or investigational therapy.

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Visit Window	**-28 days to -1 day**	**± 3 days**			**± 7 days**	**± 3 days**	**± 7 days**	**+ 7 days**	**± 7 days**	**Participant completion of 10 txt cycles, EOT visit should be scheduled 21 days (+/-7 days) after C10D1**
Diagnosis of Second Primary Malignancy	X	X								Record any second primary malignancy including MDS, T-LBL, and AML and report as an SAE. See Section 4.1.
Survival Status									X	Obtain survival status by telephone, clinic visit, chart review, or by communicating with an individual (eg, email, family, friend, or referring health care provider) who is knowledgeable of the participant's survival status. Publicly available information may be used to determine vital status only as appropriately directed in accordance with local law. See Section 7.1.2
Healthcare Resource Assessments										
Healthcare resource use for Hospitalizations			X	X		X	X	X		See Section 8.2.6. Healthcare resource for hospitalizations use will be collected once every cycle (starting at Cycle 2)

2. INTRODUCTION

PF-06821497 is a potent and selective inhibitor of EZH2 enzymatic activity. In vivo, PF-06821497 showed robust TGI and induced durable tumor regressions, including in a prostate xenograft model, in combination with enzalutamide and anti-tumor activity in an enzalutamide-resistant prostate cancer tumor model.

2.1. Study Rationale

The rationale of the study is to evaluate whether the addition of EZH2 inhibition to enzalutamide can reverse anti-androgen resistance, thereby increasing the duration of clinical benefit of enzalutamide in participants who have previously been treated with abiraterone. This Phase 3 randomized study (C2321014) is designed to demonstrate that PF-06821497 plus enzalutamide provides superior clinical benefit compared to physician's choice of treatment (enzalutamide or docetaxel) in participants with metastatic castration resistant prostate cancer who have previously been treated with abiraterone acetate.

2.2. Background

Prostate cancer is the second most commonly diagnosed malignancy and fifth leading cause of cancer death in men in the US, with an estimated 1,414,000 new cancer cases and 375,304 deaths in 2020 (Sung et al, 2021). Currently available therapies consist of medical castration by depletion of testicular androgens or by targeting AR signaling pathways. Although initial response to ADT occurs, tumors often relapse after castration, in what is termed CRPC. CRPC represents a lethal transition in the progression of prostate cancer, with most patients ultimately succumbing to the disease. The five-year survival rate for patients with mCRPC remains at approximately 30% (Sung et al, 2021).

Over the last 10 years, multiple therapies have been shown to confer a survival benefit for patients with mCRPC and were approved on this basis, including docetaxel, cabazitaxel, Sipuleucel-T, Radium 223 dichloride, and ARSi or abiraterone which target the AR pathway, abiraterone acetate and enzalutamide. Based on their tolerability and proven efficacy in the pre-chemotherapy setting, ARSi or abiraterone have often been used as first-line therapies for mCRPC. However, resistance inevitably occurs, and AR signaling remains a key oncogenic driver in prostate cancer, even after treatment with AR-targeted therapies.

Research in prostate cancer is now focused on understanding the mechanisms of resistance to ARSi or abiraterone. Resistance to abiraterone acetate and enzalutamide is believed to be driven by the preservation of AR signaling via similar mechanisms found in resistance to first-line androgen deprivation treatment, such as increased expression of the AR(Armenia et al, 2018; Quigley et al, 2018; Wedge et al, 2018), AR gene amplification (Quigley et al, 2018), the presence of recurrent GOF mutations in the AR gene (Robinson et al, 2015; Lallous et al, 2016), or expression of AR splice variants that lack the ligand binding domain (Antonarakis et al, 2014; Scher et al, 2016a).

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2.2.1. EZH2 Inhibition

Co-treatment with therapies that modulate alternative signaling pathways involved in oncogenesis in prostate cancer have been proposed as a modality to overcome resistance to ARSi or abiraterone, including epigenetic modifiers such as EZH2.

EZH2 is the catalytic subunit of the PRC2, which acts as a histone methyltransferase. PRC2 is part of a multiprotein complex that regulates cell development through chromatin compaction and gene expression. Mutation and/or overexpression of EZH2 has been implicated in the pathogenesis of multiple tumor types including prostate cancer, FL, DLBCL, SCLC, and epithelioid sarcoma which can promote increased gene repression leading to cancer growth, metastatic dissemination, and immunity. Clinical studies have demonstrated significant benefit with EZH2 inhibition, leading to the approval of an EZH2 inhibitor as monotherapy (tazemetostat) in epithelioid sarcoma and FL.

EZH2 has been implicated in the pathogenesis of prostate cancer through multiple pathways, including:

- Its canonical function as part of the PRC2 complex to epigenetically silence genes, including tumor suppressors, through histone H3 tri-methylation (H3K27me3) (Yang & Yu, 2013);
- PRC2-independent methylation and activation of transcription factors, including the AR (Xu et al, 2012);
- PRC2-independent methylation and stabilization of cellular proteins including FOXA1- leading to cell cycle dysregulation (Park et al, 2021);
- Increased transcription of AR through binding to the AR promoter (Kim et al, 2018);
- Induction of neuroendocrine differentiation that is associated with AR independence and poor prognosis (Varambally et al, 2002; Ku et al, 2017).

Patients with clinically localized prostate cancers with higher expression of EZH2 show a worse prognosis. EZH2 is a transcriptional repressor due to its methyl transferase activity and may also act as a transcriptional activator targeting the AR and its downstream signaling pathway. In prostate cancer, increasing EZH2 messenger RNA and protein expression correlates with advancing disease progression (Bryant et al, 2007).

2.2.2. PF-06821497

PF-06821497 is a potent and selective inhibitor of EZH2 enzymatic activity which inhibits WT and EZH2 Y641N mutant enzymes. PF-06821497 is a competitive inhibitor with respect to S-adenosyl-L-methionine substrate, based on the mechanism of action data for the structurally related inhibitor PF-06726304. It shows strong reduction in H3K27me3 levels (EZH2 pharmacodynamic biomarker) both biochemically and in cellular assays (Kung et al, 2018) and data on file.

2.2.3. Rationale for Development in Prostate Cancer

ARSi or abiraterone and enzalutamide are efficacious and established standard of care treatment options for participants with prostate cancer. In the first line mCRPC setting enzalutamide reduced the risk of or death by 68% (hazard ratio [HR] 0.32, 95% confidence interval [CI] 0.28–0.37; $p < 0.0001$) and the risk of death by 23% (HR 0.77, 95% CI 0.67–0.88; $p = 0.0002$). There was an improvement of median PFS (20.0 months vs 5.4 months) and OS (35.3 months vs 31.3 months) (Beer et al, 2017). As outlined above, EZH2 has been shown to cooperate with the AR to drive prostate cancer progression through both catalytic and non-catalytic mechanisms (Fong et al, 2017; Khalaf et al, 2019). For patients who continue treatment, resistance to ARSi or abiraterone inevitably develops and is associated with transition to an AR-independent neuroendocrine phenotype. In addition to its oncogenic role in AR-dependent disease, EZH2 has been shown to promote transition of prostate cancer to NEPC state. EZH2 inhibition has been shown to reverse the reprogramming of these tumors back to a luminal state (Ku et al, 2017). Studies conducted in genetically engineered mouse models demonstrated that Rb1 and TP53 cooperate to suppress prostate cancer lineage plasticity, metastasis, and anti-androgen therapy resistance by repressing reprogramming factors such as EZH2 and SOX2 (Ku et al, 2017). Dual knock-out of these tumor suppressors in vivo increased expression of SOX2 and EZH2, and treatment with EZH2 inhibitors led to re-expression of AR and sensitized the models to enzalutamide including significantly inhibited tumor growth by combination of the EZH2 inhibitor GSK503 with enzalutamide (Ku et al, 2017). In in vivo studies conducted in the CRPC C4-2 prostate cancer model, PF-06821497 showed dose-dependent anti-tumor in vivo efficacy (maximum efficacy: 9% tumor regression at 300 mg/kg/dose BID); enzalutamide monotherapy showed a TGI benefit of 89%. While some moderate anti-tumor activity was observed with PF-06821497 and enzalutamide monotherapies, it was the combination of PF-06821497 with AR antagonist enzalutamide which was significantly better than both monotherapies inducing durable tumor regression and long-lasting complete responses (CRs). Combination of 100 mg/kg PF-06821497 with 10 mg/kg enzalutamide resulted in 122% TGI (95% tumor regression). PF-06821497 also demonstrated dose-dependent in vivo inhibition of tumor growth (maximum efficacy: 63.5% TGI at 100 mg/kg QD SC) aligned with reduced H3K27Me3 levels in the enzalutamide-resistant 22RV1 xenograft prostate cancer tumor model. Combining 100 mg/kg PF-06821497 treatment with 10 mg/kg enzalutamide increased TGI to 72.5%.

C2321001 is a Phase 1, first in human, open-label, muti-center study in patients with advanced solid tumors including CRPC (refer to Section 2.2.4). An analysis from the randomized dose expansion (Part 2B) of this study, with a data cutoff of 14 August 2023 with 16 rPFS events, estimates the hazard ratio as 0.33 (90% CI 0.131-0.847), strongly indicating that the combination of PF-06821497 and enzalutamide significantly reduces the risk of progression compared to enzalutamide monotherapy in abiraterone acetate exposed mCRPC patients.

These data support a combination approach of PF-06821497 with a second-generation ARSi in the mCRPC setting in patients following progression on abiraterone acetate.

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2.2.4. Clinical Overview

The safety, tolerability, PK, PD, and anti-tumor activity of PF-06821497 were evaluated in C2321001, a Phase 1, first in human, open-label, multi-center study in participants with advanced solid tumors including CRPC. Preliminary data from this ongoing study (N=112 as of 18 Nov 2022, including N=66 with CPRC) indicate that PF-06821497 as monotherapy and in combination with SOC agents including enzalutamide has been well tolerated in doses from 75 mg to 1250 mg BID.

Of 107 participants receiving PF-06821497 as monotherapy or in combination with SOC, 105 participants had at least 1 TEAE, with the most frequently reported (≥20%) treatment related TEAEs of any grade being anaemia, nausea, neutropenia, thrombocytopenia, diarrhea, decreased appetite, and fatigue. A total of 71 SAEs (all causalities) were reported in 40 participants. There was one fatal event considered related to PF-06821497, hepatic failure (Refer to Investigator's Brochure).

In this Phase 1 study, PF-06821497 exhibited PD activity in blood and tumor tissue: A ≥70% maximal decrease in H3K27Me3 marker level from baseline was observed in granulocytes from participants treated with PF-06821497 monotherapy at ≥75 mg BID (up to 500 mg BID) and from participants with mCRPC treated with PF-06821497 at doses ≥ 375 mg BID in combination with enzalutamide (Schweizer, 2022). Further, PF-06821497 in combination with enzalutamide reduced tumor H3K27Me3 PD marker more effectively at 1250 mg BID than at 500 mg BID.

The randomized Phase 1 expansion Part 2B portion of the C2321001 study included participants with mCRPC who had previously received abiraterone and has completed enrollment with 82 participants dosed in total. Thirty-seven participants were randomized to the PF-06821497 in combination with enzalutamide and 38 to enzalutamide alone. Baseline characteristics were well balanced across both arms, approximately 40% of patients received prior docetaxel.

Preliminary data (with a median follow-up of 2.6 and 2 months in the combination arm and enzalutamide arm respectively), demonstrated that PF-06821497 + enzalutamide significantly reduced the risk of progression in participants with mCRPC previously treated with abiraterone, with a stratified HR of 0.33 (90% CI 0.131, 0847). Median rPFS in the combination arm was not evaluable (90% CI: 7.1, NE) vs 4.2 months for enzalutamide (90% CI: 2.1, NE). PF-06821497 + enzalutamide. The most frequent all-causality adverse events (≥10%) occurring with the combination of PF-06821497 plus enzalutamide include diarrhea, dysgeusia, decreased appetite, anemia, fatigue, nausea, alopecia, asthenia, vomiting, hypocalcemia, hypokalemia, neutropenia, thrombocytopenia, weight decreased and edema peripheral.

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2.3. Benefit/Risk Assessment

More detailed information about the known and expected benefits and risks and reasonably expected AEs of PF-06821497 may be found in the IB, which is the SRSD for this study. Refer to the Study Intervention(s) table in Section 6.1 for a complete description of SRSDs, including SRSDs for enzalutamide, docetaxel, prednisone, prednisolone, and dexamethasone (pre-medications for docetaxel).

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2.3.1. Risk Assessment

Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Study Intervention(s) PF-06821497		
Hepatic toxicity (Hepatic failure, important potential risk)	One death due to hepatic failure was reported in a participant with CRPC enrolled in C2321001 (phase 1 study) and treated with 150 mg BID single agent PF-06821497, details of this case are available in the Investigators Brochure. Hepatic toxicity is categorized as an important potential risk.	Participants with moderate/severe hepatic impairment are excluded. Specific dosing interruptions are to be implemented in case of suspected drug-induced liver toxicity. Rules for temporary and permanent discontinuation are described in Section 6.6.6.
GI toxicity- diarrhea	EZH2 has a role in maintaining the intestinal epithelial integrity and homeostasis under inflammatory conditions, and its inhibition may cause disruption of intestinal homeostasis. Diarrhea is a frequently observed adverse event with PF-06821497 and is one of the most common TEAE across all tumor types in monotherapy and in combination with SOC. The cases are predominantly low grade.	Diarrhea will be managed within current protocol guidance providing detail on the mitigation of diarrhea. Reference guidelines for the management of diarrhea dose modifications are in Table 6 and Section 6.9.6.
Opportunistic infections Myelosuppression/Cytopenias	Non-clinical findings indicate a potential role for EZH2i in T-cell function.	Regular monitoring of lymphocyte counts, and CBC are to be performed, with implementation of additional tests and prophylaxis as needed. An appropriate prophylactic treatment including standard PCP prophylaxis should be considered. See Table 6 for dose modification guidance.
Secondary malignancies	Since mevrometostat is in the EZH2i drug class, secondary malignancies are categorized as an important potential risk. Details for potential second primary malignancies in adult participants receiving mevrometostat in the Phase 1 study (C2321001) are available in the IB. PF-06821497 was not found to be genotoxic in preclinical assessments.	Monitoring and reporting of all occurrences of secondary malignancies as SAEs regardless of any grade or causality attribution. Please refer to Section 8.4 for additional details.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
Embryo-fetal toxicity	Pharmacology based risk of fetal harm and infertility. PF-06821497 has the potential to cause embryo fetal toxicity; the tazemetostat prescribing information indicates fetal skeletal malformations and variations in EFD studies in rat and rabbit.	All participants will use effective contraception methods; contraception use will be monitored throughout the study.
Study Intervention(s) **Enzalutamide**		
Seizure	In 8 RCTs, (of participants receiving enzalutamide) 0.6% participants treated with enzalutamide experienced a seizure.	Participants with clinically significant history of seizure or any condition that may predispose to seizure will be excluded.
PRES	There have been rare reports of PRES in participants receiving enzalutamide during post-approval use.	Permanently discontinue enzalutamide in participants who develop PRES
CVD	In the combined data of 5 RCTs (AFFIRM, PROSPER, PREVAIL,ARCHES and EMBARK), IHD occurred in 3.5% of participants in the enzalutamide arm.	Participants with clinically significant cardiovascular disease will be excluded.
Hypersensitivity	Hypersensitivity reactions including edema of the face (0.5%) tongue (0.1%) or lip (0.1%) have been observed with enzalutamide in 7 RCT.	In accordance with the local label, participants who experience any symptoms of hypersensitivity should temporarily discontinue enzalutamide and promptly seek medical care. Permanently discontinue enzalutamide for serious hypersensitivity reactions.
Falls and Fractures	In the combined data of 5 RCTs falls occurred in 12% of participants treated with enzalutamide compared to 6% treated with placebo. Fractures occurred in 13% of participants treated with enzalutamide and 6% of participants with placebo.	Further details and guidelines for safety monitoring of enzalutamide are according to the IB and local prescribing information.
Dysphagia	Severe dysphagia or choking, including events that could be life-threatening requiring medical intervention or fatal, can occur due to enzalutamide product size	Discontinue enzalutamide for patients who cannot swallow capsules.
Study Intervention(s): Docetaxel		
Hematologic toxicity	Neutropenia is the most frequent AE of docetaxel. Neutrophil nadirs occurred at a median of 7 days,	Investigators to follow dose modification guidance per SRSD.

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Potential Risk of Clinical Significance	Summary of Data/Rationale for Risk	Mitigation Strategy
	but the interval may be shorter in heavily pre-treated patients.	Adequate bone marrow function is required for study inclusion and participants will be monitored throughout the study intervention period (See SoA). Unplanned hematological assessment as clinically indicated. Study interventions will be interrupted, reduced, or permanently discontinued based on severity of the toxicity as outlined in the docetaxel SRSD. Participants with active infections are excluded from participation in the study. Moreover, participants will be monitored for signs and symptoms of infection and treated as medically appropriate.
Hypersensitivity reactions	These risks are based on the docetaxel SRSD	Participants will be premedicated with an oral corticosteroid prior to docetaxel treatment.
Study Procedures		
Participants will have CT/MRI scans and bone scans during study participation.	CT scans expose participants to a small dose of radiation. Contrast dye used for CT scans may cause pain or burning upon injection, may worsen kidney function in participants with kidney disease and may cause allergic reactions that could be severe and life-threatening. Radioactive material for bone scan may cause discomfort or burning upon injection.	Radiation exposure during the study is not expected to exceed what a participant would receive under standard of care (SOC) and should not create a significant risk to health. Participants with renal function impairment are excluded from participation in the study. CT scans will be conducted at facilities prepared for adverse reactions to the contrast dye. Hydration and/or premedication prior to contrast media administration or steroids administration in case of allergic reactions may be instituted as per local standard practice.
In the event that a participant is not able to visit the pre-specified laboratory, protocol-specified safety laboratory evaluations or ECGs may be conducted at a different local laboratory once it is approved by the investigator and the sponsor.	Participant safety may be jeopardized if safety assessments are not correctly performed and/or the investigator is not made aware of abnormal findings in a timely manner.	Only routine safety assessments will be performed by alternative facilities, and the test results must be provided to qualified site staff as soon as possible.

2.3.2. Benefit Assessment

The results of an ongoing Phase 1 study of PF-06821497, C2321001, either as monotherapy in participants with solid tumors or in combination with enzalutamide in participants with mCRPC, support this investigation of PF-06821497 plus enzalutamide for participants with mCRPC after treatment with abiraterone acetate. Taking into account the measures to minimize risk to participants, the potential risks associated with PF-06821497 plus enzalutamide are justified by the benefits that may be afforded to participants with mCRPC following prior abiraterone acetate treatment. Therefore, there is a favorable benefit-risk profile to support the study rationale.

Participants may experience delayed progression of disease and improved quality of life from the combination of PF-06821497 plus enzalutamide. Study participants may also benefit from more intense monitoring and more frequent assessments.

Based on the Phase 1 clinical data for mCRPC participants, the most frequent all-causality adverse events (≥10%) occurring with the combination of PF- 06821497 plus enzalutamide include diarrhea, decreased appetite, nausea, fatigue, dysgeusia, asthenia, edema peripheral, vomiting, weight decreased, alopecia, hypokalemia, hypocalcemia, anemia, thrombocytopenia and neutropenia. Study safety data will be monitored on a regular basis by the sponsor as well as an independent E-DMC.

Male participants will be notified to seek advice regarding sperm cryopreservation due to the possibility of reduced sperm counts during treatment.

2.3.3. Overall Benefit/Risk Conclusion

Taking into account the measures taken to minimize risk to study participants, the potential risks identified in association with PF-06821497 are justified by the anticipated benefits that may be afforded to participants with mCRPC.

3. OBJECTIVES, ENDPOINTS, AND ESTIMANDS

Objectives	Endpoints	Estimands
Primary:	**Primary:**	**Primary:**
• To demonstrate that PF-06821497 in combination with enzalutamide is superior to PCT of enzalutamide or docetaxel in prolonging rPFS.	• BICR assessed rPFS per RECIST v1.1 (soft tissue disease) and PCWG3 (bone disease).	• Refer to Section 9.1.1.1
Key Secondary:	**Key Secondary:**	**Key Secondary:**
• To demonstrate that PF-06821497 in combination with enzalutamide is superior to PCT (enzalutamide or docetaxel) in prolonging OS.	• OS (alpha protected).	• Refer to Section 9.1.1.2

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Objectives	Endpoints	Estimands
Other Secondary:	**Other Secondary:**	**Other Secondary:**
To evaluate anti-tumor activity	• Proportion of participants with measurable soft tissue disease at baseline with an objective response per RECIST v1.1 (assessed by BICR).	• Not applicable
	• Duration of response in soft tissue disease per RECIST v1.1 (assessed by BICR).	
	• Proportion of participants with PSA response ≥50% in participants with detectable PSA values at baseline.	• Not applicable
	• Time to PSA progression.	• Not applicable
	• Time to initiation of new antineoplastic therapy.	• Not applicable
	• Time to first symptomatic skeletal event.	• Not applicable
	• PFS2 based on investigator assessment.	• Not applicable
• To compare safety and tolerability between the treatment arm and the control arm	• Type, incidence, severity (as graded by NCI CTCAE v5.0), seriousness and relationship to study medications of AEs.	• Not applicable
• To compare patient reported outcomes between the treatment arm and the control arm	• Change from baseline in patient reported pain symptoms per BPI-SF; • Change from baseline in HRQoL, functioning and symptoms per FACT-P; • Change from baseline in patient reported health status per EQ-5D-5L; • Symptomatic toxicity and the overall side effect burden as measured by items from the Patient-Reported Outcome CTCAE (PRO-CTCAE) and FACT-GP5; • Time to confirmatory deterioration in patient-reported pain symptoms per BPI-SF Item 3 “worst pain in 24 hours” • Time to definitive deterioration in patient-reported HRQoL and physical well-being per FACT-P.	• Refer to Section 9.3.3.2
• To evaluate the PK of PF-06821497 when dosed with enzalutamide	• PK characterized by pre-dose trough and post-dose plasma concentrations of PF-06821497 at selected visits.	• Not applicable
• To assess the relationship between ctDNA burden and outcome.	• ctDNA burden at baseline and on study.	• Not applicable

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Objectives	Endpoints	Estimands
Exploratory:	**Exploratory:**	**Exploratory:**
• To understand the relationship between the therapeutic intervention(s) being studied and the biology of the participant's disease.	• Measurements of biomarkers, which may consist of DNA, RNA, protein or defined cell types, resulting from analyses of peripheral blood and/or tumor tissue biospecimens obtained at baseline, on treatment and/or at EOT.	• Not applicable
• To evaluate healthcare resource use data	• Hospitalizations including length of stay	• Not applicable

4. STUDY DESIGN

4.1. Overall Design

This is a global, multicenter, randomized, open-label Phase 3 study evaluating PF-06821497 plus enzalutamide versus physician's choice of either enzalutamide or docetaxel for participants with mCRPC after progression on prior abiraterone acetate. Approximately 174 investigational sites will participate in the study. Participants may have received prior chemotherapy for mCSPC but are required to be chemotherapy naïve in the castration-resistant setting. Participants who received prior treatment with 2nd generation ARSi's in any setting (eg, enzalutamide, apalutamide or darolutamide) will be excluded. This study consists of a Screening Phase, Randomization, Treatment Phase, Safety Follow-up and Long-Term Follow-up.

Participants will receive PF-06821497 in combination with enzalutamide (Arm A) or physician's choice of comparator therapy of either enzalutamide or docetaxel (Arm B) in the Treatment Phase. The physician's choice of agent in the control arm must be prespecified prior to randomization. Participants should start treatment within 3 days after randomization.

Approximately 600 Participants will be randomly assigned on a 1:1 basis to:

- Investigational Arm A: PF-06821497 875 mg BID + enzalutamide 160 mg QD
- Comparator Arm B: enzalutamide 160 mg QD or docetaxel 75 mg/m^2 IV every 21 days

Docetaxel at a dose of 75 mg/m^2 will be administered IV every 21 days, for up to a maximum of 10 cycles. Docetaxel is given in combination with prednisone, prednisolone, and dexamethasone.

Randomization will be stratified by:

- Previous treatment with docetaxel in the mCSPC setting (yes vs. no)
- Pre-specified Physicians' choice of comparator (docetaxel vs enzalutamide)
- Presence of hepatic metastases (yes vs. no)

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Randomized participants will receive protocol-assigned treatment as per assigned treatment arm continuing until study intervention discontinuation criteria (Section 7.1) are met. Participants should start treatment within 3 days after randomization.

Efficacy analyses (rPFS, ORR, DoR) will be performed using BICR.

Participants on the docetaxel physician's choice treatment arm will continue to undergo study assessments following completion of up to 10 cycles of study intervention and will be able to continue to receive ADT (GnRH agonist/antagonist) as per SOC.

Participants will undergo study-related safety and efficacy assessments, as outlined in the relevant Schedule of Activities, Table 1 and Table 2.

Disease status will be assessed at Screening and at regular intervals during the study by CT of chest and CT or MRI of abdomen and pelvis for assessment of soft tissue disease, whole body bone scan (eg, radionuclide (Tc-99m) bone scintigraphy), PSA measurements, and PRO assessments. To determine the primary endpoint of radiographic progression, response and progression for soft tissue disease will be evaluated per RECIST 1.1 and bone disease progression will be evaluated per PCWG3 (See Appendix 10). A third-party core imaging laboratory will perform BICR assessment of rPFS data. All radiographic assessments must be forwarded to the independent core imaging laboratory as requested by the sponsor. BICR review of radiographic assessments ends following BICR determined radiographic progression.

Safety will be assessed by AEs, physical examinations, vital signs, and clinical laboratory tests. An independent external E-DMC will monitor the safety of the participants on a periodic basis.

During the Treatment Phase, treatment continues until BICR determined radiographic disease progression, unacceptable toxicity, an AE leading to permanent study intervention discontinuation, or other discontinuation of study intervention criteria are met (Section 7.1). Participants discontinuing the Treatment Phase will continue to the End of Treatment (EOT) Visit and Safety Follow-up (28 - 35 days after last dose of study intervention or before initiation of a new antineoplastic or investigational therapy, whichever occurs first).

Participants enter the LTFU period after Safety Follow-up. If the participant has not discontinued from study procedures and/or post treatment study follow-up, long term follow-up will continue until the end of study.

Participants who discontinue treatment and do not have radiologic disease progression will continue radiographic imaging and PSA assessments (per SoA) until they progress radiographically as determined by BICR (regardless of the initiation of a new antineoplastic therapy) or death. After confirmation of radiographic progression by BICR, further radiographic imaging and PSA assessments will continue as per local clinical practice until the next report of progression after the start of subsequent antineoplastic therapy.

For all participants in the LTFU period, survival status, initiation of new antineoplastic therapy, diagnosis of MDS, T-LBL, and AML or second primary malignancy will be monitored and collected every 12 weeks (± 7 days) from Safety Follow-up visit until the

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participant dies or withdraws consent for follow-up, or the study is terminated by the sponsor. LTFU will no longer be required after the desired number of OS events have been achieved. Crossover will not be allowed in the trial.

Tumor tissue and blood samples will be collected to evaluate biomarkers (genes and proteins) involved in the biology of the participant's disease to understand the relationship between the therapeutic interventions being studied and the biology of the participant's disease. The measurement of the biomarkers may consist of DNA, RNA, protein, or defined cell types resulting from analyses of peripheral blood and/or tumor tissue biospecimens obtained at baseline, on treatment, and/or at EOT. Assessment of the relationship between ctDNA burden and outcome is a secondary objective of this study. Unless prohibited by local regulation or ethics committee decision, a retained research sample for biomarker analyses related to the study intervention(s) and prostate cancer will be obtained.

Once communication is sent to the investigational sites that the primary completion date (PCD) has been reached, participants who are still on study treatment will follow a reduced study schedule. PCD is expected to occur at the time the number of rPFS events have been achieved for the primary endpoint analysis (Section 9.4).

Pharmacokinetics is a secondary endpoint for the study. Collection of PK samples enables population PK analyses to understand the effect of participant characteristics (body weight, age, race and ethnicity, sex, or organ impairment) on PF-06821497.

4.1.1. Diversity of Study Population

Reasonable attempts will be made to enroll participants with different characteristics to ensure the study population is representative of the patient population that will use the study intervention, PF-06821497, in clinical practice. The diversity strategy will include sites with the potential to support the recruitment of diverse populations across geographically diverse countries.

4.2. Scientific Rationale for Study Design

4.2.1. Patient Input Into Design

Prostate cancer patients shared insights during a series of several meetings, including a meeting specific to the current study. No significant findings preventing study participation in the study design and schedule of activities have been identified.

4.2.2. Rationale for Physician's Choice of Comparators

Participants will be randomized to receive PF-06821497 in combination with enzalutamide (Arm A) or physician's choice of comparator therapy of either enzalutamide or docetaxel (Arm B) in the Treatment Phase. The choice of enzalutamide or docetaxel as the control group was selected to allow the investigators to decide on the appropriate treatment based on factors such as the individual participant's clinical condition, prior treatment history and local standard of care practices. Both enzalutamide and docetaxel are approved for mCRPC (Section 4.2.2.1 and Section 4.2.2.2) and are well established standard of care therapies.

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4.2.2.1. Enzalutamide

ARSi's such as enzalutamide are an efficacious and established SOC in prostate cancer. In the PREVAIL study, which included chemotherapy naïve participants with mCRPC, enzalutamide vs placebo demonstrated an improvement of median PFS (20.0 months vs 5.4 months) and an improvement in OS (35.3 months vs 31.3 months) (Evans et al, 2016; Beer et al, 2017), respectively.

Given these data, enzalutamide has become an approved therapy for patients with mCRPC and is considered SOC in this setting. Accordingly, the current NCCN and ESMO guidelines (Parker et al, 2020) both recommend the use of enzalutamide in combination with ADT as an option for mCRPC and therefore is an appropriate control for this study.

Sequencing from abiraterone acetate to enzalutamide in mCRPC

Enzalutamide or abiraterone are currently chosen as the first agent in most patients with CRPC. Following disease progression, the alternative agent is a common standard of care treatment that may be used subsequently instead of chemotherapy. A number of retrospective studies have analyzed the outcomes of sequential treatment with abiraterone acetate and enzalutamide in either order. The findings have been varied. Some studies have reported a very low PSA response rate (around 10%) which suggests a cross-resistance between the two agents. However, other studies reported an acceptable PSA response rate (around 30%), which suggests that sequential treatment of enzalutamide and abiraterone could be beneficial in a certain group of patients (Chung et al, 2019; Bjartell et al, 2022).

Chopra et al. performed indirect comparisons between abiraterone acetate and enzalutamide in both pre-docetaxel and post-docetaxel settings based on published phase 3 RCTs and found that enzalutamide outperformed abiraterone acetate with prednisone in terms of radiographic PFS, time until PSA progression, and PSA response rate but not OS in both the pre- and post-docetaxel settings (Chopra et al, 2017). The lack of randomized head-to-head comparison data examining sequencing between abiraterone acetate to enzalutamide and between enzalutamide to abiraterone acetate makes it difficult to choose the optimal first-line treatment either pre- or post-chemotherapy in patients with mCRPC.

A small, randomized phase 2 crossover trial (n=202) investigated the best sequence between abiraterone acetate → enzalutamide (group A) vs. enzalutamide → abiraterone acetate (group B) for the first-line treatment of newly diagnosed mCRPC (Khalaf et al, 2019). Longer time to PSA progression on second-line therapy (19.3 vs. 15.2 months, HR 0.66, 95% CI 0.45–0.97) and PSA response rates to second-line therapy (36% vs. 4%, $p < 0.0001$) were observed in participants treated with the abiraterone acetate→ enzalutamide sequence, with a trend towards improvement in OS (28.8 vs. 24.7 months, HR 0.79, 95% CI 0.54–1.16, $p = 0.23$). However, there were no benefits seen with the enzalutamide → abiraterone acetate sequence. Another systematic review of various clinical trials evaluating abiraterone acetate and enzalutamide sequencing arrived at a similar conclusion (Chung et al, 2019).

A single-arm phase IV study (n=214;145 were chemo naïve) evaluated the efficacy and safety of enzalutamide in participants with mCRPC who progressed after ≥24 wk of abiraterone acetate plus prednisone treatment. The study demonstrated antitumor activity in some participants with mCRPC who had previously progressed following at least 24 weeks

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of abiraterone acetate treatment, also suggesting an important degree of cross-resistance between the two agents (de Bono et al, 2018).

4.2.2.2. Docetaxel

The role of chemotherapy in mCRPC was established in 2 Phase 3 randomized landmark trials. In the TAX-327 trial (Tannock et al, 2004), in a population of 1006 participants with mCRPC, docetaxel (75 mg/m^2 every 3 weeks) combined with prednisone significantly increased OS as compared with mitoxantrone plus prednisone (HR 0.76; 95% CI 0.62,0.94). Similarly, the SWOG-9916 trial (Petrylak et al, 2004) showed that the combination of docetaxel (60 mg/m^2 every 3 weeks), estramustine and prednisone was superior to mitoxantrone plus prednisone in prolonging OS (HR 0.8; 95% CI 0.67,0.97). In both studies, docetaxel increased the risk of myelosuppression, febrile neutropenia, fatigue, alopecia, diarrhea, neuropathy, and peripheral edema. Data from the studies led to the approval of docetaxel as the first line chemotherapy for mCRPC by the US FDA in 2004 and EMA in 2005. Updated data from the TAX-327 trial showed that the median overall survival time was 19.2 months in the docetaxel plus prednisone arm and 16.3 months in the mitoxantrone plus prednisone arm (Hazard ratio = 0.79, 95% confidence interval = 0.67 to 0.93, P = 0.004) (Berthold et al, 2008). Docetaxel is a recommended treatment for mCRPC and therefore is an appropriate comparator for the C2321014 study (Parker et al, 2020).

4.2.3. Choice of Contraception/Barrier Requirements

Docetaxel has been shown to be genotoxic by an aneugenic mechanism in the in vitro micronucleus and chromosome aberration test in CHO-K1 cells and in the in vivo micronucleus test in the mouse. However, it did not induce mutagenicity in the Ames test or the CHO/HGPRT gene mutation assay. These results are consistent with the pharmacological activity of docetaxel. Undesirable effects on the testis observed in rodent toxicity studies suggest that docetaxel may impair male fertility. Therefore, male participants with female partners of reproductive potential are advised to use effective contraception during treatment and for 4 months after the last dose of docetaxel.

PF-06821497 has not yet been tested for teratogenicity/fetotoxicity. However, developmental and reproductive toxicities are common for oncology drugs. The target EZH2 is abundantly expressed during embryonic development, and knockout of EZH2 causes embryonic lethality in mice.

An embryo-fetal toxicity study in mice (study 9785-TX-0009) showed that enzalutamide induced premature deliveries in dams and dose-dependent increases in the incidence of decreased anogenital distance and skeletal abnormalities, such as cleft palate associated with absent palatine bone. These effects are assumed to be related to the pharmacology of enzalutamide because they were previously reported in rats treated with bicalutamide (Iswaran et al, 1997), as well as mice and rabbits treated with chlormadinone acetate (Takano et al, 1966). Based on the embryofetal toxicity study and the known reproductive and developmental toxicity of other antiandrogens, it is assumed that enzalutamide may cause fetal harm and is thus contraindicated in pregnant women.

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Based on information and findings for teratogenicity/fetotoxicity all participants receiving PF-06821497/enzalutamide who are, in the opinion of the investigator, sexually active with female partners of reproductive potential must agree to use highly effective contraception throughout the study and for up to 3 months after the last dose of medication (See Appendix 4).

4.3. Justification for Dose

Extensive dose exploration has been conducted for PF-06821497 in combination with enzalutamide within a Phase 1 dose escalation (Part 2A) portion of Study C2321001 in participants with CRPC.

Eight PF-06821497 dose levels, administered on empty stomach, were evaluated in combination with the approved dose of enzalutamide 160 mg QD: 150 mg BID (n=4), 250 mg BID (n=3), 375 mg BID (n=3), 500 mg (n=14), 625 mg (n=3), 750 mg (n=4), 875 mg BID (n=6), 1250 mg BID (n=10). This combination of PF-06821497 and enzalutamide was well tolerated across the evaluated dose range, with 1 DLT of Grade 3 weakness observed in a participant dosed at PF-06821497 1250 mg BID dose administered on an empty stomach.

There is an approximately 60% reduction in PF-06821497 steady-state AUCtau when given in combination with enzalutamide as compared to PF-06821497 monotherapy due to the known strong CYP3A mediated induction effect of enzalutamide. No MTD has been determined for either monotherapy or combination regimens. Based on the totality of the safety, anti-tumor activity, PK and PD data from study Part 2A dose escalation in combination with enzalutamide in participants with CRPC, PF-06821497 1250 mg BID administered on an empty stomach was selected as the recommended dose for expansion to further characterize the safety and efficacy of the combination in Part 2B of the study C2321001.

Exploratory exposure-response analyses for key efficacy endpoints indicated that higher PF-06821497 plasma exposures are associated with increased rPFS as well as increased probability of PSA50 response. In addition, as an EZH2 target engagement pharmacodynamic marker, intra-tumoral H3K27Me3 reduction was seen to be dose dependent within the range of 500 mg BID – 1250 mg BID in combination with enzalutamide, with 1250 mg BID on empty stomach providing maximum inhibition. In contrast, no exposure-response relationships have been identified for the safety endpoints of clinical interest, with the exception of any grade diarrhea.

In summary, current data from the Phase 1 study C2321001 indicates that plasma exposures associated with the 1250 mg BID dose administered on empty stomach represent an efficacious and well tolerated exposure for combination with enzalutamide in CRPC.

Preliminary safety data from PF-06821497 administration with food indicated a trend toward lower incidence of diarrhea as compared to administration on empty stomach conditions. Therefore, an exposure-matched dose of PF-06821497 under fed conditions that will provide equivalent plasma exposures as 1250 mg BID on empty stomach was evaluated. Based on available data, the 875 mg BID dose administered with food results in similar PF-06821497 plasma exposures as that of 1250 mg on empty stomach. This supports PF-06821497 875 mg BID administered with food as the recommended dose for the further clinical development of

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PF-06821497 in combination with enzalutamide. Therefore, PF-06821497 875 mg BID with food in combination with enzalutamide is to be evaluated in this study.

4.4. End of Study Definition

The end of the study globally is defined as the date of the LPLV.

A participant is considered to have completed the study if they have completed all phases of the study, including long-term follow-up, or until criteria for study discontinuation have been met.

5. STUDY POPULATION

This study can fulfill its objectives only if appropriate participants are enrolled, including participants across diverse and representative racial and ethnic backgrounds. If a prescreening tool is utilized for study recruitment purposes it will include collection of information that reflects the enrollment of a diverse participant population including, where permitted under local regulations, age, sex, and race, and ethnicity. The following eligibility criteria are designed to select participants for whom participation in the study is considered appropriate. All relevant medical and non-medical conditions should be taken into consideration when deciding whether a particular participant is suitable for this protocol.

Prospective approval of protocol deviations to recruitment and enrollment criteria, also known as protocol waivers or exemptions, is not permitted.

5.1. Inclusion Criteria

Participants are eligible to be included in this study only if all of the following criteria apply:

Age and Sex:

1. Male Participants aged ≥18 years (or the minimum age of consent in accordance with local regulations) at screening.
    a. Refer to Appendix 4 for reproductive criteria for male participants (Section 10.4.1).

Disease Characteristics:

2. Histologically or cytologically confirmed adenocarcinoma of the prostate without small cell features (neuroendocrine differentiation and other histologic components are permitted if adenocarcinoma is the primary histology). For participants without a prior histological diagnosis, a baseline de novo biopsy must be used to confirm the diagnosis.

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3. Metastatic disease in bone documented on bone scan, or in soft tissue documented on CT/MRI scan.

    a. Measurable soft tissue disease (per RECIST v1.1) is required if there is not an evaluable bone lesion (per PCWG3 criteria).

    b. For participants with measurable soft tissue disease only, regional lymph node disease (ie below the aortic bifurcation) alone does not qualify the participant for the study. PET and SPECT are not evaluable imaging modalities for this study and will not qualify a participant for the study.

    c. A bone scan showing an intense symmetric activity in the bones (referred to as a superscan) is not considered evaluable, therefore deeming the participant to be ineligible.

4. Surgically or medically castrated, with serum testosterone ≤50 ng/dL (≤1.73 nmol/L) at screening.

    a. Ongoing androgen deprivation therapy with a GnRH agonist or antagonist for participants who have not undergone bilateral orchiectomy must have been initiated at least 4 weeks before randomization and must continue throughout the study.

    b. Participants who are not orchiectomized will also continue to receive a GnRH (gonadotropin releasing hormone) agonist or antagonist on study.

5. Progressive disease in the setting of surgical or medical castration as defined by 1 or more of the following 3 criteria:

    a. Prostate Specific antigen (PSA) progression defined as a minimum of two rising PSA levels with an interval of ≥1 week between each determination within the last 12 months. If the third is not greater than the second measure, a fourth PSA measure is required to be taken and be greater than the second measure. The last of these confirmatory PSA values must be taken within 4 weeks prior to randomization. The PSA value at the Screening visit must be ≥1 ng/mL if confirmed rise in PSA is the only indication of progression per PCWG3 criteria;

    b. Soft tissue disease progression as defined by RECIST v1.1.

    c. Bone disease progression defined by PCWG3 with 2 or more new metastatic bone lesions on a whole-body radionuclide bone scan.

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d. Evidence of disease progression on treatment with at least 12 weeks of abiraterone acetate in the mCSPC setting or first line mCRPC setting is required. In the non-metastatic setting, evidence of disease progression during treatment (for at least 12 weeks) or within 3 months of treatment completion. In first line mCRPC, prior treatment with abiraterone acetate (for at least 12 weeks) in conjunction with olaparib or niraparib is permissible.

e. Prior treatment with PARP monotherapy for BRCAm/HRRm gene mutated mCRPC following cancer progression on abiraterone acetate is not permissible.

6. Resolution of acute effects of any prior therapy to either baseline severity or CTCAE Grade ≤ 1 (except for AEs such as alopecia and peripheral neuropathy not constituting a safety risk in the investigator's judgment).

Other Inclusion criteria:

7. ECOG performance status 0 - 2, with life expectancy of at least 6 months as assessed by the investigator.

5.2. Exclusion Criteria

Participants are excluded from the study if any of the following criteria apply:

Medical Conditions:

1. Any medical (including active or clinically significant bacterial, fungal or viral infection) or psychiatric condition including recent (within the past year) or active suicidal ideation in the past year or suicidal behavior in the past 5 years or laboratory abnormality that may increase the risk of study participation or, in the investigator's judgment, make the participant inappropriate for the study.

   a. HIV/HBV/HCV testing is not required unless mandated by local health authority.

   b. Participants with known human immunodeficiency virus (HIV) or acquired immunodeficiency syndrome (AIDS)-related illness, or active hepatitis B or C are excluded.

   c. Participants with a known history of chronic liver diseases including alcoholic liver disease, primary biliary cirrhosis, primary sclerosing cholangitis, autoimmune hepatitis, Wilson's disease, hemochromatosis, alpha-1 antitrypsin deficiency, or other chronic liver disease are excluded.

   d. Participants with a known history of active inflammatory gastrointestinal disease, chronic diarrhea, or previous gastric resection or lap-band surgery are excluded.

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2. Clinically significant cardiovascular disease defined as:

    a. Any of the following in the previous 6 months: myocardial infarction, severe/unstable angina, coronary/peripheral artery bypass graft, symptomatic congestive heart failure (New York Heart Association Class III or IV), cerebrovascular accident, or symptomatic pulmonary embolism or other clinically significant episode of thromboembolic disease, congenital long QT syndrome, Torsade de Pointes, clinically important arrhythmias, left anterior hemiblock, bifascicular block, ongoing cardiac dysrhythmias of NCI CTCAE Grade ≥2.

    b. Cardiac rhythm device/pacemaker.

    c. QTcF >480 msec on screening ECG.

3. CNS pathology/neurological findings:

    a. Known or suspected brain metastasis or active leptomeningeal disease.

    b. Symptomatic or impending spinal cord compression or cauda equina syndrome

    c. Participants with epidural disease, canal disease and prior cord involvement **are NOT excluded** if those areas have been treated, are stable, and not neurologically impaired.

    d. Clinically significant history of seizure or any condition that may predispose to seizure (eg, prior cortical stroke, significant brain trauma). Also, history of unexplained loss of consciousness or transient ischemic attack within 12 months of randomization.

4. Any history of myelodysplastic syndrome, acute myeloid leukemia, or any other prior malignancy except for any of the following:

    a. Carcinoma in situ or non-melanoma skin cancer.

    b. Any prior malignancies ≥3 years before randomization with no subsequent evidence of recurrence or progression regardless of the stage.

    c. Stage 0 or Stage 1 cancer <3 years before randomization that has a remote probability of recurrence or progression in the opinion of the investigator.

Prior/Concomitant Therapy:

5. Prior treatment for prostate cancer at any stage with any cytotoxic chemotherapy, radioligand therapy (ie, ^{177}Lu-PSMA-617, radium-223), ARSi (including enzalutamide, apalutamide, darolutamide), PARP monotherapy or other systemic anti-cancer treatment (approved drugs or experimental compounds such as, antibody

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therapy, immunotherapy, gene therapy, angiogenesis inhibitors, CDK4/6 inhibitors, 5-alpha reductase inhibitors, EZH2 inhibitors) with the following exceptions:

a. Treatment with first-generation antiandrogen agents (eg, bicalutamide, nilutamide, and flutamide), estrogens, progestins, cyproterone acetate, but must be discontinued prior to the first dose of study medication.

b. Docetaxel treatment is allowed for mCSPC, as long as no signs of failure, or disease progression occurred during treatment or within 3 months of treatment completion.

c. Current use of 5-alpha reductase inhibitors is prohibited within 28 days prior to randomization.

6. Previous administration with an investigational product (drug or vaccine which does not meet exclusion criterion 5 above) within 30 days (or as determined by the local requirement) or 5 half-lives preceding the first dose of study intervention used in this study (whichever is longer).

7. Current use or anticipated need for drugs that are known strong CYP3A4/5 inhibitors and inducers (with exception of enzalutamide as part of this study) outlined in Sections 6.9.1 and 6.9.2, including their administration within 10 days or 5 half-lives, whichever is longer prior to randomization.

8. Major surgery or palliative localized radiation therapy within 14 days before randomization.

Diagnostic Assessments:

9. Inadequate renal function defined by an eGFR <45 mL/min/1.73 m^2. Based upon participant age at screening, eGFR is calculated using the recommended formulas in Section 10.7.1 to determine eligibility and to provide a baseline to quantify any subsequent kidney safety events.

   For eligibility assessment based upon estimated renal function, the higher of the screening and baseline eGFR values may be used.

10. Hepatic dysfunction defined as:

    a. Total bilirubin ≥1.5 × ULN

    b. AST >.2.5 × ULN

    c. ALT >.2.5 × ULN

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11. Hematologic abnormalities defined as:
    - ANC <1500/mm^3
    - Platelets <100,000/µL
    - Hemoglobin <9 g/dL, independent of transfusion within 14 days of randomization

Other Exclusion Criteria:

12. Investigator site staff directly involved in the conduct of the study and their family members, site staff otherwise supervised by the investigator, and sponsor and sponsor delegate employees directly involved in the conduct of the study and their family members.

5.3. Lifestyle Considerations

5.3.1. Contraception

The investigator or their designee, in consultation with the participant, will confirm that the participant is using an appropriate method of contraception for the individual participant and their partner(s) from the permitted list of contraception methods (See Section 10.4.1) and will confirm that the participant has been instructed in its consistent and correct use. The investigator or designee will advise the participant to seek advice about the donation and cryopreservation of germ cells prior to the start of study intervention, if applicable.

At time points indicated in the SoA, Table 1 and Table 2, the investigator or designee will inform the participant of the need to use highly effective contraception consistently and correctly and document the conversation and the participant's affirmation in the participant's chart. Participants need to affirm their consistent and correct use of at least 1 of the selected methods of contraception, considering that their risk for pregnancy may have changed since the last visit.

In addition, the investigator or designee will instruct the participant to call immediately if the selected contraception method is discontinued and document the requirement to use an alternate protocol-specified method, including if the participant will no longer use abstinence as the selected contraception method, or if pregnancy is known or suspected in the participant or partner.

If the participant is capable of ejaculating, instruct the participant to use a condom to avoid partner/fetal exposure and instruct the participant not to donate sperm for between 3 – 4 months after last dose of study intervention depending on treatment assignment (3 months for PF-06821497/enzalutamide and 4 months for docetaxel) (See Section 10.4.1). For these participants the investigator is to confirm correct contraception use or note changes in contraception use in the participant's medical record.

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5.3.2. Meals and Dietary Restrictions

Participants must be instructed to take PF-06821497 with food (ie, a meal). Participants should be instructed to take their medication at approximately the same time each day (approximately every 12 hours) and not to take more than the prescribed dose at any time.

5.4. Screen Failures

Screen failures are defined as participants who consent to participate in the clinical study but are not subsequently enrolled in the study. A minimal set of screen failure information is required to ensure transparent reporting of screen failure participants to meet the CONSORT publishing requirements and to respond to queries from regulatory authorities. Minimal information includes demography, screen failure details, and any SAE.

Individuals who do not meet the criteria for participation in this study (screen failure) may be rescreened only once. Rescreened participants will screen fail the original SSID and then will be entered as a new participant and will receive a new SSID. When rescreening, if earlier assessments are within 28 days of randomization, the assessments will not need to be repeated.

Note: Any participant at Screening who tested positive or had a known COVID-19/SARS-CoV2 active infection who screen failed is allowed to rescreen.

6. STUDY INTERVENTION(S) AND CONCOMITANT THERAPY

Study interventions are all prespecified investigational and non-investigational medicinal products/auxiliary medicinal products, medical devices, and other interventions (eg, surgical and behavioral) intended to be administered to the study participants during the study conduct.

For the purposes of this protocol, study intervention refers to those summarized in Section 6.1 and Table 3.

6.1. Study Intervention(s) Administered

Table 3. Study Intervention(s) Administered

Intervention Name	**PF-06821497**	**Enzalutamide**	**Docetaxel**	**Prednisone, Prednisolone**	**Dexa-methasone**
Type	Drug	Drug	Drug	Drug	Drug
Use	Experimental	Active comparator	Active Comparator	Required in combination with docetaxel	Required premedication
IMP or NIMP/AxMP	IMP	IMP	IMP	NIMP/AxMP	NIMP/AxMP
Dose Formulation	Tablet	Capsule	Solution for injection in single use vials	Various	Various

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Table 3. Study Intervention(s) Administered

Intervention Name	PF-06821497	Enzalutamide	Docetaxel	Prednisone, Prednisolone	Dexa-methasone
Unit Dose Strength(s)	125 mg, 250 mg	40 mg	Various 10 mg/mL, 20 mg/mL	Prednisone 5 mg or Prednisolone 5 mg	Dexamethasone 4 mg or 8 mg
Dosage Level(s)	875 mg BID (twice daily) Refer to Section 6.1.1.1	Dose of enzalutamide is 160 mg given once daily given in 4 capsules (40 mg each) by mouth.	Docetaxel 75 mg/m^2 by intravenous (IV) infusion given over one hour on Day 1, repeated every 21 days for 10 cycles.	Prednisone: 5 mg BID for 21 days or Prednisolone 5 mg BID for 21 days. Can be given per standard of care.	Dexamethasone 8 mg (or two 4 mg tablets) by mouth 12 hours, 3 hours, and 1 hour before the docetaxel infusion, or by IV infusion. Dexamethasone can be given per standard of care.
Route of Administration	Oral	Oral	IV	Oral	Oral /IV
Sourcing	Provided centrally by the sponsor.	Provided centrally by the sponsor.	Provided centrally by the sponsor.	Sourced either locally or centrally.	Sourced either locally or centrally.
Packaging and Labeling	Study intervention (open label) will be provided in bottles. Each bottle will be labeled as required per country requirement.	Study intervention (open label) will be provided in bottles. Each bottle will be labeled as required per country requirement.	Study intervention (open label) will be provided in vials. Each vial will be labeled as required per country requirement.	Locally obtained supplies (open label) will be used in accordance with local regulations and the package insert, supplied in separate bottles and labelled according to local regulatory requirements	Locally obtained supplies (open label) will be used in accordance with local regulations and the package insert, supplied in separate bottles and labelled according to local regulatory requirements
SRSD	IB	IB	EU SmPC	Irish SmPC for each of these medications	UK SmPC

Table 4. Study Arm(s)

Study Arm(s)		
Arm Title	**Arm A**	**Arm B**
Arm Description	Participants will receive PF-06821497 875 mg BID (2 times a day) + enzalutamide 160 mg QD. Cycles are 28 days, and treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, lost to follow-up, or study termination.	Physician's choice: enzalutamide: Participants will receive enzalutamide 160 mg QD. Cycles are 28 days; and treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, lost to follow-up, or study termination. Physician's choice docetaxel: Participants will receive docetaxel at a dose of 75 mg/m^2 IV every 21 days, for up to a maximum of 10 cycles. Docetaxel is given in combination with prednisone or prednisolone, 5 mg BID for 21 days, and premedication of oral dexamethasone 8 mg at 12 hours, 3 hours, and 1 hour (or IV per standard of care) before the docetaxel infusion. Treatment will be administered until confirmed disease progression, unacceptable toxicity, withdrawal of consent, or lost to follow-up or study termination.

Participants who are not orchiectomized will also continue to receive a GnRH (gonadotropin releasing hormone) agonist or antagonist on study.

6.1.1. Administration of Study Interventions

Administration of study interventions will be performed at the site by an appropriately qualified and trained member of the study staff as allowed by local, state, and institutional guidance.

6.1.1.1. PF-06821497

PF-06821497 will be self-administered by mouth outside the clinic, except on days of site visits; participants will withhold their dose until after assessments are complete per SoA. On these days, study medications should be taken in the clinic under the supervision of the study site personnel.

PF-06821497 will be provided in bottles containing 125 mg or 250 mg tablets appropriate to the dose level the participant will receive. Based on regional availability of tablet strengths, participants may be provided with a lower strength tablet for the same dose level in comparison to different regions. The participant will be dispensed a sufficient quantity of bottles/tablets for 28 days of continuous dosing, plus additional tablets to cover potential delays in visiting the site. Site personnel must ensure that participants clearly understand the directions in the dosing diary. PF-06821497 will be dispensed at the beginning of each treatment cycle (or as otherwise indicated). Study participants should be instructed to keep their medication in the bottles provided and not transfer the tablets to any other container.

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PF-06821497 will be administered with food (ie, a meal) orally BID on a continuous basis without adjustment for body size at every cycle.

Participants will swallow the tablets whole and will not manipulate or chew the tablets prior to swallowing. Participants are to be instructed to take their medication at approximately the same time each day (approximately every 12 hours) and to not take more than the prescribed dose at any time.

If a participant misses a morning or evening dose by more than 6 hours, they must be instructed not to "make it up" but to resume subsequent doses at the next scheduled dose as prescribed. In addition, if a participant vomits any time after taking a dose, they must be instructed not to "make it up" but to resume subsequent doses the next scheduled dose as prescribed.

If a participant inadvertently takes 1 extra dose during a day, the participant should not take the next dose of PF-06821497.

6.1.1.2. Enzalutamide

Enzalutamide is provided in 40 mg soft gel capsules in bottles. (Local variations may apply, please see the IP manual for more information).

The study participant will be dispensed a sufficient quantity of bottles/capsules for 28 days of continuous dosing, plus additional capsules to cover potential delays in visiting the site. Site personnel must ensure that participants clearly understand the directions in the dosing diary. Enzalutamide will be dispensed at the beginning of each treatment cycle (or as otherwise indicated). Study participants should be instructed to keep their medication in the bottles provided and not transfer the capsules to any other container.

Enzalutamide will be administered orally with or without food, and without adjustment for body size at every cycle.

Enzalutamide 160 mg will be administered QD on a continuous basis. Participants will swallow the capsules whole and will not manipulate or chew the capsules prior to swallowing. Participants are to be instructed to take their medication at approximately the same time each day and to not take more than the prescribed dose at any time.

If a participant misses a day of treatment, they must be instructed not to "make it up" but to resume subsequent doses the next day as prescribed.

If a participant vomits any time after taking a dose, they must be instructed not to "make it up" but to resume subsequent doses the next day as prescribed.

If a participant inadvertently takes 1 extra dose during a day, the participant should not take the next dose of enzalutamide.

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6.1.1.3. Docetaxel

Docetaxel will be administered at a dose of 75 mg/m^2 intravenously (IV) every 21 days, for up to a maximum of 10 cycles on site by study personnel (whereupon no additional study intervention shall be given, but assessments continue). Docetaxel is given in combination with prednisone or prednisolone 5 mg BID for 21 days.

Oral or intravenous dexamethasone 8 mg will be given at 12 hours, 3 hours, and 1 hour before the docetaxel infusion as a premedication or per local practice / standard of care guidelines.

Details for preparation of docetaxel infusion are provided in the current Investigational Product Manual. All participants should be weighed within 72 hours prior to dosing for every cycle to ensure they did not experience either a weight loss or gain >10% from the prior weight used to calculate the amount of docetaxel required for dose preparation. The decision to recalculate docetaxel dose based on the weight obtained at each cycle can be in accordance with institutional practice; however, if the participant experienced either a weight loss or gain >10% compared to the weight used to calculate the prior dose, the amount of docetaxel required for preparation and administration for the current cycle must be recalculated using this most recent weight obtained.

Administration of docetaxel will be performed at the site by an appropriately qualified and trained member of the study staff as allowed by local, state, and institutional guidance.

Following administration of docetaxel at the site, participants will be observed for 30 minutes or per local and institutional guidelines by an appropriately qualified and trained member of the study staff. Appropriate medication and other supportive measures for management of a medical emergency will be available in accordance with local guidelines and institutional guidelines.

6.2. Preparation, Handling, Storage and Accountability

1. The investigator or qualified designee must confirm that appropriate conditions (eg, temperature) have been maintained during transit for all study interventions received and any discrepancies are reported and resolved before use of the study intervention.

2. Only participants enrolled in the study may receive study intervention and only authorized site staff may supply, prepare, and/or administer study intervention.

3. All study interventions must be stored in a secure, environmentally controlled, and monitored (manual or automated recording) area in accordance with the labeled storage conditions with access limited to the investigator and authorized site staff. At a minimum, daily minimum and maximum temperatures for all site storage locations must be documented and available upon request. Data for nonworking days must indicate the minimum and maximum temperatures since previously documented upon return to business.

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4. Any excursions from the study intervention label storage conditions should be reported to Pfizer upon discovery along with actions taken. The site should actively pursue options for returning the study intervention to labeled storage conditions as soon as possible. Once an excursion is identified, the study intervention must be quarantined and not used until Pfizer provides permission to use the study intervention. Specific details regarding the excursion definition and information to report for each excursion will be provided to the site in the IPM.

5. Any storage conditions stated in the SRSD will be superseded by the storage conditions stated on the label. Site staff will instruct participants on the proper storage requirements for take-home study intervention. See the IPM, package insert, or equivalent for storage conditions of the study intervention once reconstituted and/or diluted.

6. Study interventions should be stored in their original containers.

7. The investigator, institution, head of the medical institution, (where applicable) or authorized site staff is responsible for study intervention accountability, reconciliation, and record maintenance (ie, receipt, reconciliation, and final disposition records), such as the IPAL or sponsor-approved equivalent. All study interventions will be accounted for using a study intervention accountability form/record. All PF-06821497 and enzalutamide that is taken home by the participant, both used and unused, must be returned to the investigator by the participant. Returned study intervention must not be re-dispensed to the participants.

8. Further guidance and information for the final disposition of unused study interventions are provided in the IPM. All destruction must be adequately documented. If destruction is authorized to take place at the investigator site, the investigator must ensure that the materials are destroyed in compliance with applicable environmental regulations, institutional policy, and any special instructions provided by Pfizer.

Upon identification of a product complaint, notify the sponsor within 1 business day of discovery as described in the IPM.

6.2.1. Preparation and Dispensing

See the IPM, local prescribing information, or equivalent for instructions on how to prepare the study intervention for administration. Study intervention should be prepared and dispensed by an appropriately qualified and experienced member of the study staff (eg, physician, nurse, physician's assistant, nurse practitioner, pharmacy assistant/technician, or pharmacist) as allowed by local, state, and institutional guidance. A second staff member will verify the dispensing.

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6.2.1.1. Arm A (PF-06821497 and Enzalutamide)

A qualified staff member will dispense the PF-06821497 and enzalutamide bottles using unique container numbers assigned via an IRT system according to the SoA. The participant/caregiver should be instructed to maintain the product according to the instructions provided in the participant dosing card throughout the course of dosing. Participants are to keep the investigational product away from children and return the investigational product to the site at the next study visit. Women who are or may become pregnant should not handle damaged or opened enzalutamide capsules without protection, eg, gloves.

6.2.1.2. Arm B (Physician's choice of enzalutamide or docetaxel)

A qualified staff member will dispense the enzalutamide and docetaxel using unique container numbers assigned via an IRT system according to the SoA, Table 1 and Table 2. A second staff member will verify the dispensing.

See the IPM, package insert, or equivalent for instructions on how to prepare the docetaxel for administration. Docetaxel is to be prepared and dispensed by an appropriately qualified and experienced member of the study staff (eg, physician, nurse, physician's assistant, nurse practitioner, pharmacy assistant/technician, or pharmacist) as allowed by local, state, and institutional guidance. Only qualified personnel who are familiar with procedures that minimize undue exposure to themselves and to the environment should undertake the preparation, handling, and safe disposal of chemotherapeutic, hazardous agents.

The participant/caregiver should be instructed to maintain the enzalutamide product according to the instructions provided in the participant dosing card throughout the course of dosing. Participants are to keep the enzalutamide product away from children and return the investigational product to the site at the next study visit. Women who are or may become pregnant should not handle damaged or opened enzalutamide capsules without protection, eg, gloves.

6.3. Assignment to Study Intervention

The study interventions to be dispensed to the participant will be assigned using an IRT system in a 1:1 randomization scheme. The site will utilize the IRT system to assign the DU or container number(s) prior to the start of study intervention administration for each participant. The site will record the study intervention assignment on the applicable CRF, if required. Randomization will be stratified by:

- Previous treatment with docetaxel in the mCSPC setting (Yes vs. No)
- Physicians' choice of comparator (docetaxel vs enzalutamide)
- Presence of hepatic metastases (yes vs. no)

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This is an open-label study. The investigator's knowledge of the treatment assignment must not influence the decision to enroll a particular participant or affect the order in which participants are enrolled. Potential bias will be reduced by use of central randomization and BICR of efficacy data.

Study intervention will be dispensed at the study visits summarized in the SoA, Table 1 and Table 2

The study specific IRT reference manual and IPM will provide the contact information and further details on the use of the IRT system.

6.4. Blinding

This is an open-label study. Participants, investigators and other site staff, and sponsor staff will be unblinded to participants' assigned study intervention. As this is an open label study, the sponsor may conduct reviews of the data during the course of the study for the purpose of safety assessment, facilitating PK/PD modeling, and/or supporting clinical development.

6.5. Study Intervention Compliance

When participants are dosed at the site, the date and time of each dose taken by the participant in the clinic will be recorded in the source documents and recorded in the CRF. The dose of study intervention and study participant identification will be confirmed at the time of dosing by a member of the study site staff.

The site will complete the required dosage Preparation Record (docetaxel) located in the IPM. The use of the Preparation Record is preferred, but it does not preclude the use of an existing appropriate clinical site documentation system. The existing clinical site's documentation system should capture all pertinent/required information on the preparation and administration of the dose. This may be used in place of the Preparation Record after approval from the sponsor and/or designee.

When participants self-administer study intervention(s) at home, compliance with study intervention will be assessed at each visit. Compliance will be assessed by direct questioning, counting returned tablets/capsules, etc. during the site visits and documented in the source documents and patient diaries. Deviation(s) from the prescribed dosage regimen should be recorded in the CRF.

A record of the number of PF-06821497 tablets and enzalutamide capsules dispensed to and taken by each participant must be maintained and reconciled with study intervention and compliance records. Intervention start and stop dates, including dates for intervention delays and/or dose reductions, will also be recorded in the CRF.

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6.6. Dose Modification

6.6.1. PF-06821497 Dose Modifications

Every effort should be made to administer investigational product on the planned dose and schedule.

Participants who require a treatment interruption >4 weeks should discontinue the study drug thought to be responsible for the treatment interruption unless agreed with the investigator and sponsor medical monitor.

In the event of significant toxicity, dosing may be delayed and/or reduced as described below. In the event of multiple toxicities, dose modification should be based on the worst toxicity observed and the relatedness to the offending agent. Participants are to be instructed to notify investigators at the first occurrence of any adverse symptom.

Dose modifications may occur in one of three ways:

- Within a cycle: dosing interruption until adequate recovery and dose reduction, if required, during a given treatment cycle;
- Between cycles: dosing administration may be delayed due to persisting toxicity when a new cycle is scheduled to begin.
- In the next cycle: dose reduction may be required in a subsequent cycle based on toxicity experienced in the previous cycle. Every effort should be made to administer investigational product on the planned dose and schedule.

If PF-06821497 is dose reduced, dose-modified or discontinued, enzalutamide dose can remain at the same dose provided the investigator has determined that the cause of AE is due to PF-06821497 only. If relationship to enzalutamide cannot be excluded, please refer to Section 6.6.2 "Enzalutamide Dose Modifications" for additional guidance.

Table 5. PF-06821497 Dose Modification Levels for Adverse Reactions

Dose Level	Dose	Administration of Dose
Starting Dose	875 mg	Three 250 mg tablets and one 125 mg tablet, BID
First Dose Reduction	750 mg	Three 250 mg tablets, BID
Second Dose Reduction	625 mg	Two 250 mg tablets and one 125 mg tablet, BID

6.6.2. Enzalutamide Dose Modifications

Dose modifications and reductions are to occur per the local prescribing information and institutional practice. Toxicities will be managed as defined in the local prescribing information or manufacturer's prescribing information and institutional practice.

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Dose modification of enzalutamide could have an impact on PF-06821497 plasma exposure therefore, the PF-06821497 dose will need to be modified as described below.

If enzalutamide dose is held or dose-modified for a short time period (less than or equal to 1 week), the PF-06821497 dose can remain at the same dose provided the investigator has determined that the cause of the AE is due to enzalutamide only. If enzalutamide is held for more than 1 week, PF-06821497 should be dose reduced to the next lower dose level, based on the current PF-06821497 dosage (and would remain reduced).

Enzalutamide discontinuation:
If enzalutamide is permanently discontinued, PF-06821497 administration will also be discontinued.

6.6.3. Docetaxel Dose Modifications

Dose modifications and reductions are to occur per the local prescribing information and institutional practice. Toxicities will be managed as defined in the local prescribing information or manufacturer's prescribing information and institutional practice.

6.6.4. Modifications Due to Abnormal Liver Tests

PF-06821497 is to be withheld if liver enzymes abnormalities are reported as per guidance in Table 6.

Participants receiving PF-06821497, enzalutamide or docetaxel who have abnormal liver tests or meet the criteria for permanent discontinuation or temporary withholding of study intervention will be followed up according to the recommendations in this section and Table 11. All cases meeting the criteria for an AE or SAE should be reported as per the criteria and timeframe detailed in Appendix 3. Potential Hy's law cases (refer to Appendix 6) or cases resulting in permanent study intervention discontinuation are to be reported as SAEs.

Participants who develop abnormal liver tests (AST, ALT, T bili), abnormal INR values, or signs or symptoms consistent with hepatitis during study intervention may meet the criteria for temporarily withholding or permanently discontinuing (Section 7.1.5) study intervention.

When study intervention is temporarily withheld or permanently discontinued due to a potential drug induced liver injury, a period of close observation is to commence until the liver test abnormalities return to baseline or normal values. The evaluations listed in Table 11 should be performed.

Rechallenge with enzalutamide may only be considered according to local label.

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6.6.5. Dose Reductions for PF-06821497

Following dosing interruption due to toxicity, the PF-06821497 dose may need to be reduced when treatment is resumed.

In general, no specific dose adjustments are recommended for Grade 1 or 2 treatment related toxicity except diarrhea. However, investigators should always manage their participants according to their medical judgment based on the particular clinical circumstances.

Dose reduction of PF-06821497 by 1 and, if needed, 2 dose levels (See Table 5) will be allowed depending on the type and severity of toxicity encountered. Participants requiring more than 2 dose reductions will be discontinued from the treatment and entered into the Follow-up- Phase, unless otherwise agreed between the investigator and the sponsor. All dose modifications/adjustments must be clearly documented in the participant's source notes and CRF.

Once a dose has been reduced for a given participant, all subsequent cycles should be administered at that dose level, unless further dose reduction is required. Intraparticipant dose re-escalation is not allowed.

Table 6. Dose Modifications for PF-06821497 – Related Toxicity

Toxicity	Grade 1	Grade 2	Grade 3	Grade 4
Serum bilirubin*	Continue at the same dose level.	Continue at the same dose level.	When serum bilirubin >3 x ULN: • Hold administration until recovery to Grade ≤1 (total serum bilirubin ≤1.5 x ULN) or baseline and reduce by 1 dose level. (only one dose reduction allowed). • If toxicity reoccurs despite dose reduction, permanently discontinue treatment.	Discontinue treatment.

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Table 6. Dose Modifications for PF-06821497 – Related Toxicity

Toxicity	Grade 1	Grade 2	Grade 3	Grade 4
Grade ≥3 abnormal AST or ALT levels.			• Withhold dose of PF-06821497 until recovery to baseline or normal levels and dose reduce by one level. • If toxicity recurs despite dose reduction, permanently discontinue. • Rechallenge may be considered if an alternative cause for the abnormal liver tests is discovered and the laboratory abnormalities resolve to normal or baseline values. • Rechallenge is not permitted if these abnormalities are accompanied with signs or symptoms consistent with drug induced liver toxicity or meet permanent discontinuation criteria. • PF-06821497 must be permanently discontinued if any elevation persists for more than 7 days. • Guidelines for follow-up for possible drug induced liver injury and for resuming after the liver test abnormalities resolve to baseline grade are provided in Appendix 6 and Section 6.6.4. • The criteria for permanent discontinuation are provided in Section 7.1.	Discontinue treatment.

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Table 6. Dose Modifications for PF-06821497 – Related Toxicity

Toxicity	Grade 1	Grade 2	Grade 3	Grade 4
Non-hematologic toxicity considered related to PF-06821497, except abnormal liver tests	Continue at the same dose level.	Continue at the same dose level.	• Withhold dose until toxicity is Grade ≤1, (Grade ≤2 if not considered a safety risk for the participant). • Then resume treatment at the same dose level or at the next lower dose level at the discretion of the investigator.	Permanent discontinuation for treatment related Grade 4 adverse events of any duration. (Exclusions include discontinuation for emesis, and clinically insignificant laboratory abnormalities that resolve within two days on optimum treatment, additional exclusions will require agreement between the investigator and sponsor).
Hematologic	Continue at the same dose level.	Continue at the same dose level.	• Withhold dose until toxicity is Grade ≤ 2 or has returned to baseline then resume treatment at the same dose level or reduce by one dose level. • If ≥Grade 3 toxicity recurs after the dose reduction, hold PF-06821497 and implement supportive care per local guidelines. Monitor weekly until toxicity <-Grade 2, then resume at a further reduced dose. • If ≥Grade 3 toxicity persists for >4 weeks without recovery to ≤Grade 2 despite supportive care measures, discontinue PF-06821497 and refer to a hematologist for evaluation.	Same as Grade 3.

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Table 6. Dose Modifications for PF-06821497 – Related Toxicity

Toxicity	Grade 1	Grade 2	Grade 3	Grade 4
Lymphocyte counts decreased	Continue at the same dose level	Continue at the same dose level**	• Withhold dose until toxicity is Grade ≤2 or has returned to baseline within 2 weeks. Resume treatment at the same dose level or 1 dose level lower. • Consider obtaining a CD4 count. If CD4 < 200/µL, consider prophylaxis for pneumocystis jirovecii.	Same as Grade 3.
Diarrhea	Start loperamide (up to 16 mg daily) at first sign of loose stool. Participant to be instructed to notify Investigator	In addition to starting loperamide (up to 16 mg daily) at first sign of loose stool, hold PF-06821497 dosing until it resolves to Grade 1. After resolution to Grade 1, restart at same dose level. If Grade 2 recurs, upon resolution to Grade 1, restart one dose level lower.	• In addition to starting loperamide (up to 16 mg daily) at first sign of loose stool, hold PF-06821497 dosing until it resolves to Grade 1, followed by one dose level reduction. • Consider IV fluids, electrolyte replacement and hospitalization for signs of dehydration	Permanently discontinue PF-06821497.

*If concurrent elevation of AST or ALT ≥ 3 X ULN and total bilirubin ≥2 X ULN occurs, refer for management to the DILI section/Appendix 6.
**For Grade 3 lymphocyte counts (<500/µL), consider obtaining CD4 counts.

Table 7. Dysgeusia Management and Dose Modification Guidance

Initial management or low-grade symptoms	Follow-up or higher-grade symptoms
Dysgeusia (prophylaxis and treatment) • Consider recommending that participants: ○ Take PF-06821497 with abundant water. ○ Minimize the time the tablets stay in contact with the mouth mucosa. ○ Rinse their mouth with water after dosing.	For Grade 2 dysgeusia, consider dose interruption and dose reduction by 1 level if associated with other symptoms such as decreased appetite or weight loss until dysgeusia returns back to baseline and nutritional consult if dysgeusia is associated with weight loss and decreased appetite.

6.6.6. Dosing Interruptions

With respect to study intervention, participants experiencing AEs should have their treatment interrupted as described in PF-06821497 dose modification Table 5: Refer to enzalutamide and docetaxel prescribing information for dose interruption of enzalutamide and docetaxel respectively.

Appropriate follow-up assessments should be done until adequate recovery occurs as assessed by the investigator. Criteria required before treatment can resume are described in the Dose Delays (Section 6.6.7).

Doses may be held up to 4 weeks until toxicity resolution. Depending on when the AE resolved, a treatment interruption may lead to the participant missing all subsequent planned doses within that same cycle.

If the AE that led to the treatment interruption recovers within the same cycle, then re-dosing in that cycle is allowed. Doses omitted for toxicity are not replaced within the same cycle. The need for a dose reduction at the time of treatment resumption should be based on the criteria defined in Table 5 or as applicable, unless expressly agreed otherwise following discussion between the investigator and the sponsor.

In the event of a treatment interruption for reasons other than treatment-related toxicity (eg, elective surgery) lasting >4 weeks), treatment resumption will be decided in consultation with the sponsor.

6.6.7. Dose Delays

Re-treatment following treatment interruption for treatment-related toxicity or at the start of any new cycle may not occur until all the following parameters have been met:

- ANC ≥1,000/mm^3.
- Platelet count ≥50,000/mm^3.
- Grade ≥3 Nonhematologic toxicities have returned to baseline or Grade ≤1 severity (or, at the investigator's discretion, Grade ≤2 if not considered a safety risk for the participant).

If a treatment delay results from worsening of hematologic or biochemical parameters, the frequency of relevant blood tests should be increased as clinically indicated.

If these conditions are met within 4 weeks of treatment interruption, study intervention may be resumed. Refer to Table 5 for recommended dose modification PF-06821497 for AEs requiring dose reduction at the time of treatment resumption. Refer to docetaxel and enzalutamide prescribing information for dose modifications and interruption guidelines.

Refer to the Dose Reductions (Section 6.6.1) for AEs requiring dose reduction at the time of treatment resumption.

If participants require discontinuation of study intervention for more than 4 weeks at any time during the study, then study intervention should be permanently discontinued, unless the investigator's benefit/risk assessment suggests otherwise after discussion with the sponsor.

For docetaxel arm, if a treatment interruption continues beyond Day 21 of the current cycle, then the day when treatment is restarted will be counted as Day 1 of the next cycle.

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6.7. Continued Access to Study Intervention After the End of Study

At the end of study, participants who are still deriving clinical benefit from study intervention may have the option to continue receiving study intervention until it becomes commercially accessible, in accordance with local regulations and requirements.

6.8. Treatment of Overdose

For this study, any dose of PF-06821497, enzalutamide, or docetaxel, greater than the prescribed dose (ie, extra pill[s] or extra full dose) will be considered an overdose. There is no specific treatment for an overdose. Local prescribing information should be consulted in the setting of an overdose.

In the event of overdose of PF-06821497, enzalutamide, or docetaxel, the investigator should:

1. Contact the study medical monitor within 24 hours.
2. Closely monitor the participant for any AEs/SAEs and laboratory abnormalities as medically appropriate and follow-up until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.2.2).
3. Document the quantity of the excess dose as well as the duration of the overdose in the CRF.
4. Overdose is reportable to Pfizer Safety **only when associated with an SAE.**
5. Obtain a blood sample for PK analysis (PF-06821497) within 2 days from the date of the last dose of study intervention if requested by the study medical monitor (determined on a case-by-case basis).

6.9. Prior and Concomitant Therapy

Ongoing androgen deprivation therapy with a GnRH agonist or antagonist for participants who have not undergone bilateral orchiectomy must be initiated at least 4 weeks before randomization and must continue throughout the study (refer to Inclusion Criteria, Section 5.1).

Concomitant treatment considered necessary for the participant's well-being may be given at discretion of the treating HCP.

All concomitant treatments, blood products, as well as nondrug interventions received by participants from screening until the end of treatment visit will be recorded on the CRF.

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6.9.1. Prohibited During the Study

No additional anti-tumor treatment will be permitted while participants are receiving study intervention, including any hormonal therapy (eg, bicalutamide, nilutamide, flutamide, estrogens, progestins, 5-alpha reductase inhibitors, cyproterone acetate), cancer-related surgery or concurrent radiotherapy (excluding palliative radiotherapy). Additionally, treatment for prostate cancer outlined in the exclusion criterion are prohibited during study (refer to Section 5.2).

The Pfizer study team is to be notified of any prohibited medications taken during the study. After consulting with the sponsor, the investigator will make a judgment on the ongoing participation of any participant with prohibited medication use during the study.

This list of drugs prohibited for potential DDI concerns with the IMP may be revised during the course of the study with written notification from sponsor to include or exclude specific drugs or drug categories for various reasons (eg, emerging DDI results for the IMP, availability of new information in literature on the DDI potential of other drugs).

This is not an all-inclusive list. Site staff should consult with the sponsor or designee with any questions regarding potential DDI.

<u>Prohibited medication due to potential DDI with enzalutamide:</u>

- **Strong CYP2C8 inhibitors**- Use of strong CYP2C8 inhibitors should be avoided. If participants must be coadministered a strong CYP2C8 inhibitor, reduce the enzalutamide dose to 80 mg once daily. If co-administration of the strong CYP2C8 inhibitor is discontinued, the enzalutamide dose should be returned to the dose used prior to initiation of the strong CYP2C8 inhibitor.

- **Certain CYP3A4, CYP2C9, or CYP2C19 substrates with narrow therapeutic index:** Enzalutamide is a strong CYP3A4 inducer and a moderate CYP2C9 and CYP2C19 inducer. The coadministration of enzalutamide decreases the concentrations of certain CYP3A4, CYP2C9, or CYP2C19 substrates, which may reduce the efficacy of these substrates. Avoid the coadministration of enzalutamide with certain CYP3A4 (eg, alfentanil, cyclosporine, dihydroergotamine, ergotamine, fentanyl, pimozide, quinidine, sirolimus and tacrolimus), CYP2C9 (eg, phenytoin, warfarin, and acenocoumarol) or CYP2C19 (eg, S-mephenytoin) substrates for which a minimal decrease in concentration may lead to therapeutic failure of the substrate. If the coadministration cannot be avoided, increase the dosage of these substrates in accordance with their Prescribing Information. In cases where active metabolites are formed, there may be increased exposure to the active metabolites. If co-administration with warfarin cannot be avoided, conduct additional INR monitoring.

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- **Direct oral anticoagulants:** Rivaroxaban and apixaban (both are direct oral anticoagulants and are substrates of CYP3A4 and P-gp) should be avoided as enzalutamide might result in reduced plasma exposure which may lead to loss of efficacy or bleeding complications. Alternative allowable approaches include warfarin with additional monitoring as stated above. Dabigatran and edoxaban (direct oral anticoagulants that are substrates for P-gp but not for CYP3A4) are allowable but should be used with caution given the mild clinical effect of enzalutamide on P-gp inhibition; additional monitoring should be conducted as warranted.

Prohibited medication due to potential DDI with PF-06821497:

- **Strong CYP3A4/5 inhibitors-** Since the inhibition of CYP3A4/5 isoenzymes may increase PF-06821497 plasma exposure leading to potential increases in toxicities, the use of known strong inhibitors is not permitted within 10 days or 5 half-lives of the CYP3A4/5 inhibitors, whichever is longer, prior to the first dose of investigational product and should be avoided during study conduct. Strong CYP3A4/5 inhibitors may include grapefruit juice or grapefruit/grapefruit related citrus fruits (eg, Seville oranges, pomelos), ketoconazole, miconazole, itraconazole, voriconazole, posaconazole, clarithromycin, telithromycin, indinavir, saquinavir, ritonavir, nelfinavir, nefazodone, lopinavir, troleandomycin, mibefradil, and conivaptan.

- **Strong CYP3A4/5 inducers**- Since the induction of CYP3A4/5 isoenzymes may decrease PF-06821497 exposure leading to potential decrease in efficacy, the use strong CYP3A4/5 inducers are not permitted within [10 days or 5 half-lives of CYP3A4/5 inducers, whichever is longer] prior to the first dose of investigational product, with the exception of enzalutamide as part of this study. Strong CYP3A4/5 inducers may include phenobarbital, rifampin, phenytoin, carbamazepine, rifabutin, rifapentin, clevidipine, and St. John's Wort.

6.9.2. Permitted with Caution During the Study

- Moderate CYP3A4/5 inhibitors and/or inducers- It is preferred that moderate CYP3A4/5 inhibitors and/or inducers be replaced prior to the first dose of study intervention and during study conduct. Moderate CYP3A4/5 inhibitors may include erythromycin, ciprofloxacin, verapamil, diltiazem, atazanavir, fluconazole, darunavir, delavirdine, amprenavir, fosamprenavir, aprepitant, imatinib, tofisopam, and cimetidine. Moderate CYP3A4/5 inducers may include bosentan, efavirenz, etravirine, modafinil, and nafcillin. If the replacement is not possible, then caution should be exercised with coadministration of PF-06821497 with moderate CYP3A4/5 inhibitors and/or inducers.

  Concomitant use of PF-06821497 and a substrate of the MATE-1, OATP1B1, OATP1B3, or OCT1 transporter may increase the exposure of the substrate. Therefore, caution is warranted for coadministration of PF-06821497 with MATE-1, OATP1B1, OATP1B3, OCT1 substrates (FDA, 2023).

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- **Granulocyte-colony stimulating factors-**See Section 6.9.5**.**
- **Primary prophylaxis of diarrhea, nausea and vomiting**- See Section 6.9.6.
- **Herbal medicine:** use of herbal medicine is not recommended or should be used with caution.

6.9.3. Permitted During the Study

- **Palliative radiotherapy**- Palliative radiotherapy is allowed for symptomatic treatment of bone metastases present at baseline or to control symptoms related to the primary prostate tumor such as lower urinary tract symptoms, hematuria and pain. In view of the current lack of data about the interaction of PF-06821497 with radiotherapy, PF-06821497 treatment should be interrupted during palliative radiotherapy, stopping 5 days before and resuming treatment after recovery to baseline; or a shorter interruption period may be considered for participants after discussion with the Pfizer Medical Monitor or designee.
- Treatment with bisphosphonates or denosumab during the study are permitted.
- **Authorized or approved vaccines** may be provided to participants, including COVID-19 vaccine, per local/institutional guidelines or regulations.

6.9.4. Supportive Care

Palliative and supportive care for disease related symptoms may be administered at the investigator's discretion and according to the specific supportive care product Prescribing Information or the current ASCO guidelines.

Treatment with 4 g or less per day of acetaminophen (eg no more than 1 gm per single dose) is permitted. However, in the case of increased liver function tests, bilirubin, or alkaline phosphatase, discontinuation of acetaminophen/paracetamol use is recommended.

6.9.5. Hematopoietic Growth Factors

Primary use of colony stimulating factors is not permitted but they may be used to treat treatment emergent neutropenia as indicated by the current ASCO guidelines (Smith et al, 2015).

6.9.6. Antidiarrheal, Antiemetic Therapy

Anti-diarrheal therapy:

Participants should receive appropriate supportive care measures as deemed necessary by the Investigator. For diarrhea cases of any grade, consider recommending increased oral intake of fluids and dietary modifications (eg eliminating lactose, dietary supplements with high-osmolar). Rule out alternative etiologies for diarrhea early in the course of the event and in

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recurrent or severe cases. For diarrhea refractory to PF-06821497 dose hold, consider performing a colonoscopy and investigations to rule out infectious causes.

See Section 6.6.1 for dose modifications for PF-06821497 diarrhea related adverse events.

Antiemetic Therapy:

Nausea and vomiting should be treated aggressively, and consideration should be given in subsequent cycles to the administration of prophylactic antiemetic therapy according to standard institutional practice. Participants should be strongly encouraged to maintain liberal oral fluid intake. The choice of prophylactic drug as well as the duration of treatment is up to the investigator assuming there is no known or expected drug-drug interaction and assuming the drug is not included in the Concomitant Treatment(s) section.

6.9.7. Corticosteroids

Chronic systemic corticosteroid use for palliative or supportive purposes is not permitted. Physiologic replacement doses of corticosteroids are permissible (eg, 1.5 mg of dexamethasone or equivalent daily (eg, ≤10 mg prednisone). For participants randomized to Arm B physician's choice – docetaxel, prophylactic administration of corticosteroids will be given as outlined in Table 3. Acute emergency administration corticosteroids, and acute and chronic topical applications, inhaled sprays, eye drops, or local injections of corticosteroids are allowed.

6.9.8. Surgery

Caution is advised on theoretical grounds for any surgical procedures during the study. The appropriate interval of time between surgery and PF-06821497 required to minimize the risk of impaired wound healing and bleeding has not been determined. Stopping PF-06821497 is recommended at least 5 days prior to surgery. Postoperatively, the decision to reinitiate PF-06821497 treatment should be based on a clinical assessment of satisfactory wound healing and recovery from surgery.

6.9.9. Rescue Medicine

There is no rescue therapy to reverse the AEs observed with PF-06821497, enzalutamide, or docetaxel; standard medical supportive care (ie, transfusions, erythropoiesis stimulating agents, G-CSF and GM-CSF for hematological toxicities) must be provided to manage the AEs. Refer to Table 5 for dose modifications.

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7. DISCONTINUATION OF STUDY INTERVENTION AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

7.1. Discontinuation of Study Intervention

It may be necessary for a participant to permanently discontinue study intervention. Reasons for permanent discontinuation of study intervention may include the following:

- BICR determined radiographic progression. However, participants with confirmed disease progression assessed by BICR who are continuing to derive clinical benefit from the study intervention will be eligible to continue receiving study intervention, provided that the treating physician has determined that the benefit/risk for doing so is favorable (See criteria below).
    - Participants should meet the following criteria to continue study intervention despite progression of disease:
        - Absence of symptoms and signs indicating clinically significant progression of disease
        - No decline in ECOG performance status
        - Absence of symptomatic rapid disease progression requiring urgent medical intervention (eg, symptomatic pleural effusion, spinal cord compression)
- Participant no longer benefits in the opinion of the investigator from continuing study intervention.
- Unacceptable toxicity or AE leading to permanent discontinuation. If the unacceptable toxicity is attributed to PF-06821497, enzalutamide or docetaxel, the investigator (may consult the sponsor's medical monitor to discuss) may continue treatment with the other study intervention(s) since more than 1 treatment may be given (if participant is receiving docetaxel). (See Sections 6.6.1, 6.6.2, 6.6.3)
- Global deterioration of health status requiring discontinuation;
- Significant protocol violation;
- Lost to follow-up;
- Participant refused further treatment;
- Study terminated by sponsor;
- Death.

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Discontinuation of study intervention does not represent withdrawal from the study. If study intervention is permanently discontinued, the participant will remain in the study to be evaluated for safety and efficacy including survival at EOT, the 28 day Safety Follow-up, and LTFU visits. See the SoA, Table 1 and Table 2 for data to be collected at the time of discontinuation of study intervention and follow-up for any further evaluations that need to be completed.

In the event of discontinuation of study intervention, it must be documented on the appropriate CRF/in the medical records whether the participant is discontinuing further receipt of study intervention or also from study procedures, further study follow-up, and/or future collection of additional information.

When the participant permanently discontinues all study intervention, the EOT visit should be conducted. If the EOT visit coincides with a regular study visit, the EOT evaluations per the SoA will supersede those of the scheduled visit. The EOT visit should occur no longer than 1 week (± 7 days) after the discontinuation of all study intervention. All participants who permanently discontinue study drug intervention will remain in the study, complete safety follow-up, and subsequently commence long-term follow-up, which may include imaging for participants who have not yet had BICR confirmed radiographic progression.

7.1.1. 28 Day Safety Follow-Up

At least 28 calendar days, and no more than 35 calendar days after last dose of study intervention or prior to initiation of any new antineoplastic or investigational therapy whichever comes first, participants will undergo the 28 day safety follow-up assessments as detailed in the SoA. Any SAEs occurring during this period must still be reported to Pfizer Safety irrespective of any intervening treatment. See Section 8.4.1 for details for AE/SAE reporting requirements.

7.1.2. Long Term Follow-Up

For all participants in the LTFU period, participants will be monitored and data will be collected every 12 weeks (± 7 days) from Safety Follow-up until the participant dies or withdraws consent for follow-up, or the study is terminated by the sponsor. The LTFU period will conclude at the time of the final OS analysis. Crossover will not be allowed in the trial.

Reference the Schedule of Activities for additional details on assessments to be performed in Long Term Follow-up.

7.1.3. Survival Follow-Up

During the Survival Follow-up period, post-treatment survival status will be collected every 12 weeks (± 7 days) from the date of the Safety Follow-up visit. If the safety follow-up visit is not performed, it will be collected from the date of permanent discontinuation of study intervention. Participants will be contacted (eg, telephone, email, site visits, family member contact) for survival status and collection of subsequent anticancer therapies (eg, start and

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stop dates, date of disease progression of the therapy), until death, withdrawal of consent, lost to follow-up, or end of study, whichever occurs first. On sponsor request, participants may be contacted for survival information, eg, prior to each protocol-specified interim and final analysis. As permitted by local law, study site personnel may use public databases, perform an internet search, or review obituaries to determine date of death. Survival information should be recorded in the CRF.

7.1.4. Potential Cases of Acute Kidney Injury

The investigator should consult with the sponsor for any suspected cases of kidney injury.

In the event of new changes in kidney function [≥30% increase in serum creatinine from baseline or decrease ≥30% in eGFR], there should be further evaluation to rule out acute kidney injury. Evaluation should include physical examination, laboratory tests, detailed medical and surgical history, review of all medications (including recreational drugs and herbal supplements), family history, sexual history, travel history, blood transfusion, and potential occupational exposure to chemicals. If there is a reason to expect obstruction of the urinary tract as a cause of the changes in kidney function, evaluation for obstruction should be considered, and additional assessments such as renal ultrasound may also be obtained if appropriate. Other assessments that may be performed include: urine protein excretion, urine volume, and urine microscopic examination. A nephrology consult may be requested for further clarification. If decreased kidney function is suspected, participants should return to the clinic as soon as possible to confirm abnormal results and to conduct additional assessments if needed.

All confirmed cases of clinically relevant decrease in kidney function should be considered potential cases of drug-induced kidney injury if no other reason for the kidney function abnormalities is identified.

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7.1.5. Liver Injury

Study intervention should be permanently discontinued if any of the following criteria are met, and no alternative cause explains the laboratory abnormalities:

- Participants with AST/ALT and T bili baseline values within the normal range who subsequently present with AST **OR** ALT values≥3 × ULN **AND** a T bili value ≥2 × ULN with no evidence of hemolysis and an alkaline phosphatase value <2 × ULN or not available (Note: in the presence of elevated alkaline phosphatase associated with bone metastases, GGT should be tested, and the results should be within the reference range);

- For participants with baseline AST **OR** ALT **OR** T bili values above the ULN, the following threshold values are used in the definition mentioned above, as needed, depending on which values are above the ULN at baseline:
  - Preexisting AST or ALT baseline values above the normal range: AST or ALT values >2 times the baseline values AND >3 × ULN; or >8 × ULN (whichever is smaller).
  - Preexisting values of T bili above the normal range: T bili level increased from baseline value by an amount of ≥1 × ULN **or** if the value reaches ≥3 × ULN (whichever is smaller).
- Participants with AST/ALT >5 × ULN that persists for more than 7 days (AST/ALT >8 × ULN for participants with hepatic involvement).
  - Participants with AST/ALT >20 × ULN that persists for longer than 3 days.
  - Participants with T bili >3 × ULN that persists for longer than 7 days (>5 × ULN for participants with Gilbert's disease).

Criteria for temporary withholding of study intervention in association with liver test abnormalities is outlined in Section 6.6.4. All cases meeting the criteria for an AE or SAE should be reported as per the criteria and timeframe detailed in Appendix 3. Potential Hy's law cases (refer to Appendix 6) or cases resulting in permanent study intervention discontinuation are to be reported as SAEs.

7.1.6. COVID-19

If a participant has COVID-19 during the study, this should be reported as an AE or SAE (as appropriate) and appropriate medical intervention provided. Dose modification guidelines should be followed as clinically appropriate (Section 6.6.5). Study intervention should continue unless the investigator/treating physician is concerned about the safety of the participant, in which case temporary or permanent discontinuation may be required.

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It is recommended that the investigator discuss temporary or permanent discontinuation of study intervention with the study medical monitor.

7.1.7. Rechallenge

Rechallenge with enzalutamide and docetaxel may only be considered according to prescribing information. The investigator and sponsor should discuss and agree with any decision to rechallenge any of the assigned study interventions. Rechallenge with PF-06821497 following liver test abnormalities is not permitted if these abnormalities are accompanied with signs or symptoms consistent with drug induced liver toxicity or meet permanent discontinuation criteria. Rechallenge with PF-06821497 may be considered at the same dose or at 1 lower dose level once an alternative cause for the abnormal liver tests (ALT, AST, T bili) is discovered and the laboratory abnormalities resolve to normal or baseline values. Following rechallenge, participants should be closely monitored for signs and symptoms of hepatitis and/or abnormal liver test results.

7.1.8. Blinded Independent Central Review of Tumor Assessments

A third-party core imaging laboratory will perform BICR of radiographic images for all participants to determine the protocol defined endpoint of radiographic disease progression (rPFS).

It is important to the integrity of the study that the following materials are provided to BICR as each participant enrolls and progresses through the study: imaging studies for all participants in the following phases; Screening, Active Treatment Phase and Follow-up until confirmation of objective disease progression by BICR as per RECIST v1.1 and PCWG3, irrespective of the discontinuation of study treatment.

7.1.8.1. Expedited Blinded Independent Central Review for Disease Progression

To mitigate the potential for bias in determining disease progression, expedited BICR will be performed for investigator assessed disease progression. Upon investigator assessed disease progression, the radiographic images as well as all the other information reported as per Section 8.2.2.1 will be submitted to the BICR for expedited review. See the Study Imaging Manual for process details. At investigator discretion, the participant may remain on study intervention until the BICR has completed the review.

7.1.8.2. Management of Incidental Findings

An incidental finding is one unknown to the participant that has potential health or reproductive importance, which is discovered unexpectedly in the course of a research study but is unrelated to the purpose and beyond the aims of the study.

Tumor assessment images will be reviewed by a central review facility. The purpose of this review is to evaluate images for disease assessment. Central image review is not a complete medical review of the participant. If, during the central review process, an unexpected observation is identified and this finding could, in the opinion of the central reviewer, have a significant health or reproductive consequence, this finding may be shared with the study sponsor for disclosure to the PI. All follow-up testing and final diagnosis will be left to the discretion of the medical professionals at the site or those with an existing physician participant relationship. The PI will be responsible for reporting any AEs identified from

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incidental findings as described in the AE reporting section. Identification of such incidental findings during the central review process should not be expected, and the site maintains responsibility for performing a general safety review of all images as per site protocols.

7.2. Participant Discontinuation/Withdrawal From the Study

A participant may withdraw from the study at any time at their own request. Reasons for discontinuation from the study include the following:

- Refused further follow-up;
- Lost to follow-up;
- Death;
- Study terminated by sponsor;

The participant will be permanently discontinued from the study intervention and the study at that time.

If a participant withdraws from the study, they may request destruction of any remaining samples taken and not tested, and the investigator must document any such requests in the site study records and notify the sponsor accordingly.

If the participant withdraws from the study and also withdraws consent (See Section 7.2.1) for disclosure of future information, no further evaluations will be performed and no additional data will be collected. The sponsor may retain and continue to use any data collected before such withdrawal of consent in accordance with local law.

7.2.1. Withdrawal of Consent

Participants who request to discontinue receipt of study intervention will remain in the study and must continue to be followed for protocol-specified follow-up procedures. The only exception to this is when a participant specifically withdraws consent for any further contact with them or persons previously authorized by the participant to provide this information. Participants should notify the investigator in writing of the decision to withdraw consent from future follow-up, whenever possible. The withdrawal of consent should be explained in detail in the medical records by the investigator, as to whether the withdrawal is only from further receipt of study intervention or also from study procedures, post treatment study follow-up, contacting the participant or other individual such as family member, and/or review of medical records, and entered on the appropriate CRF page. In the event that vital status (whether the participant is alive or dead) is being measured, publicly available information should be used to determine vital status only as appropriately directed in accordance with local law.

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7.2.2. Lost to Follow-Up

A participant will be considered lost to follow-up if the participant repeatedly fails to return for scheduled visits and is unable to be contacted by the study site.

The following actions must be taken if a participant fails to attend a required study visit:

- The site must attempt to contact the participant and reschedule the missed visit as soon as possible. Counsel the participant on the importance of maintaining the assigned visit schedule and establish whether the participant wishes to continue, if eligible to do so;
- Before a participant is deemed lost to follow-up, the investigator or designee must make every effort to regain contact with the participant (where possible, 3 telephone calls and, if necessary, a certified letter to the participant's last known mailing address) These contact attempts should be documented in the participant's medical record;
- Should the participant continue to be unreachable, the participant will be considered to have withdrawn from the study.

8. STUDY ASSESSMENTS AND PROCEDURES

8.1. Administrative and Baseline Procedures

The investigator or qualified designee must obtain a signed and dated ICD before performing any study-specific procedures.

Study procedures and their timing are summarized in the SoA. Protocol waivers or exemptions are not allowed.

Adherence to the study design requirements, including those specified in the SoA, is essential and required for study conduct.

All screening evaluations must be completed and reviewed to confirm that potential participants meet all eligibility criteria. The investigator will maintain a screening log to record details of all participants screened and to confirm eligibility or record reasons for screening failure, as applicable.

Procedures conducted as part of the participant's standard of care and obtained before signing of the ICD may be utilized for eligibility provided the procedures met the protocol-specified criteria and were performed within the time frame defined in the SoA.

Every effort should be made to ensure that protocol-required tests and procedures are completed as described. However, it is anticipated that from time to time there may be circumstances outside the control of the investigator that make it unfeasible to perform the test. In these cases, the investigator must take all steps necessary to ensure the safety and well-being of the participant. When a protocol-required test cannot be performed, the

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investigator will document the reason for the missed test and any corrective and preventive actions that they have taken to ensure that required processes are adhered to as soon as possible. The study team must be informed of these incidents in a timely manner.

For samples being collected and shipped, detailed collection, processing, storage, and shipment instructions and contact information will be provided to the investigator site prior to initiation of the study.

Demographic data and medical history will be collected at screening by the investigator or qualified designee, including relevant medical and surgical history, and current illnesses. Investigator's reason for treatment choice of docetaxel or enzalutamide will be collected prior to randomization and documented on the eCRF.

The total blood sampling volume for individual participants in this study is approximately 50 mL. The actual collection times of blood sampling may change. Additional blood samples may be taken for safety assessments as needed, provided the total volume taken during the study does not exceed 550 mL during any period of 56 consecutive days.

8.1.1. Telehealth Visits

In the event that in-clinic study visits cannot be conducted, every effort should be made to follow-up on the safety of study participants at scheduled visits per the SoA or unscheduled visits. Telehealth visits may be used to assess participant safety and collect data points. Telehealth includes the exchange of healthcare information and services via telecommunication technologies (eg, audio, video, video conferencing software) remotely, allowing the participant and the investigator to communicate on aspects of clinical care, including medical advice, reminders, education, and safety monitoring. Assessments that may be performed during a telehealth visit are described in the SoA, Table 1 and Table 2.

The following assessments are mandatory to be performed at a telehealth visit if they are required to be performed at the corresponding in-clinic visit:

- Review and record study intervention(s), including compliance and missed doses.
- Review and record any AEs and SAEs since the last contact.
- Review and record any new and changes in concomitant medications since the last contact.
- Review and record contraceptive method.

PRO assessments should be completed by participant with an electronic device prior to a telehealth visit.

Any telehealth visit may be changed to an in-person visit based on participant preference or investigator discretion.

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Participants must be reminded to promptly notify site staff about any change in their health status. Participants may request an unscheduled telehealth or in-person visit with site staff.

8.2. Efficacy Assessments

8.2.1. Assessment of Efficacy

Study assessments of efficacy are measures of prostate cancer status to evaluate the primary and secondary study endpoints. These assessments will include disease status, survival status, PSA, symptomatic skeletal events, pain scores, quality of life and PFS2.

8.2.2. Assessment of Primary Efficacy Endpoint: rPFS

The primary efficacy endpoint is BICR assessed rPFS per RECIST v1.1 (soft tissue disease) and PCWG3 (bone disease) which will be assessed radiographically by blinded central radiology review(s) at the time points specified in the SoA, Table 1 and Table 2 by CT of chest and CT or MRI of abdomen and pelvis. Bone disease status will be assessed by whole body radionuclide bone scan (ie radionuclide [Tc-99m] bone scintigraphy).

PET and SPECT scans are not permitted imaging modalities for this study. However, if a PSMA PET is conducted as SOC, these data should be entered on the CRF.

When medically acceptable, contrast enhanced CT scan is the preferred method for assessment of soft tissue disease and will be performed on the chest, abdomen, and pelvis. Participants who are intolerant to CT contrast agents should undergo CT scans without contrast enhancement of the chest and contrast enhanced MRI scans of the abdomen and pelvis for assessment of soft tissue disease. In cases where a participant is not tolerant of an MRI of the abdomen / pelvis, CT scans of the chest/abdomen and pelvis without contrast enhancement are acceptable. Soft tissue response and progression will be evaluated per RECIST v1.1 (Eisenhauer et al, 2009). Whole body bone scans will be performed to assess the presence of bone disease. Bone disease is not to be assessed by CT or MRI; progression of bone disease will be evaluated per PCWG3 (Hussain et al, 2020).

In order to ensure consistency in assessment of imaging throughout the study, the imaging modality(s) used for soft tissue assessment of a given lesion at screening should remain the same throughout the study, unless doing so would compromise participant safety. The investigator is asked to inform the sponsor prior to a change in soft tissue imaging modality and the reason for doing so, prior to the radiographic assessment, whenever possible.

The scheduling of imaging assessments should be fixed according to the calendar, regardless of treatment schedule, treatment delays or interruptions. Imaging assessments are to be scheduled using the randomization date as the reference date for all time points and are NOT to be scheduled based on the date of the previous imaging time point.

All images will be read by site staff at the study site and locally determined assessments will be entered in the appropriate eCRF. Each study site should designate a radiologist or

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investigator to ensure that all images are read as specified by the protocol and to ensure consistency in the readings. All participants' files and radiologic images must be available for source verification and for potential peer review.

8.2.2.1. Blinded Independent Central Review of Tumor Assessments

A third-party core imaging laboratory will perform BICR of radiographic images for all participants to determine the protocol defined endpoint of radiographic disease progression.

It is important to the integrity of the study that the following materials are provided to BICR as each participant enrolls and progresses through the study: imaging studies for all participants in the following phases; Screening, Active Treatment Phase and, Follow-up. images. Additionally, it is important to the integrity of the study that all imaging studies, clinical information (as applicable) are provided to BICR on an ongoing basis (as soon as they are available) until confirmation of objective disease progression by BICR as per RECIST v1.1, regardless of the discontinuation study treatment.

8.2.2.2. Expedited Blinded Independent Central Review for Disease Progression

To mitigate the potential for bias in determining disease progression, expedited BICR will be performed for investigator assessed disease progression. Upon investigator assessed disease progression, the radiographic images as well as all the other information reported as per Section 8.2.2.1 will be submitted to the BICR for expedited review. See the Study Imaging Manual for process details. At investigator discretion, the participant may remain on study intervention until the BICR has completed the review.

8.2.2.3. Management of Incidental Findings

An incidental finding is one unknown to the participant that has potential health or reproductive importance, which is discovered unexpectedly in the course of a research study but is unrelated to the purpose and beyond the aims of the study.

Tumor assessment images will be reviewed by a central review facility. The purpose of this review is to evaluate images for disease assessment. Central image review is not a complete medical review of the participant. If, during the central review process, an unexpected observation is identified and this finding could, in the opinion of the central reviewer, have a significant health or reproductive consequence, this finding may be shared with the study sponsor for disclosure to the PI. All follow-up testing and final diagnosis will be left to the discretion of the medical professionals at the site or those with an existing physician-participant relationship. The PI will be responsible for reporting any AEs identified from incidental findings as described in the AE reporting section. Identification of such incidental findings during the central review process should not be expected, and the site maintains responsibility for performing a general safety review of all images as per site protocols.

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8.2.3. Assessment of Secondary Efficacy Endpoints

The study assessments of efficacy for the secondary endpoints will include standard radiographic and imaging methods to evaluate OR and DoR. Additionally, time to PSA progression, time to initiation of anti-neoplastic therapy, time to first SSE, OS, PFS2, survival status monitoring, and evaluation of concomitant treatment (Section 6.9), at time points specified in the SoA (Table 1 and Table 2) will be evaluated.

Blood samples will be drawn for analysis of PSA levels to determine biochemical response and progression. PSA will be assessed centrally throughout the study until radiographic progression according to the SoA (Table 1 and Table 2). Investigators are strongly discouraged from discontinuing study interventions or initiating new systemic therapy due to a rising PSA, study intervention administration should continue until study intervention discontinuation criteria (Section 7.1) are met.

Symptomatic skeletal event evaluation includes tumor-related symptomatic fractures, surgery, or radiotherapy to the bone and spinal cord compression. All fractures and spinal cord compression (including those that are asymptomatic or un-related to prostate cancer) must also be recorded in the eCRF as an AE.

8.2.4. Patient Reported Outcome Assessments

PRO assessments will be collected by electronic device at screening, C1D1 and at the timepoints specified in the SoA. For C1D1, randomization must occur first; then participants are to complete questionnaires before the first dose of study treatment.

For the scheduled PRO-CTCAE and FACT-GP5 assessments during the first 5 weeks (Week 6), participant is to complete the questionnaires +/- 1 days of the scheduled assessment. Then for all subsequent PRO-CTCAE and FACT-GP5 assessments after Week 6, starting Week 7 the patient is to complete the questionnaires +/- 3 days of the scheduled assessment.

At all subsequent scheduled PRO assessments for the BPI-SF, FACT-P and EQ-5D-5L, participant is to complete the questionnaires +/- 3 days of the scheduled assessment.

When PRO assessment timepoints do not overlap with a study visit, all PROs must be completed using an ePRO device. In cases in which the PRO assessment timepoint overlaps with a study visit, all PROs must be completed using an ePRO device prior to receiving any study intervention, prior to any other study assessment or consultation with the investigator, and prior to being informed of their current disease status. Questionnaires should be completed independently.

<u>Pain Symptoms</u>

Brief Pain Inventory – Short Form: The BPI-SF is a 9-item measure of self-reported pain with general activity, mood, walking, work, relations with others, sleep, and the ability to enjoy life during the previous 24 hours. The BPI-SF has been used extensively and has significant evidence of validity in assessing cancer-related pain (Gelhorn et al, 2017). The worst pain or average pain item from the BPI-SF has been recommended by IMMPACT as a

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core outcome measure in clinical trials of analgesics (Dworkin et al, 2008). BPI-SF pain items use a 0-10 NRS, and the symptom items have verbal anchors of 0=”No Pain” and 10=”Pain as bad as you can imagine. In this study, a single item of the BPI-SF, the “worst pain” item 3 will be used as one of two measures of potential pain progression.

HRQoL, Functioning and Symptoms

The FACT-P questionnaire is a 39-item multidimensional, self-reported, quality-of-life instrument specifically designed for use in men with prostate cancer. The questionnaire contains 27 core items to assess participant’s function during the prior 7 days in the 4 domains of physical, social/family, emotional, and functional well-being; it also contains 12 site-specific items to assess prostate-related symptoms. Each item is rated on a 0 to 4 Likert-type scale and then combined to produce subscale scores for each domain, as well as the FACT-P total score, with higher scores representing better health-related quality of life.

Health Status

The EQ-5D-5L is a validated and standardized instrument that measures the general health status of participants. Participants will self-rate their current state of mobility, self-care, usual activities, pain/discomfort, and anxiety/depression by choosing 1 of 5 possible responses that record the level of severity (no problems, slight problems, moderate problems, severe problems, or extreme problems) within each dimension. The questionnaire also includes a visual analog scale to self-rate general health state on a scale from “the worst health you can imagine” to “the best health you can imagine.”

Symptomatic AEs and Overall Side Effect Burden

Patient-Reported Outcomes - Common Terminology Criteria for Adverse Events (PRO-CTCAE): The PRO-CTCAE is a PRO measure developed to evaluate symptomatic toxicity in participants enrolled in cancer clinical trials. It is complementary to the CTCAE. The PRO-CTCAE items selected for this study are diarrhea, taste changes, vomiting, nausea and decreased appetite. The selection was based on preliminary adverse event profile of PF-06821497 with or without enzalutamide from the C2321001 study. Each item will be assessed related to one or more attributes that include the frequency, severity, and/or interference with usual or daily activities.

The FACT-GP5 item is included in the FACT-P. The FACT-GP5 question “I am bothered by side effects of treatment,” is rated on a 5-point scale ranging from “not at all” (0) to “very much” (4). The GP5 item will capture overall AE impact and burden (FDA, 2021).

8.2.5. Exploratory Endpoint Assessments – Biomarkers

Tumor tissue and blood samples will be collected to evaluate biomarkers (genes and proteins) involved in the biology of the participant’s disease to understand the relationship between the therapeutic interventions being studied and the biology of the participant’s disease. The measurement of the biomarkers may consist of DNA, RNA, protein or defined cell types resulting from analyses of peripheral blood and/or tumor tissue biospecimens obtained at baseline, on treatment, and/or at EOT.

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Refer to the SoA (Table 1 and Table 2) for sample collection time points and Laboratory Manual for sample processing and shipping. Unless prohibited by local regulations or IRBs/ECs, the following samples for biomarker research are required and will be collected from all participants in this study as specified in the SoA (Table 1 and Table 2):

- Tumor tissue (de novo or archived) for retrospective molecular profiling analysis, if available. See Section 8.8.1
- Blood samples for ctDNA and cfRNA analysis. See Section 8.8.2.
- Blood samples for matched germline. See Section 8.8.3.
- Blood samples for serum biomarker research. See Section 8.8.4.
- Retained research sample. See Section 8.7.2.

Additional information on tissue collection procedures, and sample preparations including processing, storage, and shipment to the sponsor designated laboratories for biomarker analysis will be provided in the Laboratory Manual.

8.2.6. Healthcare Resource Utilization

Healthcare resource utilization for hospitalizations and length of stay will be collected in this study for health economic purposes.

Healthcare resource utilization for hospitalizations and length of stay will be reported for unscheduled visits and will be captured via the CRF. Healthcare resource use will be collected at the times specified in the SoA, Table 1 and Table 2.

8.3. Safety Assessments

Planned time points for all safety assessments are provided in SoA, (Table 1 and Table 2. Unscheduled safety measurements may be obtained at any time during the study to assess any perceived safety issues.

Safety endpoints include adverse events (graded according to the NCI CTCAE, version 5.0), physical examination (including blood pressure and pulse), and laboratory tests (hematology and chemistry).

8.3.1. Vital Signs and Physical Examinations

Vital sign measurements (blood pressure, heart rate, and temperature [ºC]) height (cm) and weight (kg) will be assessed as shown in Table 1 and Table 2. Height is only required to be collected at Screening.

Physical examinations will be conducted according to the schedule of activities in Table 1 and Table 2. At Screening, this will constitute an assessment of systems (eg, general appearance, head, eyes, ears, nose, mouth, throat, skin, heart, lung, lymph nodes, gastrointestinal, genitourinary, neurologic, and skeletal). During treatment, systems will be assessed per standard of care at the study site or as clinically indicated by symptoms.

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PE findings collected during the study will be considered source data and will not be required to be reported, unless otherwise noted. Any untoward physical examination findings that are identified during the active collection period and meet the definition of an AE or SAE (Appendix 3) must be reported according to the processes in Section 8.4.1 to 8.4.3.

8.3.2. (12-Lead) Electrocardiogram

Standard 12-lead ECGs utilizing limb leads (with a 10 second rhythm strip) should be collected at times specified in the SoA (Table 1 and Table 2) section of this protocol using an ECG machine that automatically calculates the heart rate and measures PR, QT, and QTcF intervals and QRS complex. For ECG machines that do not report QTcF, calculation of QTcF from QT and heart rate should be performed. All scheduled ECGs should be performed after the participant has rested quietly for at least 5 minutes in a supine position.

The participant should undergo additional ECG monitoring if a) a post dose QTcF interval remains ≥60 ms from the baseline **and** is >450 ms; or b) an absolute QT value is ≥500 ms for any scheduled ECG for greater than 4 hours (or sooner, at the discretion of the investigator); or c) QTcF intervals get progressively longer. A cardiologist should be consulted if QTcF values do not return to less than the criteria listed above after 8 hours of monitoring (or sooner, at the discretion of the investigator).

In some cases, it may be appropriate to repeat abnormal ECGs to rule out improper lead placement as contributing to the ECG abnormality. It is important that leads be placed in the same positions each time in order to achieve precise ECG recordings. If a machine-read QTcF value is prolonged, as defined above, repeat measurements may not be necessary if a qualified medical provider's interpretation determines that the QTcF values are in the acceptable range.

When an ECG is to be performed at the same time point as a blood collection, the ECG must be performed prior to obtaining the blood draw.

ECG values of potential clinical concern are listed in Appendix 8. Any clinically significant changes from the baseline / Day 1 ECG may potentially be AEs (See Appendix 8) and should be evaluated further, as clinically warranted.

8.3.2.1. Alternative Facilities for Electrocardiograms

The participant may visit an alternative facility to have the ECGs performed. Qualified study site personnel must order, receive, and review results.

8.3.3. Clinical Safety Laboratory Assessments

See Appendix 2 for the list of clinical safety laboratory tests to be performed and the SoA for the timing and frequency. All protocol required laboratory assessments, as defined in Appendix 2, must be conducted in accordance with the laboratory guidance (eg manual and the SoA (Table 1 and Table 2). Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues.

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Central and/or Local laboratories will perform safety clinical laboratory assessments including hematology, chemistry, PSA and testosterone. At Screening, laboratory results from the central laboratories (unless otherwise specified) should be used by the investigator to determine the participant's eligibility for the study. Local laboratory results may be used if needed.

The investigator must review the laboratory report, document this review, and record any clinically significant changes occurring during the study in the AE section of the CRF. Clinically significant abnormal laboratory test findings are those that are not associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition.

All laboratory tests with values considered clinically significant and abnormal during participation in the study or within 28 days after the last dose of study intervention should be repeated until the values return to normal or baseline or are no longer considered clinically significant by the investigator or study medical monitor.

If such values do not return to normal/baseline within a period of time judged reasonable by the investigator, the etiology should be identified, and the sponsor notified.

See Appendix 6 for suggested actions and follow-up assessments in the event of potential DILI.

See Appendix 7 for instructions for laboratory testing to monitor kidney function and reporting laboratory test abnormalities.

No need to repeat a clinical laboratory assessment on Cycle 1 Day 1 if the baseline assessment was performed within 7 days prior to that date.

8.3.3.1. Alternative Facilities for Clinical Safety Laboratory Assessment

If a local laboratory is used, qualified study site personnel must order, receive, and review results. Site staff must collect the local laboratory reference ranges and certifications/ accreditations for filing at the site. Laboratory test results are to be provided to the site staff as soon as possible. The local laboratory reports should be filed in the participant's source documents/medical records.

8.3.4. Symptomatic Skeletal Event Evaluation

Symptomatic skeletal event evaluation includes tumor-related symptomatic fractures, surgery, or radiotherapy to the bone and spinal cord compression. All fractures and spinal cord compression must also be recorded in the eCRF as an AE. This assessment will occur at baseline, throughout the treatment phase, and in LTFU.

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8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting

The definitions of an AE and an SAE can be found in Appendix 3.

AEs may arise from symptoms or other complaints reported to the investigator by the participant (or, when appropriate, by a caregiver, surrogate, or the participant's legally authorized representative), or they may arise from clinical findings of the investigator or other healthcare providers (clinical signs, test results, etc).

The investigator and any qualified designees are responsible for detecting, documenting, and recording events that meet the definition of an AE or SAE and remain responsible to pursue and obtain adequate information both to determine the outcome and to assess whether the event meets the criteria for classification as an SAE or caused the participant to discontinue the study intervention (See Section 7.1).

During the active collection period as described in Section 8.4.1 each participant will be questioned about the occurrence of AEs in a nonleading manner.

In addition, the investigator may be requested by Pfizer Safety to obtain specific follow-up information in an expedited fashion.

8.4.1. Time Period and Frequency for Collecting AE and SAE Information

The time period for actively eliciting and collecting AEs and SAEs ("active collection period") for each participant begins from the time the participant provides informed consent, which is obtained before undergoing any study-related procedure and/or receiving study intervention, through and including a minimum of 28 calendar days, except as indicated below, after the last administration of the study intervention.

Follow-up by the investigator continues throughout and after the active collection period and until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator.

When a clinically important AE remains ongoing at the end of the active collection period, follow-up by the investigator continues until the AE or SAE or its sequelae resolve or stabilize at a level acceptable to the investigator and Pfizer concurs with that assessment.

For participants who are screen failures, the active collection period ends when screen failure status is determined.

If the participant withdraws from the study and also withdraws consent for the collection of future information, the active collection period ends when consent is withdrawn.

If a participant permanently discontinues or temporarily discontinues study intervention because of an AE or SAE, the AE or SAE must be recorded on the CRF and the SAE reported using the applicable reporting method as described in Appendix 3, Section 10.3.4 .

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Investigators are not obligated to actively seek information on AEs or SAEs after the participant has concluded study participation. However, if the investigator learns of any SAE, including a death, at any time after a participant has concluded study participation, and they consider the event to be reasonably related to the study intervention, the investigator must promptly report the SAE to Pfizer.

8.4.1.1. Reporting Nonserious AEs and SAEs to Pfizer Safety

All SAEs occurring in a participant during the active collection period as described in Section 8.4.1 are reported to Pfizer Safety immediately upon awareness and under no circumstance should this exceed 24 hours, as indicated in Appendix 3. The investigator will submit any updated SAE data to the sponsor within 24 hours of these data being available.

All nonserious AEs and any updated information that occur during the active collection period (as defined in Section 8.4.1) that are assessed as causally related to an NIMP/AxMP (as listed in Section 6.1) must be reported to Pfizer Safety via SAE+ in the EDC system, as indicated in Appendix 3.

If a participant begins a new anticancer therapy, SAEs occurring during the above-indicated active collection period must still be reported to Pfizer Safety irrespective of any intervening treatment. Note that a switch to a commercially available version of the study intervention is considered as a new anticancer therapy for purposes of SAE reporting.

8.4.1.1.1. Reporting Second Primary Malignancies

All AEs and any diagnosis of second primary malignancies (with the exception of non-melanoma skin cancers) are to be reported to Pfizer Safety as an SAE regardless of the active collection period and irrespective of study intervention, investigator's opinion of causality or time of diagnosis. In addition, Investigators should report any cases of MDS, T-LBL, and AML as they become aware of irrespective of causality attribution for up to 4 years after the initiation of PF-06821497 treatment.

8.4.1.2. Recording Nonserious AEs and SAEs on the CRF

All nonserious AEs and SAEs occurring in a participant during the active collection period which begins after obtaining informed consent as described in Section 8.4.1, will be recorded on the AE section of the CRF.

The investigator is to record on the CRF all directly observed, and all spontaneously reported AEs and SAEs reported by the participant.

As part of ongoing safety reviews conducted by the sponsor, any nonserious AE that is determined by the sponsor to be serious will be reported by the sponsor as an SAE. To assist in the determination of case seriousness, further information may be requested from the investigator to provide clarity and understanding of the event in the context of the clinical study.

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If a participant begins a new anticancer therapy, the recording period for nonserious AEs ends at the time the new treatment is started; however, SAEs must continue to be recorded on the CRF during the above indicated active collection period. Note that a switch to a commercially available version of the study intervention is considered as a new anticancer therapy for the purposes of SAE reporting.

Reporting of AEs and SAEs for participants who fail screening are subject to the CRF requirements as described in Section 5.4.

8.4.2. Method of Detecting AEs and SAEs

The method of recording, evaluating, and assessing causality of AEs and SAEs and the procedures for completing and transmitting SAE reports are provided in Appendix 3.

Care will be taken not to introduce bias when detecting AEs and/or SAEs. Open-ended and nonleading verbal questioning of the participant is the preferred method to inquire about AE occurrences.

8.4.3. Follow-Up of AEs and SAEs

After the initial AE or SAE report, the investigator is required to proactively follow each participant at subsequent visits/contacts. For each event, the investigator must pursue and obtain adequate information until resolution, stabilization, the event is otherwise explained, or the participant is lost to follow-up (as defined in Section 7.2.2).

In general, follow-up information will include a description of the event in sufficient detail to allow for a complete medical assessment of the case and independent determination of possible causality. Any information relevant to the event, such as concomitant medications and illnesses, must be provided. In the case of a participant death, a summary of available autopsy findings must be submitted as soon as possible to Pfizer Safety.

Further information on follow-up procedures is provided in Appendix 3.

8.4.4. Regulatory Reporting Requirements for SAEs

Prompt notification by the investigator to the sponsor of an SAE is essential so that legal obligations and ethical responsibilities toward the safety of participants and the safety of a study intervention under clinical investigation are met.

The sponsor has a legal responsibility to notify both the local regulatory authority and other regulatory agencies about the safety of a study intervention under clinical investigation. The sponsor will comply with country-specific regulatory requirements relating to safety reporting to the regulatory authority, IRBs/ECs, and investigators.

Investigator safety reports must be prepared for SUSARs according to local regulatory requirements and sponsor policy and forwarded to investigators as necessary.

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An investigator who receives SUSARs or other specific safety information (eg, summary or listing of SAEs) from the sponsor will review and then file it along with the SRSD(s) for the study and will notify the IRB/EC, if appropriate according to local requirements.

8.4.5. Environmental Exposure, Exposure During Pregnancy or Breastfeeding, and Occupational Exposure

Environmental exposure occurs when a person not enrolled in the study as a participant receives unplanned direct contact with or exposure to the study intervention. Such exposure may or may not lead to the occurrence of an AE or SAE. Persons at risk for environmental exposure include healthcare providers, family members, and others who may be exposed. An environmental exposure may include EDP, EDB, and occupational exposure.

Any such exposure to the study intervention under study are reportable to Pfizer Safety within 24 hours of investigator awareness.

8.4.5.1. Exposure During Pregnancy

An EDP occurs if:

- A male participant who is receiving or has discontinued study intervention inseminates a female partner.
- A female nonparticipant is found to be pregnant while being exposed or having been exposed to study intervention because of environmental exposure. Below are examples of environmental EDP:
    - A female family member or healthcare provider reports that she is pregnant after having been exposed to the study intervention by ingestion.
    - A male family member or healthcare provider who has been exposed to the study intervention by ingestion then inseminates his female partner prior to or around the time of conception.

The investigator must report EDP to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. The initial information submitted should include the anticipated date of delivery (See below for information related to termination of pregnancy). For information on how to report to Pfizer Safety, refer to Section 10.3.4.

- If EDP occurs in a participant's partner after the start of study intervention and until 30 days after the last dose, the investigator must report this information to Pfizer Safety regardless of whether an SAE has occurred. Details of the pregnancy will be collected after the start of study intervention and until 30 days after last dose of PF-06821497.

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- If EDP occurs in the setting of environmental exposure, the investigator must report information to Pfizer Safety. Since the exposure information does not pertain to the participant enrolled in the study, the information is not recorded on a CRF; however, a copy of the completed report is maintained in the investigator site file.

Follow-up is conducted to obtain general information on the pregnancy and its outcome for all EDP reports with an unknown outcome. The investigator will follow the pregnancy until completion (or until pregnancy termination) and notify Pfizer Safety of the outcome as a follow-up to the initial report. In the case of a live birth, the structural integrity of the neonate can be assessed at the time of birth. In the event of a termination, the reason(s) for termination should be specified and, if clinically possible, the structural integrity of the terminated fetus should be assessed by gross visual inspection (unless pre-procedure test findings are conclusive for a congenital anomaly and the findings are reported).

Abnormal pregnancy outcomes are considered SAEs. If the outcome of the pregnancy meets the criteria for an SAE (ie, ectopic pregnancy, spontaneous abortion, intrauterine fetal demise, neonatal death, or congenital anomaly in a live-born baby, a terminated fetus, an intrauterine fetal demise, or a neonatal death), the investigator should follow the procedures for reporting SAEs. Additional information about pregnancy outcomes that are reported to Pfizer Safety as SAEs follows:

- Spontaneous abortion including miscarriage and missed abortion should be reported as an SAE;

- Neonatal deaths that occur within 1 month of birth should be reported, without regard to causality, as SAEs. In addition, infant deaths after 1 month should be reported as SAEs when the investigator assesses the infant death as related or possibly related to exposure to the study intervention.

Additional information regarding the EDP may be requested by the sponsor. Further follow-up of birth outcomes will be handled on a case-by-case basis (eg, follow-up on preterm infants to identify developmental delays). In the case of paternal exposure, the investigator will provide the participant with the Pregnant Partner Release of Information Form to deliver to his partner. The investigator must document in the source documents that the participant was given the Pregnant Partner Release of Information Form to provide to his partner.

8.4.5.2. Exposure During Breastfeeding

An EDB occurs if:

- A female nonparticipant is found to be breastfeeding while being exposed or having been exposed to study intervention (ie, environmental exposure). An example of environmental EDB is a female family member or healthcare provider who reports that she is breastfeeding after having been exposed to the study intervention by ingestion.

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The investigator must report EDB to Pfizer Safety within 24 hours of the investigator's awareness, irrespective of whether an SAE has occurred. For information on how to report to Pfizer Safety, refer to Section 10.3.4 . When EDB occurs in the setting of environmental exposure, the exposure information does not pertain to the participant enrolled in the study**,** so the information is not recorded on a CRF. However, a copy of the completed report is maintained in the investigator site file.

An EDB report is not created when a Pfizer drug specifically approved for use in breastfeeding individuals (eg, vitamins) is administered in accordance with authorized use. However, if the infant experiences an SAE associated with such a drug, the SAE is reported together with the EDB.

8.4.5.3. Occupational Exposure

The investigator must report any instance of occupational exposure to Pfizer Safety within 24 hours of the investigator's awareness using the CT SAE Report Form regardless of whether there is an associated SAE. Since the information about the occupational exposure does not pertain to a participant enrolled in the study, the information is not recorded on a CRF; however, a copy of the completed report must be maintained in the investigator site file.

8.4.6. Cardiovascular and Death Events

Cardiovascular AEs, if meeting the definition of seriousness (Appendix 3) will be reported according to the standard process for expedited reporting of SAEs. Death is considered an outcome and not an AE. AEs with a death outcome will be reported according to the standard process for expedited reporting of SAEs.

8.4.7. Disease-Related Events and/or Disease-Related Outcomes Not Qualifying as AEs or SAEs

Not applicable

8.4.8. Protocol-Specified Adverse Events of Special Interest

Protocol-specified AESIs are examined as part of routine safety data review procedures throughout the clinical trial and as part of signal detection processes. Should an aggregate analysis indicate that these prespecified events occur more frequently than expected, eg, based on epidemiological data, literature, or other data, then this will be submitted and reported in accordance with Pfizer's safety reporting requirements. Aggregate analyses of safety data will be performed on a regular basis per internal SOPs.

All protocol-specified AESIs must be reported as an AE or SAE following the procedures described in Sections 8.4.1 through 8.4.4. A protocol-specified AESI is to be recorded as an AE or SAE on the CRF. In addition, a protocol-specified AESI that is also an SAE must be reported to Pfizer Safety.

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8.4.9. Medical Device Deficiencies

Not applicable.

8.4.10. Medication Errors

Medication errors may result from the administration or consumption of the study intervention by the wrong participant, or at the wrong time, or at the wrong dosage strength.

Medication errors are recorded and reported as follows:

Recorded on the Medication Error Page of the CRF	Recorded on the AE Page of the CRF	Reported to Pfizer Safety Within 24 Hours of Awareness
All (regardless of whether associated with an AE)	Any AE or SAE associated with the medication error	Only if associated with an SAE

Medication errors include:

- Medication errors involving participant exposure to the study intervention;
- Potential medication errors or uses outside of what is foreseen in the protocol that do or do not involve the study participant.

Other examples include, but are not limited to:

- The administration of expired study intervention;
- The administration of an incorrect study intervention;
- The administration of an incorrect dosage;
- The administration of study intervention that has undergone temperature excursion from the specified storage range, unless it is determined by the sponsor that the study intervention under question is acceptable for use.

Whether or not the medication error is accompanied by an AE, as determined by the investigator, such medication errors occurring to a study participant are recorded on the medication error page of the CRF, which is a specific version of the AE page and, if applicable, any associated serious and nonserious AE(s) are recorded on the AE page of the CRF.

In the event of a medication dosing error, the sponsor should be notified within 24 hours. Medication errors should be reported to Pfizer Safety within 24 hours **only when associated with an SAE.**

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8.5. Pharmacokinetics

8.5.1. Plasma for PK analysis of PF-06821497

Blood samples (2 mL) to provide approximately 1 mL plasma for PK analysis will be collected into appropriately labeled tubes containing ethylenediaminetetraacetic acid (EDTA) (anticoagulant) as outlined in the Schedule of Activities, Table 1.

On days when participants have clinical visits where PK assessments are to be obtained, the morning dose of PF-06821497 should be held (NOT taken) prior to the study visit. On those days, the PF-06821497 morning dose should be taken after the study procedures required immediately prior to the PF-06821497 morning dose have been performed.

In addition to samples collected at the scheduled times, an additional blood sample may be collected from participants experiencing unexpected and/or serious AEs and the date and time of blood sample collection and of last dosing prior to PK collection documented on the CRF.

All efforts will be made to obtain the PK samples at the scheduled nominal time relative to dosing as outlined in SoA, and the exact time of the sample collection will be noted on the CRF. If a scheduled blood sample collection cannot be completed for any reason, the missed sample time may be re-scheduled with agreement of the clinical investigator, participant, and sponsor.

PK samples will be assayed for PF-06821497 using a validated analytical method in compliance with Pfizer/vendor SOPs. The PK samples must be processed and shipped as indicated in the instructions provided to the investigator site to maintain sample integrity. Any deviations from the PK sample handling procedure (eg, sample collection and processing steps, interim storage or shipping conditions), including any actions taken, must be documented and reported to the sponsor. On a case-by-case basis, the sponsor may make a determination as to whether sample integrity has been compromised.

As part of understanding the PK of the investigational product, samples may be used for the evaluation of the bioanalytical method, as well as for other internal exploratory purposes. These data will not be included in the CSR.

8.6. Pharmacodynamics

Biomarker samples may be used for exploratory pharmacodynamics analyses (See Biomarker Section 8.8).

As part of understanding the pharmacodynamics of the investigational products, samples may be used for evaluation of the bioanalytical method, as well as for other internal exploratory purposes. These data may not be included in the CSR.

The pharmacodynamic samples must be processed and shipped as indicated in the instructions provided to the investigator site to maintain sample integrity. Any deviations from the pharmacodynamic sample handling procedures (eg sample collection and

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processing steps, interim storage or shipping conditions), including any actions taken, must be documented and reported to the sponsor. On a case-by-case basis, the sponsor may decide whether sample integrity has been compromised.

8.7. Genetics

8.7.1. Specified Genetics

Specified genetics are not evaluated in this study.

8.7.2. Retained Research Samples for Genetics

A 4-mL whole blood sample optimized for DNA isolation Prep D1 will be collected according to the SoA, as local regulations and IRBs/ECs allow.

Retained Research Samples may be used for research related to the study intervention(s) and cancer. Genes and other analytes (eg, proteins, RNA, nondrug metabolites) may be studied using the banked samples. See Appendix 5 for information regarding genetic research. Details on processes for collection and shipment of these samples can be found in the SoA and central laboratory manual.

Banked biospecimens will not be collected from participants enrolled in China.

8.8. Biomarkers

Unless prohibited by local regulations or ethics committee decision, biospecimens will be collected to analyze DNA, RNA, protein or metabolic biomarkers for achieving study objectives. Specific analyses may not be performed if emerging data indicate that they would no longer support study objectives. Details regarding the types of biospecimen and purposes of collection are listed below.

The following samples for biomarker research are required and will be collected from all participants in this study as specified in the SoA (Table 1 and Table 2):

Table 8. Summary of Biomarker Sample Requirements

Sample types	Visits	Analyses
Tumor baseline tissue if available (de novo or archival)	C1 D1	Tumor genetic and biomarker analyses including DNA mutation, RNA expression profiling
Blood sample for circulating free nucleic acids	Arm A and Enzalutamide: Predose at C1 D1 (Wk 1), C3 D1 (Wk 9), C7 D1 (Wk 25), EOT Docetaxel: Predose at C1 D1	1) ctDNA burden 2) cfDNA mutation profiling 3) cfRNA variant profiling including AR-V7 splice variants

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Table 8. Summary of Biomarker Sample Requirements

Sample types	Visits	Analyses
	(Wk1), C3 D15 (Wk 9), C9 D1 (Wk 25), EOT	
Blood sample for serum biomarkers	Arm A and Enzalutamide: Predose at C1 D1 (Wk 1), C3 D1 (Wk 9), C7 D1 (Wk 25), EOT Docetaxel: Predose C1 D1 (Wk 1), C3 D15 (Wk 9); C9 D1 (Wk 25), EOT	Soluble protein markers including chromogranin A
Blood for matched CHIP/germline control	Predose at C1 D1	Filtering somatic vs germline and CHIP mutations

Refer to the schedule of activities for sample collection time points and Study/Laboratory Manual for sample processing and shipping details.

8.8.1. Tumor Tissue for Genetic/Biomarker Analysis

If available, tumor biospecimens from archival and/or de novo biopsies collected at baseline may be used to analyze candidate nucleic acid and protein biomarkers, or relevant signature of markers, for their ability to identify those participants who are most likely to benefit from treatment with the study drugs. Biomarkers may include, but are not limited to target expression, nucleic acid analyses, as well as cell types and constituents of the tumor microenvironment. RNA and/or DNA sequencing analysis, which may include targeted and/or whole exome/genome sequencing and/or transcriptome and/or epigenetic analyses, may be performed to examine genomic landscape, correlations between gene mutation status or gene expression signatures and clinical outcome.

If available, Archival or Newly acquired pretreatment FFPE/NBF tumor tissue obtained at Week 1 Day 1 visit (preferably tumor block; if not, unstained slides are also acceptable) will be submitted for genetic/biomarker analysis. The baseline tissue chosen should be the most recently collected specimen before the start of the study therapy and after the most recent systemic therapy.

Information on tissue collection procedures can be found in the Laboratory Manual.

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8.8.2. Blood Samples for Circulating Free Nucleic Acids Analyses

A blood sample for circulating free (cf) nucleic acids analyses will be collected pre-dose, at designated time points and processed into plasma as specified in the SoA.

The cf nucleic acids sample may be used 1) to assess genetic changes in ctDNA to evaluate ctDNA burden (as assessed by variant allele fraction or other methods) with response to study intervention, 2) to explore cfDNA alterations (eg, mutations, copy number variations) including changes with treatment in association with treatment outcome and to understand potential mechanisms of acquired resistance to study intervention; 3) to explore cfRNA transcripts, including splice variant transcripts, for androgen receptor and/or other genes, including changes with treatment and potential associations with outcome. A targeted sequencing panel may be used for these analyses. Exploratory whole exome/genome sequencing and/or epigenetic analyses may be performed.

8.8.3. Blood Samples for Matched Germline Control.

Blood samples will be collected from all participants on C1 D1 at predose. This sample may be used to confirm somatic vs germline origin of DNA alterations in tumor DNA or cell-free DNA.

8.8.4. Blood Samples for Serum Biomarker Research

A blood sample will be collected at pre-dose at designated time points and processed into serum as specified in the SoA. This sample may be used for circulating biomarker analysis, which may include protein analysis, nucleic acid analysis and/or other metabolomic analysis.

8.9. Immunogenicity Assessments

Immunogenicity assessments are not included in this study.

8.10. Health Economics

Health economics/medical resource utilization and health economics parameters are not evaluated in this study.

9. STATISTICAL CONSIDERATIONS

Detailed methodology for summary and statistical analyses of the data collected in this study is outlined here and further detailed in the SAP, which will be maintained by the sponsor. The SAP may modify what is outlined in the protocol where appropriate; however, any modifications of the primary endpoint definitions or their analyses will also be reflected in a protocol amendment.

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9.1. Statistical Hypotheses

The following statistical hypothesis will be tested to address the primary objective:

H_{01}: HR_{rPFS} =1 vs. H_{11}: HR_{rPFS} ≠1

where HR_{rPFS} is the HR (PF-06821497 and enzalutamide versus enzalutamide or docetaxel) of rPFS based on BICR assessment.

In addition, the following statistical hypotheses will be tested to address the key secondary objective:

H_{02}: HR_{OS} =1 vs. H_{12}: HR_{OS} ≠1

where HR_{OS} is the HR (PF-06821497 and enzalutamide versus enzalutamide or docetaxel) of OS.

The study will be considered positive if the null hypotheses H_{01} is rejected and HR_{rPFS} <1. A hierarchical gatekeeping procedure (testing strategy) will be used to test OS. Specifically, H_{02} will only be tested if H_{01} is rejected.

9.1.1. Estimands

9.1.1.1. Primary Estimand

rPFS: treatment effect, estimated in the analysis population, of the experimental arm on PFS based on BICR per RECIST v1.1 (soft tissue disease) and PCWG3 (bone disease) compared with the control arm from randomization to PD or death due to any cause.

- Variable: rPFS defined as the time from randomization until PD based on BICR assessment per RECIST v1.1 (soft tissue disease) and PCWG3 (bone disease), or death due to any cause, whichever occurs first.
- Censoring: rPFS data will be censored on the date of the last adequate tumor assessment for participants who do not have an event (PD based on BICR assessment per RECIST v1.1 [soft tissue disease] and PCWG3 [bone disease] or death due to any cause). Participants who start a new anticancer therapy with no event or with first event after the initiation of new anticancer therapy will be censored on the date of last adequate radiological assessment prior to initiation of new anti-cancer therapy. Participants with the first event right after 2 or more missing tumor assessments will be censored on the date of last adequate tumor assessment immediately prior to the event. Participants who do not have an adequate baseline tumor assessment or who do not have an adequate post-baseline tumor assessment will be censored on the date of randomization.
- Analysis population: all randomized participants regardless of tolerability, duration of study intervention.
- Population-level summary measure: log-rank p-value, HR for rPFS and 95% CI for HR calculated based on Cox's proportional hazard model stratified by the randomization strata and including data from all randomized participants.

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9.1.1.2. Secondary Estimands

OS: treatment effect, in the analysis population, of the experimental arm on OS compared with the control arm from randomization to the date of death due to any cause or the last known alive date.

- Variable: OS defined as the time from the date of randomization until the date of death due to any cause.
- Censoring: data for participants not known to have died are censored on the date of last known alive.
- Analysis population: all randomized participants. Survival status is expected to be collected irrespective of study intervention discontinuation or participant's request to discontinue study procedures. All participants who have not withdrawn consent for further participation in the study should be followed for survival until the end of the study.
- Population-level summary measure: log-rank p-value, HR for OS and 95% CI for HR calculated based on Cox's proportional hazard model stratified by the randomization strata and including data from all randomized participants.

9.1.2. Multiplicity Adjustment

Alpha protected efficacy analyses will include tests for the primary endpoint rPFS and key secondary endpoint OS. The type-1 error will be controlled by using a hierarchal testing approach. If the primary analysis of rPFS is successful, then OS will be tested using the full alpha of 0.05. If the primary analysis of rPFS is not successful, then formal testing of OS will not be done. In the 3-look group sequential design for OS, the Lan-DeMets (O'Brien-Fleming) α-spending function will be used to control of the alpha level in OS.

9.2. Analysis Sets

For purposes of analysis, the following analysis sets are defined.

Participant Analysis Set	Description
Full Analysis Set (FAS)	All participants randomly assigned to study intervention. Participants will be analyzed according to the study intervention assigned at randomization.
Safety	All participants randomly assigned to study intervention and who take at least 1 dose of any study intervention. Participants will be analyzed according to the study intervention assigned at randomization unless the incorrect treatment(s) was/were received throughout the dosing period in which case participants will be analyzed according to the actual study intervention received.
PK	All participants who received at least 1 dose of PF-06821497 and provided an evaluable PK sample.

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9.3. Statistical Analyses

The SAP will be developed and finalized before any analyses are performed and will describe the analyses and procedures for accounting for missing, unused, and spurious data. This section is a summary of the planned statistical analyses of the primary and secondary endpoints.

9.3.1. General Considerations

All analyses will be performed by treatment arm separately. All efficacy analyses will be performed using the FAS and all safety analyses will be performed using the safety analysis set.

In general, descriptive summaries will be presented for the efficacy and safety variables collected. Continuous variables will be summarized using mean, standard deviation, minimum, median, and maximum. Categorical variables will be summarized using frequency counts and percentages.

Unless otherwise specified, the calculation of proportions will be based on the sample size of the population of interest. Counts of missing observations will be included in the denominator and presented as a separate category if not otherwise specified in the SAP.

The rates of binary proportions for each treatment arm will be presented along with a 2-sided 95% CI. Time-to-event endpoints will be summarized using the Kaplan-Meier method and will include the median and 95% CIs based on the Brookmeyer-Crowley method.

Statistical analyses will be performed using SAS® version 9.4 or higher.

9.3.2. Primary Endpoint(s)/Estimand(s)/Analysis

rPFS is defined as the time from the date of randomization to first objective evidence of radiographic progression as assessed in soft tissue per RECIST v1.1 or in bone per PCWG3 guidelines by BICR, or death, whichever occurs first, and will be summarized in months using the following calculation:

rPFS (months) = [date of first objective evidence of radiographic progression (unconfirmed RECIST v1.1 or confirmed PCWG3), death, or censoring – randomization date +1]/30.4375.

The analyses of rPFS will be compared between the PF-06821497 in combination with enzalutamide and either enzalutamide or docetaxel by using a 2-sided stratified log-rank test. The stratified analysis will be based on the stratification factors used at randomization which include:

- Previous treatment with docetaxel in the mCSPC setting (yes vs. no)
- Physicians' choice of comparator (docetaxel vs enzalutamide)
- Presence of hepatic metastases (yes vs. no)

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The HR (experimental/control) and the associated 95% CI will be estimated using a Cox proportional hazards model is stratified by the randomization stratification factors. A hazard ratio of <1 indicates a prolonged time-to-event for participants randomized to the experimental arm compared with participants randomized to the control arm.

Kaplan-Meier curves will be used to estimate the time-to-event distributions. The 50th percentile of Kaplan-Meier estimates will be used to estimate the median duration of each treatment arm. A 2-sided 95% CI based on the Brookmeyer-Crowley method will be provided for this estimate.

Median follow-up time will be estimated according to the Kaplan-Meier estimate of potential follow-up by the reverse Kaplan-Meier method.

Efficacy analyses will be performed using the tumor assessments by the BICR as the primary data source.

Subgroup analyses for the primary endpoint will be performed for age group, geographic region, ECOG status and study randomization stratification factors. Other subgroup analyses may be prespecified in the SAP.

Sensitivity analyses for the primary endpoint will be performed as described below. Other sensitivity analyses may be pre-specified in the SAP.

- Analysis of rPFS counting all progression and deaths as events, regardless of missing assessments or timing of the events (ie, not censoring due to initiation of new anticancer therapy prior to event or due to 2 or more missed assessments)
- Analysis of rPFS counting unconfirmed bone progression as event
- Analysis of rPFS based on investigator assessment.

9.3.3. Secondary Endpoint(s)/Estimands Analysis

9.3.3.1. Key Secondary Endpoint

OS is a key secondary endpoint and is defined as the time from randomization to the date of death due to any cause. Participants last known to be alive will be censored at the date of last contact. A stratified log-rank test (2-sided) stratified by randomization stratification factors will be performed on OS. The Cox proportional hazards model stratified by randomization stratification factors will be fitted to compute the HR and the corresponding 95% CIs. OS associated with each treatment arm will be summarized using the Kaplan-Meier method and displayed graphically where appropriate. Subgroup analysis for OS will be performed at the time of the final OS analysis using the same subgroups specified for rPFS.

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9.3.3.2. Other Secondary Efficacy Endpoints

BICR assessed objective response in measurable soft tissue disease

The proportion of participants with measurable soft tissue disease at baseline who have a confirmed objective response of CR or PR per RECIST v1.1 will be summarized along with the 95% CI. Soft tissue responses must be confirmed by a follow-up radiographic assessment at least 4 weeks later with no evidence of confirmed bone disease progression on repeated bone scans at least 6 weeks apart per PCWG3 criteria. Participants without documented CR or PR will be considered non-responders. Patients without any post baseline assessment or patients who are NE due to other reasons will be considered non-responders. The primary evaluation of ORR will be based on BICR using derived response based on RECIST v1.1. Comparison in ORR between treatment arms will be performed using a Cochran-Mantel-Haenszel (CMH) test controlling for the randomization stratification factors. The p-value from the stratified CMH test will be reported. In addition, a two-sided 95% exact confidence interval for each treatment arm will be reported using the Clopper-Pearson method (1934) (Clopper & Pearson, 1934).

BICR assessed duration of response in measurable soft tissue disease

The time from the first objective evidence of a CR or PR (per BICR) to the first objective evidence of disease progression (assessed in soft tissue per RECIST v1.1 or in bone upon subsequent confirmation per PCWG3 guidelines) or death, whichever occurs first, and will be analyzed in the subset of participants entering with measurable soft tissue disease. Kaplan-Meier methods will be used to graph the duration and calculate the median and its associated 95% CI for each treatment arm.

Proportion of Participants with PSA response ≥50% (PSA Response)

The proportion of participants with a 50% decline from baseline in PSA that is confirmed by a second consecutive value at least 21 days later in participants with detectable PSA values at baseline will be calculated for each treatment arm. Participants without a baseline or without at least one post baseline PSA assessment will not be analyzed for this endpoint. Comparison in PSA response between treatment arms will be performed using a Cochran-Mantel-Haenszel (CMH) test controlling for the randomization stratification factors. The p-value from the stratified CMH test will be reported. In addition, a two-sided 95% exact confidence interval for each treatment arm will be reported using the Clopper-Pearson method (1934) (Clopper & Pearson, 1934).

Time to PSA progression

Time to PSA progression is defined as the time from randomization to the date of the first PSA value demonstrating progression, while participants are on study intervention which is subsequently confirmed at least 3 weeks later. If a participant starts a new anticancer therapy prior to PSA progression, the participant will be censored on the date of last PSA assessment prior to the start of the new anticancer therapy. Participants without confirmed PSA progression will be censored at the date of the last PSA assessment. Participants without any post baseline assessments will be censored at randomization. Time to PSA progression will be compared between treatment groups using a 2-sided stratified log-rank test. The stratified

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HR and 95% CI will be provided. The median time to event and 95% CI for the median will be provided for each treatment arm.

Time to initiation of new antineoplastic therapy

Time to first use of new antineoplastic therapy is defined as the time from randomization to first use of new antineoplastic therapy for prostate cancer. This will include medications used specifically for prostate cancer treatment including hormonal treatments, immunotherapy, chemotherapy, and investigative agents. Participants not starting treatment with a new antineoplastic therapy at the time of analysis will be censored at the date of last assessment before the analysis data cutoff date. It will be compared between treatment groups using a 2-sided stratified log rank test. The stratified HR and 95% CI will be provided. The median time to event and 95% CI for the median will be provided for each treatment arm.

Time to first symptomatic skeletal event

The time to first symptomatic skeletal event is defined as the time from randomization to the date of the first tumor related symptomatic bone fracture, surgery or radiotherapy to the bone, and spinal cord compression as reported on the Symptomatic Skeletal Events CRF page, whichever occurs first. Participants without any SSE will be censored at the date of last skeletal related event evaluation. Because skeletal events are expected to occur after radiographic progression, the analysis of time to first symptomatic skeletal event will be performed with the final OS analysis. It will be compared between treatment groups using a 2-sided log-rank test. The stratified HR and 95% CI will be provided. The median time to event and 95% CI for the median will be provided for each treatment arm.

PFS on next line therapy (PFS2)

PFS2 is defined as the time from the date of randomization to the first occurrence of investigator-determined disease progression (PSA progression, progression on imaging, or clinical progression) or death due to any cause, whichever occurs first, while the participant was receiving first subsequent therapy for prostate cancer. PFS2 will be compared between treatment groups using a 2-sided log-rank test. The stratified HR and 95% CI will be provided. The median PFS2 and 95% CI for the median will be provided for each treatment arm.

Assessment of the relationship between ctDNA burden and outcome

A multivariable Cox proportional-hazards model, or similar approach, will be used to assess the association of ctDNA burden with clinical outcome measures. Details of these analyses will be provided in a separate statistical analysis plan.

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9.3.3.2.1. Patient-Reported Outcome Analyses

The BPI-SF, FACT-P and EQ-5D-5L will be analyzed for the FAS analysis set. The PRO-CTCAE and FACT-GP5 will be analyzed for Safety analysis set specified in Section 9.2.

Descriptive summary statistics and longitudinal mixed effect-model analyses will be used to assess change from baseline in worst pain (Item #3), pain severity, interference in daily activities from pain as per BPI-SF, health status as per EQ-5D-5L, HRQoL, functioning and symptoms as per FACT-P between the two treatment groups. Any missing responses will be handled per the scoring manuals of each questionnaire administered. Compliance rates will be summarized by listing the numbers and proportions of participants who completed the PRO assessments at each timepoint by treatment arm.

PRO-CTCAE and FACT-GP5 scores (range: 0-4) will be presented descriptively (eg, summary statistics, graphical presentations). PRO-CTCAE and FACT-GP5 data will be collected, analyzed, and reported separately from CTCAE adverse event data, and will not be reconciled with each other.

Time to deterioration analyses will be conducted using a 2-sided stratified log-rank test. The stratified analysis will be based on the stratification factors used at randomization and summarized using the Kaplan-Meier method which will include the median and 95% CIs based on the Brookmeyer-Crowley method. In addition, stratified proportional hazard (Cox) regression model will be used to produce corresponding hazard ratio with 95% CI.

- Pain is measured using the BPI-SF Question #3. Confirmatory Deterioration in pain is defined as ≥2-point increase from the baseline pain score and confirmed at the next consecutive assessment ≥ 4 weeks apart or an initial deterioration followed by death before the next assessment (Dworkin et al, 2008).

- Definitive deterioration in patient-reported HRQoL is defined as ≥10 point decrease from baseline and no subsequent observations with a <10 point decrease from baseline FACT-P total score assessed by the FACT-P (Yost & Eton, 2005).

- Definitive deterioration in patient-reported physical well-being is defined as ≥3 point decrease from baseline and no subsequent observations with a <3 point decrease from baseline physical well-being score assessed by the FACT-P (Yost & Eton, 2005).

9.3.4. Exploratory Endpoint(s) Analysis

The details of exploratory endpoint analyses will be described in the SAP or in a separate exploratory analysis plan.

Results of exploratory endpoint analyses will be described in the CSR to the extent possible. Due to the exploratory nature of the endpoints, the associated data analyses may not be completed at the time of the CSR preparation. If results of exploratory endpoint analyses cannot be included in the CSR, they will be disseminated to the scientific community to the

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extent possible through presentation at scientific meetings and/or publication in peer-reviewed scientific journals.

Biomarkers will be assessed separately for tumor, blood, ctDNA, germline control, and cfRNA. Summaries of baseline levels, changes from baseline (where appropriate), expression, and mutation will be reported as applicable. For continuous variables, summary statistics may include the mean, ratio to baseline, standard deviation, median, interquartile range, % CV, and minimum/maximum levels of biomarker measures; for categorical variables, summary statistics may include number and percentage and odds ratio as appropriate.

Healthcare resource utilization for hospitalizations and length of stay may be summarized and listed.

9.3.5. Safety Analyses

Safety will be evaluated for the safety population using AE, laboratory, and vital signs data. Treatment-emergent safety data will be defined as events from the first dose of study intervention through approximately 28 days after the last dose, or upon initiation of a new antineoplastic therapy, whichever occurs first.

All safety analyses will be performed on the safety population.

AEs, ECGs, BP, PR, cardiac monitoring results, and safety laboratory data will be reviewed and summarized on an ongoing basis during the study to evaluate the safety of participants. Any clinical laboratory, ECG, BP, and PR abnormalities of potential clinical concern will be described. Safety data will be presented in tabular and/or graphical format and summarized descriptively, where appropriate.

Medical history and physical examination information, as applicable, collected during the course of the study will be considered source data and will not be required to be reported, unless otherwise noted. However, any untoward findings identified on physical examinations conducted during the active collection period will be captured as AEs, if those findings meet the definition of an AE. Data collected at screening that are used for inclusion/exclusion criteria, such as laboratory data, ECGs, and vital signs, will be considered source data, and will not be required to be reported, unless otherwise noted. Demographic data collected at screening will be reported.

9.3.5.1. Adverse Events

All analyses will be based on treatment-emergent events unless otherwise specified; events not considered treatment-emergent will be flagged in data listings.

All AEs will be coded to PT and SOC using current MedDRA. AEs will be presented with and without regard to causality based on the investigator's judgment and by frequency of overall toxicity categorized by NCI CTCAE (version 5.0) grades. Summary tables will include system organ class and/or preferred term; these will be described in the full SAP.

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In case a participant has events with missing and non-missing toxicity grades, the maximum of the non-missing grade will be displayed. Missing grade will only be displayed in the event that only one event has been reported for a participant and the grade is missing.

9.3.5.2. Laboratory Test Abnormalities

The number and percentage of participants who experienced laboratory test abnormalities will be summarized according to worst toxicity grade observed for each laboratory assay. The analyses will summarize laboratory tests both on the entire study period and by cycle (Cycle 1 and Cycles beyond 1). For laboratory tests without CTCAE grade definitions, results will be categorized as normal, abnormal, or not done.

9.3.6. PK Analyses

PK analyses will be performed using the PK analysis set.

PK data analyses will include descriptive summary statistics of the pre-dose and post-dose plasma concentrations of PF-06821497 by study visit and treatment arm.

In addition, the PK data from this study may be used alone or combined with PK data from other studies, to develop a population PK model. The correlations between PF-06821497 exposure parameters in combination with enzalutamide and biomarker, efficacy and safety endpoints will be explored if data allows. The results of these modeling analyses will be reported separately from the CSR.

9.3.7. Other Analyses

Pharmacogenomic or biomarker data from Retained Research Samples may be collected during or after the trial and retained for future analyses; the results of such analyses are not planned to be included in the CSR.

9.4. Interim Analyses

Table 9. Summary of Planned Analyses Time Points

	Analysis Cut off Trigger	**Analysis**
Final rPFS analysis	Approximately 302 rPFS events	Final rPFS analysis Interim OS analysis #1
Interim Analysis 2 of OS	Approximately 271 OS events	Interim OS analysis #2
Final Analysis of OS	Approximately 386 OS events	Final OS analysis

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There will be no interim analysis for the primary endpoint (rPFS).

Interim efficacy analysis #1 for OS will be performed at the time of the final analysis of rPFS (PCD). It is anticipated that approximately 137 deaths (approximately 36% of the planned information fraction) will have occurred at the time of the interim OS analysis. The efficacy boundary will be determined using a Lan-DeMets α-spending function. With 137 OS events the efficacy boundary will be crossed if the two-sided p value is ≤ 0.0003. The boundary will be adjusted according to the number of observed events.

There will be a second OS interim analysis at approximately 271 OS events (70% of the planned information fraction). The efficacy boundary will be determined using a Lan-DeMets α-spending function. It is estimated that the efficacy boundary will be crossed if the two-sided p value is ≤ 0.0147. The exact efficacy boundary will be updated prior to the time of analysis based on the actual number of events.

Before any interim analysis is performed, the details of the objectives, decision criteria, dissemination plan, and method of maintaining the study blind (if applicable) as per Pfizer's SOPs will be documented and approved in a DMC charter. In addition, the analysis details will be documented and approved in the SAP.

For additional information on which procedures may be stopped when the required number of OS events have occurred (See Section 10.1.10).

9.5. Sample Size Determination

Approximately 600 participants will be enrolled in the study and randomized 1:1 to the 2 treatments arms (ie, 300 participants per arm).

A total of 302 rPFS events will provide approximately 95% power to detect a target HR of 0.66 using a 2-sided log-rank test with a 5% level of significance. The median rPFS in the control arm is assumed to be approximately 6.75 months (Tannock et al, 2004). Assuming an exponential distribution for rPFS with proportional hazards, it is expected that the experimental arm will result in an increase in median rPFS to approximately 10.2 months. Assuming a nonuniform enrollment period of approximately 19 months, and a lost to follow-up rate of 15% by 12 months for rPFS endpoint, the projected time from the first patient enrollment to reaching the target total number of rPFS events is approximately 20 months.

For OS, a total of 386 events will provide approximately 80% power to detect a target HR of 0.75 using a 2-sided log-rank test with a 5% level of significance and a 3-look group sequential design with Lan-DeMets (O'Brien-Fleming) α-spending function to determine the efficacy boundaries. The median OS in the control arm is assumed to be approximately 22 months (Tannock et al, 2004). Assuming an exponential distribution for OS with proportional hazards, it is expected that the experimental arm will result in an increase in median OS to approximately 29.3 months.

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To control the overall Type I error rate for the study, a hierarchical testing approach will be used, whereby rPFS will be formally tested first. If it is significant, then OS will be tested at an overall 2-sided 5% level of significance.

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10. SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

10.1.1. Regulatory and Ethical Considerations

This study will be conducted in accordance with the protocol and with the following:

- Consensus ethical principles derived from international guidelines, including the Declaration of Helsinki and CIOMS International Ethical Guidelines;
- Applicable ICH GCP guidelines;
- Applicable laws and regulations, including applicable privacy laws.

The protocol, protocol amendments, ICD, SRSD(s), and other relevant documents (eg, advertisements) must be reviewed and approved by the sponsor, submitted to an IRB/EC by the investigator, and reviewed and approved by the IRB/EC before the study is initiated.

Any amendments to the protocol will require IRB/EC approval before implementation of changes made to the study design, except for changes necessary to eliminate an immediate hazard to study participants.

Protocols and any substantial amendments to the protocol will require health authority approval prior to initiation except for changes necessary to eliminate an immediate hazard to study participants.

The investigator will be responsible for the following:

- Providing written summaries of the status of the study to the IRB/EC annually or more frequently in accordance with the requirements, policies, and procedures established by the IRB/EC.
- Notifying the IRB/EC of SAEs or other significant safety findings as required by IRB/EC procedures.
- Providing oversight of the conduct of the study at the site and adherence to requirements of 21 CFR, ICH GCP guidelines, the IRB/EC, European regulation 536/2014 for clinical studies, European Medical Device Regulation 2017/745 for clinical device research, and all other applicable local regulations.

10.1.1.1. Reporting of Safety Issues and Serious Breaches of the Protocol or ICH GCP

In the event of any prohibition or restriction imposed (ie, clinical hold) by an applicable regulatory authority in any area of the world, or if the investigator is aware of any new information that might influence the evaluation of the benefits and risks of the study intervention, Pfizer should be informed immediately.

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In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of the ICH GCP that the investigator becomes aware of.

10.1.2. Financial Disclosure

Investigators and sub investigators will provide the sponsor with sufficient, accurate financial information as requested to allow the sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the course of the study and for 1 year after completion of the study.

10.1.3. Informed Consent Process

The investigator or qualified designee will explain the nature of the study, including the risks and benefits, to the participant and answer all questions regarding the study. The investigator may not delegate obtaining consent to a local healthcare professional.

The participant should be given sufficient time and opportunity to ask questions and to decide whether or not to participate in the trial.

Participants must be informed that their participation is voluntary. Participants will be required to sign a statement of informed consent that meets the requirements of 21 CFR 50, local regulations, ICH GCP guidelines, privacy and data protection requirements, where applicable, and the IRB/EC or study center.

The investigator must ensure that each study participant is fully informed about the nature and objectives of the study, the sharing of data related to the study, and possible risks associated with participation, including the risks associated with the processing of the participant's personal data.

The participant must be informed that their personal study-related data will be used by the sponsor in accordance with local data protection law. The level of disclosure must also be explained to the participant.

The participant must be informed that their medical records may be examined by Clinical Quality Assurance auditors or other authorized personnel appointed by the sponsor, by appropriate IRB/EC members, and by inspectors from regulatory authorities.

The investigator further must ensure that each study participant is fully informed about his or her right to access and correct their personal data and to withdraw consent for the processing of his or her personal data.

The medical record must include a statement that written informed consent was obtained before the participant was enrolled in the study and the date on which the written consent was obtained. The authorized person obtaining the informed consent must also sign the ICD.

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Participants must be reconsented to the most current IRB/EC version of the IRB/EC-approved ICD(s) during their participation in the study as required per local regulations.

A copy of the ICD(s) must be provided to the participant.

A participant who is rescreened is not required to sign another ICD if the rescreening occurs within 28 days from the previous ICD signature date.

10.1.4. Data Protection

All parties will comply with all applicable laws, including laws regarding the implementation of organizational and technical measures to ensure protection of participant data.

Participants' personal data will be stored at the study site in encrypted electronic and/or paper form and will be password-protected or secured in a locked room to ensure that only authorized study staff have access. The study site will implement appropriate technical and organizational measures to ensure that the personal data can be recovered in the event of disaster. In the event of a potential personal data breach, the study site will be responsible for determining whether a personal data breach has in fact occurred and, if so, providing breach notifications as required by law.

To protect the rights and freedoms of participants with regard to the processing of personal data, participants will be assigned a single, participant-specific numerical code. Any participant records or data sets that are transferred to the sponsor will contain the numerical code; participant names will not be transferred. All other identifiable data transferred to the sponsor will be identified by this single, participant-specific code. The study site will maintain a confidential list of participants who participated in the study, linking each participant's numerical code to their actual identity and medical record ID. In case of data transfer, the sponsor will protect the confidentiality of participants' personal data consistent with the clinical study agreement and applicable privacy laws.

Information technology systems used to collect, process, and store study-related data are secured by technical and organizational security measures designed to protect such data against accidental or unlawful loss, alteration, or unauthorized disclosure or access.

The sponsor maintains SOPs on how to respond in the event of unauthorized access, use, or disclosure of sponsor information or systems.

10.1.5. Committees Structure

10.1.5.1. Data Monitoring Committee

This study will use an E-DMC. The E-DMC is independent of the study team and includes only external members. The E-DMC charter describes the role of the E-DMC in more detail.

The E-DMC will be responsible for ongoing monitoring of the safety of participants in the study according to the charter. The recommendations made by the E-DMC will be forwarded to the appropriate authorized Pfizer personnel for review and final decision. Pfizer will

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communicate such decisions, which may include summaries of aggregate analyses of safety data, to regulatory authorities and investigators, as appropriate.

10.1.6. Dissemination of Clinical Study Data

Pfizer fulfills its commitment to publicly disclose clinical study results through posting the results of studies on www.clinicaltrials.gov (ClinicalTrials.gov), the EudraCT/CTIS, and/or www.pfizer.com, and other public registries and websites in accordance with applicable local laws/regulations. In addition, Pfizer reports study results outside of the requirements of local laws/regulations pursuant to its SOPs.

In all cases, study results are reported by Pfizer in an objective, accurate, balanced, and complete manner and are reported regardless of the outcome of the study or the country in which the study was conducted.

www.clinicaltrials.gov

Pfizer posts clinical trial results on www.clinicaltrials.gov for Pfizer-sponsored interventional studies (conducted in participants) that evaluate the safety and/or efficacy of a product, regardless of the geographical location in which the study is conducted. These results are submitted for posting in accordance with the format and timelines set forth by US law.

<u>EudraCT/CTIS</u>

Pfizer posts clinical trial results on EudraCT/CTIS for Pfizer-sponsored interventional studies in accordance with the format and timelines set forth by EU requirements following the end of the study globally.

www.pfizer.com

Pfizer posts CSR synopses and plain-language study results summaries on www.pfizer.com for Pfizer-sponsored interventional studies at the same time the corresponding study results are posted to www.clinicaltrials.gov. CSR synopses will have personally identifiable information anonymized.

<u>Documents within marketing applications</u>

Pfizer complies with applicable local laws/regulations to publish clinical documents included in marketing applications. Clinical documents include summary documents and CSRs including the protocol and protocol amendments, sample CRFs, and SAPs. Clinical documents will have personally identifiable information anonymized.

<u>Data sharing</u>

Pfizer provides researchers secure access to participant-level data or full CSRs for the purposes of “bona-fide scientific research” that contributes to the scientific understanding of the disease, target, or compound class. Pfizer will make data from these trials available 18 months after study completion. Participant-level data will be anonymized in accordance

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with applicable privacy laws and regulations. CSRs will have personally identifiable information anonymized.

Data requests are considered from qualified researchers with the appropriate competencies to perform the proposed analyses. Research teams must include a biostatistician. Data will not be provided to applicants with significant conflicts of interest, including individuals requesting access for commercial/competitive or legal purposes.

10.1.7. Data Quality Assurance

All participant data relating to the study will be recorded on printed or electronic CRF unless transmitted to the sponsor or designee electronically (eg, laboratory data). The investigator is responsible for verifying that data entries are accurate and correct by physically or electronically signing the CRF.

Guidance on completion of CRFs will be provided in the CRF Completion Requirements document.

The investigator must ensure that the CRFs are securely stored at the study site in encrypted electronic and/or paper form and are password-protected or secured in a locked room to prevent access by unauthorized third parties.

QTLs are predefined parameters that are monitored during the study. Important deviations from the QTLs and any remedial actions taken will be summarized in the CSR.

The investigator must permit study-related monitoring, audits, IRB/EC review, and regulatory agency inspections and provide direct access to source records and documents. This verification may also occur after study completion. It is important that the investigator(s) and their relevant personnel are available during the monitoring visits and possible audits or inspections and that sufficient time is devoted to the process.

Monitoring details describing strategy, including definition of study-critical data items and processes (eg, risk-based initiatives in operations and quality, such as risk management and mitigation strategies and analytical risk-based monitoring), methods, responsibilities, and requirements, including handling of noncompliance issues and monitoring techniques (central, virtual, or on-site monitoring), are provided in the data management plan and monitoring plan maintained and utilized by the sponsor or designee.

The sponsor or designee is responsible for the data management of this study, including quality checking of the data.

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for up to 15 years after study completion unless local regulations or institutional policies require a longer retention period. No records may be destroyed during the retention period without the written approval of the sponsor. No records may be transferred to another location or party without written notification to the sponsor.

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The investigator must ensure that the records continue to be stored securely for as long as they are maintained.

When participant data are to be deleted, the investigator will ensure that all copies of such data are promptly and irrevocably deleted from all systems.

The investigator(s) will notify the sponsor or its agents immediately of any regulatory inspection notification in relation to the study. Furthermore, the investigator will cooperate with the sponsor or its agents to prepare the investigator site for the inspection and will allow the sponsor or its agent, whenever feasible, to be present during the inspection. The investigator site and investigator will promptly resolve any discrepancies that are identified between the study data and the participant's medical records. The investigator will promptly provide copies of the inspection findings to the sponsor or its agent. Before response submission to the regulatory authorities, the investigator will provide the sponsor or its agents with an opportunity to review and comment on responses to any such findings.

10.1.8. Source Documents

Source documents provide evidence for the existence of the participant and substantiate the integrity of the data collected. Source documents are filed at the investigator site.

Data reported on the CRF or entered in the eCRF that are from source documents must be consistent with the source documents or the discrepancies must be explained. The investigator may need to request previous medical records or transfer records, depending on the study. Also, current medical records must be available.

Definition of what constitutes source document, and its origin can be found in the Source Document Locator, which is maintained by the sponsor.

Description of the use of the computerized system is documented in the Data Management Plan, which is maintained by the sponsor.

The investigator must maintain accurate documentation (source record) that supports the information entered in the CRF.

The sponsor or designee will perform monitoring to confirm that data entered into the CRF by authorized site personnel are accurate, complete, and verifiable from source documents; that the safety and rights of participants are being protected; and that the study is being conducted in accordance with the currently approved protocol and any other study agreements, the ICH GCP guidelines, and all applicable regulatory requirements.

10.1.9. Use of Medical Records

There may be instances when copies of medical records for certain cases are requested by Pfizer Safety, where ethically and scientifically justified and permitted by local regulations, to ensure participant safety.

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Due to the potential for a participant to be re-identified from their medical records, the following actions must be taken when medical records are sent to the sponsor or sponsor designee:

- The investigator or site staff must redact personal information from the medical record. The personal information includes, but is not limited to, the following: participant names or initials, participant dates (eg, birth date, date of hospital admission/discharge, date of death), participant identification numbers (eg, Social Security number, health insurance number, medical record number, hospital/institution identifier), participant location information (eg, street address, city, country, postal code, IP address), participant contact information (eg, telephone/fax number, email address).

- Each medical record must be transmitted to the sponsor or sponsor designee using systems with technical and organizational security measures to ensure the protection of personal data (eg, Florence is the preferred system if available).

- There may be unplanned situations where the sponsor may request medical records (eg, sharing medical records so that the sponsor can provide study-related advice to the investigator). The medical records should be submitted according to the procedure described above.

10.1.10. Study and Site Start and Closure

The study start date is the date of the first participant's first visit.

The sponsor designee reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor, including (but not limited to) regulatory authority decision, change in opinion of the IRB/EC, or change in benefit-risk assessment. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time upon notification to the sponsor or designee/CRO if requested to do so by the responsible IRB/EC or if such termination is required to protect the health of study participants.

Reasons for the early closure of a study site by the sponsor may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IRB/EC or local health authorities, the sponsor's procedures, or the ICH GCP guidelines;

- Inadequate recruitment of participants by the investigator;

- Discontinuation of further study intervention development.

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If the study is prematurely terminated or suspended, the sponsor shall promptly inform the investigators, the ECs/IRBs, the regulatory authorities, and any CRO(s) used in the study of the reason for termination or suspension, as specified by the applicable regulatory requirements. The investigator shall promptly inform the participant and should assure appropriate participant therapy and/or follow-up.

If the sponsor decides to terminate the study for a reason unrelated to the safety of PF-06821497, participants may continue to receive study intervention(s) per the investigator's judgment and protocol-specified safety assessments will continue to be performed for these participants until the end of the study as defined in Section 4.4.

The following non safety-related study procedures and assessments may be stopped upon written notification from the sponsor.

- The procedures may also be stopped when the required number of OS events has occurred per Section 9.4.
- Radiographic imaging and PSA assessment will no longer be required after sponsor communication once the PCD has been achieved (Section 9.4).
- LTFU will be no longer be required after sponsor confirms that the desired number of OS events for the final OS analysis has been achieved (Section 9.4).

Study termination is also provided for in the clinical study agreement. If there is any conflict between the contract and this protocol, the contract will control as to termination rights.

10.1.11. Publication Policy

For multicenter trials, the primary publication will be a joint publication developed by the investigator and Pfizer reporting the primary endpoint(s) of the study covering all study sites. The investigator agrees to refer to the primary publication in any subsequent publications. Pfizer will not provide any financial compensation for the investigator's participation in the preparation of the primary congress abstract, poster, presentation, or primary manuscript for the study.

Investigators are free to publish individual center results that they deem to be clinically meaningful after publication of the overall results of the study or 12 months after PCD or study completion at all sites, whichever occurs first, subject to the other requirements described in this section.

The investigator will provide Pfizer an opportunity to review any proposed publication or any other type of disclosure of the study results (collectively, "publication") before it is submitted or otherwise disclosed and will submit all publications to Pfizer 30 days before submission. If any patent action is required to protect intellectual property rights, the investigator agrees to delay the disclosure for a period not to exceed an additional 60 days upon request from Pfizer. This allows Pfizer to protect proprietary information and to provide comments, and the investigator will, on request, remove any previously undisclosed

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confidential information before disclosure, except for any study-intervention or Pfizer-related information necessary for the appropriate scientific presentation or understanding of the study results. For joint publications, should there be disagreement regarding interpretation and/or presentation of specific analysis results, resolution of, and responsibility for, such disagreements will be the collective responsibility of all authors of the publication.

For all publications relating to the study, the investigator and Pfizer will comply with recognized ethical standards concerning publications and authorship, including those established by the International Committee of Medical Journal Editors. The investigator will disclose any relationship with Pfizer and any relevant potential conflicts of interest, including any financial or personal relationship with Pfizer, in any publications. When appliable, editorial or technical support provided by a third party and paid for by Pfizer, or provided by a Pfizer employee, may be a reportable transfer of value under the Sunshine Act for US licensed physicians or other healthcare professionals. All authors will have access to the relevant statistical tables, figures, and reports (in their original format) required to develop the publication.

10.1.12. Sponsor's Medically Qualified Individual

The sponsor will designate a medically qualified individual (MQI, also known as the medical monitor) to advise the investigator on study-related medical questions. The contact information for the study medical monitor is documented in the Study Team Contact List located in the Investigator Site File.

Participants are provided with a Pfizer study information card at the time of informed consent, which includes contact information for their investigator in case of study-related medical questions. The study information card contains, at a minimum, (a) study number, (b) participant's study identification number, and (c) principal investigator contact information.

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10.2. Appendix 2: Clinical Laboratory Tests

The following safety laboratory tests will be performed at times defined in the SoA section of this protocol. Additional laboratory results may be reported on these samples as a result of the method of analysis or the type of analyzer used by the clinical laboratory, or as derived from calculated values. These additional tests would not require additional collection of blood. Unscheduled clinical laboratory measurements may be obtained at any time during the study to assess any perceived safety issues.

Table 10. Core Lab Tests

Hematology	Chemistry	Additional
Hemoglobin	Albumin	PSA
Platelets	Total protein Alkaline phosphatase ALT AST Total bilirubin*** BUN or Urea Creatinine Calcium Chloride LDH Phosphate Potassium Sodium Cystatin C[a]	Serum Testosterone (screening only)
WBC with differential ANC Lymphocytes Monocytes Eosinophils Basophils		Consider obtaining a CD4 count (unscheduled- if the lymphocyte count ≤500/µL)

***For Hy's law potential cases, in addition to repeating AST and ALT, laboratory tests should include albumin, creatine kinase, total bilirubin, direct and indirect bilirubin, gamma-glutamyl transferase, PT/INR, and alkaline phosphatase
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a. Cystatin C (Scys): Screening or Baseline Scys is recommended to help differentiate post-baseline DIKI from DICI. Post-baseline, Scys is measured if and only if serum creatinine increase post-baseline is observed (See Section 10.7.1)

The investigator must review the laboratory report, document this review, and record any clinically relevant changes occurring during the study in the AE section of the CRF.

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10.3. Appendix 3: Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

10.3.1. Definition of AE

AE Definition
• An AE is any untoward medical occurrence in a patient or clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention. • Note: An AE can therefore be any unfavorable and unintended sign (including an abnormal laboratory finding), symptom, or disease (new or exacerbated) temporally associated with the use of study intervention.

Events <u>Meeting</u> the AE Definition

• Any abnormal laboratory test results (hematology, clinical chemistry, or urinalysis) or other safety assessments (eg, ECG, radiological scans, vital sign measurements), including those that worsen from baseline, considered clinically significant in the medical and scientific judgment of the investigator. Any abnormal test results that meet any of the conditions below must be recorded as an AE:
 	
  • Is associated with accompanying symptoms;
 	
  • Requires additional diagnostic testing or medical/surgical intervention;
 	
  • Leads to a change in study dosing (outside of any protocol-specified dose adjustments) or discontinuation from the study, significant additional concomitant drug treatment, or other therapy.
 	
• Exacerbation of a chronic or intermittent preexisting condition, including an increase in either frequency and/or intensity of the condition.
 	
• New condition detected or diagnosed after study intervention administration, even though it may have been present before the start of the study.
 	
• Signs, symptoms, or the clinical sequelae of a suspected drug-drug interaction.
 	
• Signs, symptoms, or the clinical sequelae of a suspected overdose of either study intervention or a concomitant medication. Overdose per se will not be reported as an AE or SAE unless it is an intentional overdose taken with possible suicidal/self-harming intent. Such overdoses should be reported regardless of sequelae.

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Events NOT Meeting the AE Definition
• Any clinically significant abnormal laboratory findings or other abnormal safety assessments that are associated with the underlying disease, unless judged by the investigator to be more severe than expected for the participant's condition. • The disease/disorder being studied or expected progression, signs, or symptoms of the disease/disorder being studied, unless more severe than expected for the participant's condition. • Worsening of signs and symptoms of the malignancy under study should be recorded as AEs in the appropriate section of the CRF. Disease progression assessed by measurement of malignant lesions on radiographs or other methods should not be reported as AEs. • Medical or surgical procedure (eg, endoscopy, appendectomy): the condition that leads to the procedure is the AE. • Situations in which an untoward medical occurrence did not occur (social and/or convenience admission to a hospital). • Anticipated day-to-day fluctuations of preexisting disease(s) or condition(s) present or detected at the start of the study that do not worsen.

10.3.2. Definition of an SAE

An SAE is defined as any untoward medical occurrence that, at any dose, meets 1 or more of the criteria listed below:
a. Results in death
b. Is life-threatening The term "life-threatening" in the definition of "serious" refers to an event in which the participant was at risk of death at the time of the event. It does not refer to an event that hypothetically might have caused death if it were more severe.
c. Requires inpatient hospitalization or prolongation of existing hospitalization In general, hospitalization signifies that the participant has been detained (usually involving at least an overnight stay) at the hospital or emergency ward for observation and/or treatment that would not have been appropriate in the physician's office or outpatient setting. Complications that occur during hospitalization are AEs. If a complication prolongs hospitalization or fulfills any other serious criteria, the event is

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	serious. When in doubt as to whether "hospitalization" occurred or was necessary, the AE should be considered serious. Hospitalization for elective treatment of a preexisting condition that did not worsen from baseline is not considered an AE.
d.	**Results in persistent or significant disability/incapacity** • The term disability means a substantial disruption of a person's ability to conduct normal life functions. • This definition is not intended to include experiences of relatively minor medical significance, such as uncomplicated headache, nausea, vomiting, diarrhea, influenza, and accidental trauma (eg, sprained ankle), that may interfere with or prevent everyday life functions but do not constitute a substantial disruption.
e.	**Is a congenital anomaly/birth defect.**
f.	**Is a suspected transmission via a Pfizer product of an infectious agent, pathogenic or non-pathogenic.** The event may be suspected from clinical symptoms or laboratory findings indicating an infection in a participant exposed to a Pfizer product. The terms "suspected transmission" and "transmission" are considered synonymous. These cases are considered unexpected and handled as serious expedited cases by pharmacovigilance personnel. Such cases are also considered for reporting as product defects, if appropriate.
g.	**Other situations:** • Medical or scientific judgment should be exercised by the investigator in deciding whether SAE reporting is appropriate in other situations, such as significant medical events that but may jeopardize the participant or may require medical or surgical intervention to prevent one of the other outcomes listed in the above definition. These events should usually be considered serious. • Examples of such events include invasive or malignant cancers, intensive treatment in an emergency room or at home for allergic bronchospasm, blood dyscrasias or convulsions that do not result in hospitalization, or development of drug dependency or drug abuse. • Progression of the malignancy under study (including signs and symptoms of progression) should not be reported as an SAE unless the outcome is fatal within the active collection period. Hospitalization due to signs and symptoms of disease progression should not be reported as an SAE. If the malignancy has a fatal outcome during the study, the event leading to death must be recorded as an AE on

the CRF, and as an SAE with CTCAE Grade 5 (See the Assessment of Severity section) if it occurs during the active collection period.

10.3.3. Recording/Reporting and Follow-Up of AEs and/or SAEs During the Active Collection Period

AE and SAE Recording/Reporting

The table below summarizes the requirements for recording AEs on the CRF and for reporting SAEs to Pfizer Safety throughout the active collection period. These requirements are delineated for 3 types of events: (1) SAEs; (2) nonserious AEs; and (3) exposure to the study intervention under study during pregnancy or breastfeeding, and occupational exposure.

It should be noted that reporting SAE information to Pfizer Safety is not the same as recording on the AE page of the CRF. When the same data are collected, these 2 reporting methods must be completed in a consistent manner. AEs should be recorded using concise medical terminology and the same AE term should be used on both the CRF and the reporting method used for reporting of SAE information to Pfizer Safety.

Safety Event	Recorded on the CRF	Reported to Pfizer Safety	Reported Within 24 Hours of Awareness
SAE	All	All	Yes
Nonserious AE	All	None All nonserious AEs assessed as causally related to the NIMP/AxMP	No
Exposure to the study intervention under study during pregnancy or breastfeeding	All AEs/SAEs associated with EDP **Note:** Instances of EDP not associated with an AE or SAE are captured on the CRF,	All instances of EDP are reported (whether or not there is an associated SAE)) All instances of EDB are reported (whether or not there is an associated SAE)	Yes

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Environmental or occupational exposure to the product under study to a non-participant (not involving EDP or EDB)	None. Exposure to a study non-participant is not collected on the CRF	The exposure (whether or not there is an associated AE or SAE) must be reported	Yes

- When an AE or SAE occurs, it is the responsibility of the investigator to review all documentation (eg, hospital progress notes, laboratory reports, and diagnostic reports) related to the event.
- The investigator will then record all relevant AE or SAE information on the CRF.
- It is **not** acceptable for the investigator to send photocopies of the participant's medical records to Pfizer Safety when the reporting method is paper (eg, in lieu of completion of the CT SAE Report Form/AE or SAE CRF page).
- There may be instances when copies of medical records for certain cases are requested by Pfizer Safety. In this case, all participant identifiers, with the exception of the participant number, will be redacted on the copies of the medical records before submission to Pfizer Safety. Refer to Section 10.1.9 for actions that must be taken when medical records are sent to the sponsor or sponsor designee.
- The investigator will attempt to establish a diagnosis of the event based on signs, symptoms, and/or other clinical information. Whenever possible, the diagnosis (not the individual signs/symptoms) will be documented as the AE or SAE.

Assessment of Intensity

- The investigator will make an assessment of intensity for each AE and SAE reported during the study and assign it to 1 of the categories listed below (as defined by the NCI CTCAE system).

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If required on the AE page of the CRF, the investigator will use the following definitions of severity in accordance with the current CTCAE version to describe the maximum intensity of the AE:	
GRADE	Clinical Description of Intensity
1	MILD AE
2	MODERATE AE
3	SEVERE AE
4	LIFE-THREATENING; urgent intervention indicated
5	DEATH RELATED TO AE
An event is defined as "serious" when it meets at least 1 of the predefined outcomes as described in the definition of an SAE, NOT when it is rated as severe.	

Assessment of Causality

- The investigator is obligated to assess the relationship between study intervention and each occurrence of each AE or SAE. The investigator will use clinical judgment to determine the relationship.
- A "reasonable possibility" of a relationship conveys that there are facts, evidence, and/or arguments to suggest a causal relationship, rather than a relationship cannot be ruled out.
- Once the causality assessment is determined, the investigator must document it in the CRF and, for safety reporting purposes, document the relatedness as "Yes" (for related) or "No" (for unrelated).
- Alternative causes, such as underlying disease(s), concomitant therapy, and other risk factors, as well as the temporal relationship of the event to study intervention administration, will be considered and investigated.
- The investigator will also consult the IB and/or product information, for marketed products, in their assessment.
- For each AE or SAE, the investigator **must** document in the medical notes that they have reviewed the AE or SAE and have provided an assessment of causality.
- There may be situations in which an SAE has occurred and the investigator has minimal information to include in the initial report to the sponsor. However, **it is**

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very important that the investigator always make an assessment of causality for every event before the initial transmission of the SAE data to the sponsor.

- The investigator may change their opinion of causality in light of follow-up information and send an SAE follow-up report with the updated causality assessment.
- The causality assessment is one of the criteria used when determining regulatory reporting requirements.
- If the investigator does not know whether or not the study intervention caused the event, then the event will be handled as "related to study intervention" for reporting purposes, as defined by the sponsor. In addition, if the investigator determines that an SAE is associated with study procedures, the investigator must record this causal relationship in the source documents and CRF, and report such an assessment in the dedicated section of the selected reporting method and in accordance with the SAE reporting requirements.

Outcome Assessment

Record the appropriate outcome of the event in relation to the participant's status:

- Fatal: The termination of life as a result of an AE.
- Not Recovered/Not Resolved: One of the possible results of an AE outcome that indicates that the event has not improved or recuperated.
- Recovered/Resolved: One of the possible results of an AE outcome that indicates that the event has improved or recuperated.
- Recovered/Resolved with Sequalae: One of the possible results of an AE outcome where the participant recuperated but retained pathological conditions resulting from the prior disease or injury.
- Recovering/Resolving: One of the possible results of an AE outcome that indicates that the event is improving.
- Unknown: Not known, not observed, not recorded, or refused.

Follow-Up of AEs and SAEs

- The investigator is obligated to perform or arrange for the conduct of supplemental measurements and/or evaluations, as medically indicated or as requested by the sponsor, to elucidate the nature and/or causality of the AE or SAE as fully as possible. This may include additional laboratory tests or

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investigations, histopathological examinations, or consultation with other healthcare providers.

- If a participant dies during participation in the study or during a recognized follow-up period, the investigator will provide Pfizer Safety with a copy of any postmortem findings, including histopathology. New or updated information will be recorded in the originally submitted documents.

- The investigator will submit any updated SAE data to the sponsor within 24 hours of receipt of the information.

10.3.4. Reporting of SAEs

SAE Reporting to Pfizer Safety via an Electronic Data Capture

- The primary mechanism for reporting an SAE to Pfizer Safety will be the EDC (eg, eSAE or SAE+) or PSSA.

- If the electronic system is unavailable, then the site will use the paper SAE report form (See next section) to report the event within 24 hours.

- The site will enter the SAE data into the EDC or PSSA or paper form (as applicable) as soon as the data become available.

- After the study is completed at a given site, the EDC will be taken off-line to prevent the entry of new data or changes to existing data.

- If a site receives a report of a new SAE from a study participant or receives updated data on a previously reported SAE after the EDC has been taken off-line, then the site can report this information on a paper SAE form (See next section) or to Pfizer Safety by telephone.

SAE Reporting to Pfizer Safety via the CT SAE Report Form

- Facsimile transmission of the CT SAE Report Form is the back-up method to transmit this information to Pfizer Safety in case the EDC or PSSA is unavailable for more than 24 hours.

- In circumstances when the facsimile is not working, an alternative method should be used, eg, secured (Transport Layer Security) or password-protected email. If none of these methods can be used, notification by telephone is acceptable with a copy of the CT SAE Report Form sent by overnight mail or courier service.

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- Initial notification via telephone does not replace the need for the investigator to complete and sign the CT SAE Report Form pages within the designated reporting time frames.

- When environmental/occupational exposure occurs, the exposure information does not pertain to the participant enrolled in the study; therefore, it must be reported using the CT SAE Report Form. If an EDP occurs in the setting of environmental exposure, the EDP Supplemental Form must be used in addition to the CT SAE Report Form.

- When EDB occurs (either for a study participant or in the setting of environmental exposure), since the exposure information pertains to the breastfed baby, the information must be reported using the CT SAE Report Form.

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10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Male Participant Reproductive Inclusion Criteria

Male participants **receiving docetaxel** are eligible to participate if they agree to the following requirements during the study intervention period and for at least 4 months after the last dose of study intervention, which corresponds to the time needed to eliminate reproductive safety risk of the study intervention(s) ***plus*** an additional 90 days (a spermatogenesis cycle):

- Refrain from donating sperm.

PLUS either:

- Be persistently abstinent from heterosexual intercourse and agree to remain abstinent.

  OR

  - Must agree to use a male condom when engaging in any activity that allows for passage of ejaculate to another person.
  - The participant should be advised of the benefit for an IOCBP partner using a highly effective method of contraception with a failure rate of <1% per year, as described in Section 10.4.2.

Male participants receiving PF-06821497/enzalutamide are eligible to participate if they agree to the following requirements during the study intervention period and for at least 3 months after the last dose of study intervention, which corresponds to the time needed to eliminate reproductive safety risk of the study intervention

- Refrain from donating sperm.

PLUS either:

- Be abstinent from heterosexual or homosexual intercourse as their preferred and usual lifestyle (abstinent on a long-term and persistent basis) and agree to remain abstinent.

  OR

  - Must agree to use a male condom when engaging in any activity that allows for passage of ejaculate to another person.
  - The participant should be advised of the benefit for a IOCBP partner using a highly effective method of contraception with a failure rate of <1% per year, as described in Section 10.4.2.

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10.4.2. Contraception Methods

Contraceptive use by participants should be consistent with local availability/regulations regarding the use of contraceptive methods for those participating in clinical trials.

The following contraceptive methods are appropriate for this study:

Highly Effective Methods That Have Low User Dependency

1. Implantable progestogen-only hormone contraception associated with inhibition of ovulation.

2. Intrauterine device (copper IUD).

3. Intrauterine hormone-releasing system (hormonal IUS/IUD).

4. Bilateral tubal occlusion (eg, bilateral tubal ligation).

5. Vasectomized partner.
   - Vasectomized partner is a highly effective contraceptive method provided that the partner is the sole sexual partner of the IOCBP and the absence of sperm has been confirmed. If not, an additional highly effective method of contraception should be used. The spermatogenesis cycle is approximately 90 days.
6. Sexual abstinence.
   - Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study intervention. Compliance with sexual abstinence needs to be evaluated and confirmed in relation to the duration of the study.

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10.5. Appendix 5: Genetics

Use/Analysis of DNA

- Genetic variation may impact a participant's response to study intervention, susceptibility to, and severity and progression of disease. Further details of sample purposes are given in Section 8.7. Therefore, where local regulations and IRBs/ECs allow, a blood sample will be collected for DNA analysis.

- The scope of the genetic research may be narrow (eg, 1 or more candidate genes) or broad (eg, the entire genome), as appropriate to the scientific question under investigation.

- The samples may be analyzed as part of a multistudy assessment of genetic factors involved in the response to this study intervention or study interventions of this class to understand treatments for the disease(s) under study or the disease(s) themselves.

- The results of genetic analyses may be reported in the CSR or in a separate study summary or may be used for internal decision making without being included in a study report.

- The sponsor will store the DNA samples in a secure storage space with adequate measures to protect confidentiality.

- The samples will be retained as indicated:

  - Retained samples will be stored indefinitely or for another period as per local requirements.

- Participants may withdraw their consent for the storage and/or use of their Retained Research Samples at any time by making a request to the investigator; in this case, any remaining material will be destroyed. Data already generated from the samples will be retained to protect the integrity of existing analyses.

- Samples for genetic research will be labeled with a code. The key between the code and the participant's personally identifying information (eg, name, address) will be held securely at the study site.

Further details of sample purpose are given in Section 8.7.

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10.6. Appendix 6: Liver Safety: Suggested Actions and Follow-Up Assessments and Study Intervention Rechallenge Guidelines

Potential Cases of Drug-Induced Liver Injury

Humans exposed to a drug who show no sign of liver injury (as determined by elevations in transaminases) are termed “tolerators,” while those who show transient liver injury but adapt are termed “adaptors.” In some participants, transaminase elevations are a harbinger of a more serious potential outcome. These participants fail to adapt and therefore are "susceptible" to progressive and serious liver injury, commonly referred to as DILI after excluding other causes of liver injury. Participants who experience a transaminase elevation above 3 × ULN should be monitored more frequently to determine if they are “adaptors” or are “susceptible.”

In the majority of DILI cases, elevations in AST and/or ALT precede T bili elevations (>2 × ULN) by several days or weeks. The increase in T Bili typically occurs while AST/ALT is/are still elevated above 3 × ULN (ie, AST/ALT and T bili values will be elevated within the same laboratory sample). In rare instances, by the time T bili elevations are detected, AST/ALT values might have decreased. This occurrence is still regarded as a potential DILI. Therefore, abnormal elevations in either AST OR ALT in addition to T bili that meet the criteria outlined below are considered potential DILI (assessed per Hy’s law criteria) cases and should always be considered important medical events, even before all other possible causes of liver injury have been excluded.

The threshold of laboratory abnormalities for a potential DILI case depends on the participant’s individual baseline values and underlying conditions. Participants who present with the following laboratory abnormalities should be evaluated further as potential DILI (Hy’s law) cases to definitively determine the etiology of the abnormal laboratory values:

- Participants with AST/ALT and T bili baseline values within (or below) the normal range who subsequently present with AST OR ALT values ≥3 × ULN AND a T bili value ≥2 × ULN with no evidence of hemolysis and an alkaline phosphatase value <2 × ULN or not available.

- For participants with baseline AST **OR** ALT **OR** T bili values above the ULN, the following threshold values are used in the definition mentioned above, as needed, depending on which values are above the ULN at baseline:

  - Preexisting AST or ALT baseline values above the normal range: AST or ALT values ≥2 times the baseline values AND ≥3 × ULN; or ≥8 × ULN (whichever is smaller).

  - Preexisting values of T bili above the normal range: T bili level increased from baseline value by an amount of ≥1 × ULN **or** if the value reaches ≥3 × ULN (whichever is smaller).

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When study intervention is temporarily withheld or permanently discontinued due to a potential drug induced liver injury, a period of close observation is to commence until the liver test abnormalities return to baseline or normal values. The evaluations listed in the table below should be performed.

Table 11. Monitoring of Liver Tests for Potential Drug Induced Liver Injury

Results	Frequency for Repeating Liver (AST, ALT, Bilirubin [Total and Direct]) and INR Tests
After the initial liver test abnormality	Within 48 hours (See Appendix 6)
If AST or ALT ≥3 × ULN, and total bilirubin ≥2 × ULN or INR >1.5	Every 24 hours until laboratory abnormalities improve
If ALT or AST ≥3 × ULN and total bilirubin or INR are normal	Every 48 to 72 hours until laboratory abnormalities improve
If the liver test abnormalities improve AND the participant is asymptomatic	Frequency may decrease

As drug induced liver injury is a diagnosis of exclusion, it is important to initiate investigation of alternative causes for abnormal liver tests, which may include consultation with a hepatologist. Contact the sponsor with questions regarding sufficient follow-up tests.

Rises in AST/ALT and T bili separated by more than a few weeks should be assessed individually based on clinical judgment; any case where uncertainty remains as to whether it represents a potential Hy's law case should be reviewed with the sponsor.

The participant should return to the investigator site and be evaluated as soon as possible, preferably within 48 hours from awareness of the abnormal results. This evaluation should include laboratory tests, detailed history, and physical assessment. The possibility of hepatic neoplasia (primary or secondary) should be considered.

In addition to repeating measurements of AST and ALT and T bili for suspected Hy's law cases, additional laboratory tests should include albumin, CK, direct and indirect bilirubin, GGT, PT/INR, eosinophils (%), and alkaline phosphatase. Consideration should also be given to drawing a separate tube of clotted blood and an anticoagulated tube of blood for further testing, as needed, for further contemporaneous analyses at the time of the recognized initial abnormalities to determine etiology. A detailed history, including relevant information, such as review of ethanol, acetaminophen/paracetamol (either by itself or as a coformulated product in prescription or over-the-counter medications), recreational drug, or supplement (herbal) use and consumption, family history, sexual history, travel history, history of contact with a jaundiced person, surgery, blood transfusion, history of liver or allergic disease, and potential occupational exposure to chemicals, should be collected. Further testing for acute hepatitis A, B, C, D, and E infection, total bile acids, liver imaging (eg, biliary tract), and

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collection of serum samples for acetaminophen/paracetamol drug and/or protein adduct levels may be warranted.

All cases demonstrated on repeat testing as meeting the laboratory criteria of AST/ALT and T bili elevation defined above should be considered potential DILI (Hy's law) cases if no other reason for the LFT abnormalities has yet been found. **Such potential DILI (Hy's law) cases are to be reported as SAEs, irrespective of availability of all the results of the investigations performed to determine etiology of the LFT abnormalities.**

A potential DILI (Hy's law) case becomes a confirmed case only after all results of reasonable investigations have been received and have excluded an alternative etiology.

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10.7. Appendix 7: Kidney Safety: Monitoring Guidelines

10.7.1. Laboratory Assessment of Change in Kidney Function and Detection of Kidney Injury

Standard kidney safety monitoring requires assessment of baseline and postbaseline Screat measurement to estimate kidney function [Screat-based eGFR] or creatinine clearance [eCrCl]).

Regardless of whether kidney function monitoring tests are required as a routine safety monitoring procedure in the study, if the investigator or sponsor deems it necessary to further assess kidney safety and quantify kidney function, then these test results should be managed and followed per standard of care.

10.7.2. Age-Specific Kidney Function Calculation Recommendations

10.7.2.1. Adults (18 Years and Above)—2021 CKD-EPI Equations

eGFR (mL/min/1.73 m^2)

2021 CKD-EPI Screat only	Screat *(mg/dL)*	Scys *(mg/L)*	Recommended eGFR Equation
Female	if ≤ 0.7	NA	$eGFR = 143 \times (Screat/0.7)^{-0.241} \times (0.9938)^{Age}$
Female	if > 0.7	NA	$eGFR = 143 \times (Screat/0.7)^{-1.200} \times (0.9938)^{Age}$
Male	if ≤ 0.9	NA	$eGFR = 142 \times (Screat/0.9)^{-0.302} \times (0.9938)^{Age}$
Male	if > 0.9	NA	$eGFR = 142 \times (Screat/0.9)^{-1.200} \times (0.9938)^{Age}$

(Inker et al, 2021)

10.7.3. Kidney Function Calculation Tools

The sponsor recommends the following manual (Section 10.7.2) and online calculators to calculate age-specific kidney function.

The United States National Kidney Foundation Online Calculators

- Adults (18 years and above) - 2021 CKD-EPI Creatinine Online Calculator (eGFR): https://www.kidney.org/professionals/KDOQI/gfr_calculator
- Adolescents (12 years to <18 years) - Cockcroft-Gault Formula (eCrCl): https://www.kidney.org/professionals/kdoqi/gfr_calculatorCoc

Please note that investigational sites are responsible to ensure that the accurate age-specific equation is selected and that the correct units are used for serum creatinine (mg/dL only), serum cystatin C (mg/L only), total body weight (kg only), and age (years). Investigators are responsible for the clinical oversight of the participant eligibility process, kidney function calculation, and dose selection and adjustments per study protocol. Investigators are encouraged to direct questions or uncertainties regarding kidney function and dosing to the Pfizer Clinical Team and Medical Monitor, if needed.

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10.7.4. Adverse Event Grading for Kidney Safety Laboratory Abnormalities

AE grading for decline in kidney function (ie, eGFR or eCrCl) will be according to CTCAE criteria.

NCI CTCAE (2017 v5.0) Criteria	**Grade 1**	**Grade 2**	**Grade 3**	**Grade 4**	**Grade 5**
Acute Kidney Injury (AKI)	N/A	N/A	Hospitalization indicated	Life-threatening consequences; dialysis indicated	Death
AKI: A disorder characterized by acute loss of kidney function (within 2 weeks) and is traditionally classified as pre-renal (low blood flow into kidneys), renal (kidney damage), or post-renal causes (ureteral or bladder outflow obstruction.					
Creatinine increased	>ULN to 1.5 x ULN	>1.5 to 3.0 x baseline OR >1.5 to 3.0 x ULN	>3.0 x baseline OR >3.0 to 6.0 x ULN	>6.0 x ULN	N/A
Chronic Kidney Disease (CKD)	eGFR or CrCl (creatinine clearance) <LLN - 60 ml/min/1.73 m^2 or proteinuria 2+ present; urine protein/creatinine >0.5	eGFR or CrCL 30 to 59 mL/min/1.73 m^2	eGFR or CrCL 15 to 29 mL/min/1.73 m^2	eGFR <15 or CrCL mL/min/1.73 m^2 OR dialysis or renal transplant indicated	Death
Proteinuria	ADULTS: Proteinuria 1+; urinary protein ≥ULN - <1.0 g/24 hrs	ADULTS: Proteinuria 2+ or 3+ urinary protein 1.0 to <3.5 g/24 hr	ADULTS: Proteinuria 4+ OR urinary protein ≥3.5 g/24 hr	N/A	N/A

CKD: A disorder characterized by gradual and usually permanent loss of kidney function resulting in kidney failure.

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10.8. Appendix 8: ECG Findings of Potential Clinical Concern

ECG Findings That May Qualify as AEs
• Marked sinus bradycardia (rate <40 bpm) lasting minutes. • New PR interval prolongation >280 ms. • New prolongation of QTcF to >480 ms (absolute). • New prolongation of QTcF by ≥60 ms from baseline. • New-onset atrial flutter or fibrillation, with controlled ventricular response rate: ie, rate <120 bpm. • New-onset type I second-degree (Wenckebach) AV block of >30 second duration. • Frequent PVCs, triplets, or short intervals (<30 seconds) of consecutive ventricular complexes.
ECG Findings That May Qualify as SAEs

• QTcF prolongation >500 ms.
• New ST-T changes suggestive of myocardial ischemia.
• New-onset LBBB (QRS complex >120 ms).
• New-onset right bundle branch block (QRS complex >120 ms).
• Symptomatic bradycardia.
• Asystole:
  • In awake, symptom-free participants in sinus rhythm, with documented asystolic pauses ≥3 seconds or any escape rate <40 bpm, or with an escape rhythm that is below the AV node;
  • In awake, symptom-free participants with atrial fibrillation and bradycardia with 1 or more asystolic pauses of at least 5 seconds or longer.
• Atrial flutter or fibrillation, with rapid ventricular response rate: rapid = rate >120 bpm.
• Sustained supraventricular tachycardia (rate >120 bpm) (“sustained” = short duration with relevant symptoms or lasting >1 minute).
• Ventricular rhythms >30 second duration, including idioventricular rhythm (heart rate <40 bpm), accelerated idioventricular rhythm (HR >40 bpm to <100 bpm), and

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monomorphic/polymorphic ventricular tachycardia (HR >100 bpm [such as torsades de pointes]). • Type II second-degree (Mobitz II) AV block. • Complete (third-degree) heart block.
ECG Findings That Qualify as SAEs
• Change in pattern suggestive of new myocardial infarction. • Sustained ventricular tachyarrhythmias (>30 second duration). • Second- or third-degree AV block requiring pacemaker placement. • Asystolic pauses requiring pacemaker placement. • Atrial flutter or fibrillation with rapid ventricular response requiring cardioversion. • Ventricular fibrillation/flutter. • At the discretion of the investigator, any arrhythmia classified as an adverse experience.

The major events of potential clinical concern listed above are recommended as “alerts” or notifications from the core ECG laboratory to the investigator and Pfizer study team, and not to be considered as all-inclusive of what is to be reported as AEs/SAEs.

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10.9. Appendix 9: Country-Specific Requirements

10.9.1. European Union

This study will be conducted in compliance with Regulation (EU) CTR No 536/2014. The recruitment plans for each EU Member State concerned are included in the respective Recruitment and Informed Consent Procedure documents.

The sponsor will notify EU Member States concerned of the following:

- Annual safety reports via submission to CTIS
- Any SUSAR via reporting to the EudraVigilance database no later than 7 days or 15 days for a fatal or life-threatening SUSAR or non-fatal or non-life-threatening SUSAR, respectively, after becoming aware of that event. A SUSAR is defined as a serious adverse reaction, the nature, severity, or outcome of which is not consistent with the reference safety information.
- Any unexpected event that affects the benefit risk-profile of the study, but are not SUSARs, no later than 15 days of becoming aware of that event
- Any serious breach, as described in Section 10.1.1.1, no later than 7 days of becoming aware of that breach
- Any urgent safety measure, as described in Section 10.1.1.1, no later than 7 days of the measure being taken
- Any inspection report of a third-country authority concerning the study

Records and documents, including signed ICDs, pertaining to the conduct of this study must be retained by the investigator for 25 years or longer if required by other European Union law.

10.9.2. France

Contrat Unique

1. GCP Training

   Before enrolling any participants, the investigator and any subinvestigators will complete the Pfizer-provided Good Clinical Practice training course ("Pfizer GCP Training") or training deemed equivalent by Pfizer. Any investigators who later join the study will do the same before performing study-related duties. For studies of applicable duration, the investigator and subinvestigators will complete Pfizer GCP Training or equivalent every 3 years during the term of the study, or more often if there are significant changes to the ICH GCP guidelines or course materials.

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2. Study Intervention

   No participants or third-party payers will be charged for study intervention.

3. Urgent Safety Measures

   In addition, the investigator will inform Pfizer immediately of any urgent safety measures taken by the investigator to protect the study participants against any immediate hazard, and of any serious breaches of this protocol or of ICH GCP that the investigator becomes aware of.

4. Termination Rights

   Pfizer retains the right to discontinue development of PF-06821497 at any time.

10.9.3. China

In compliance with HGRAC regulations, below is the list of vendors in China that will conduct testing and analysis of human genetic resource samples or disposal of remaining samples.

China will use local laboratories for hematology and chemistry testing (except PSA, testosterone and CD4 count).

Testing Name	Testing and Analysis Unit	Destruction Unit
CD4 count (unscheduled- if the lymphocyte count ≤500/µL)	PPD Laboratories (Suzhou) Co., Ltd.	Zhangjiagang Huarui Hazardous Waste Disposal Center Co., Ltd.
ctDNA analyses	Shanghai Jince Clinical Laboratories Co., Ltd	Shanghai Solid Waste Disposal Co., Ltd.

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10.10. Appendix 10: Response Evaluation Criteria In Solid Tumors (RECIST) v 1.1

10.10.1. Assessment of Soft Tissue Disease

Modification to RECIST 1.1 is a requirement for progression by imaging (omitting progression by digital photography or physical examination for superficial lesions), as described in Section 8.2.2.1. CT, MRI, X-ray and nuclear medicine whole body bone scans (ie radionuclide (Tc-99m) bone scintigraphy) will be reviewed at the investigational site by site staff trained for this trial and may be requested by the sponsor for peer review.

Guidelines Adapted from E.A. Eisenhauer, et al. New response evaluation criteria in solid tumours: Revised RECIST guideline (version 1.1). European Journal of Cancer 45 (2009) 228–247 (Eisenhauer et al, 2009).

10.10.1.1. Image Included in the Assessment

- CT is the preferred method of assessment. IV contrast is required, unless the subject has an allergy to contrast that cannot be controlled with premedications. Anatomy scanned should include the chest (starting above the lung apices), abdomen, and pelvis (coverage to include the symphysis pubis). Slice thickness should be no greater than 5 mm, with no gaps. Details of the imaging protocols will be provided to sites in an imaging manual.

- If participants are unable to receive iodinated CT contrast, the preferred imaging is non-contrast CT of the chest, and MRI of the abdomen and pelvis with contrast.

- For non-infiltrative and distinct skin nodules, CT or MRI remains the preferred method of assessment. Digital photography will not be acceptable as a method of radiographic assessment of skin lesions. Discrete skin nodules that are not visible using CT or MRI should be considered non-measurable, non-target lesions.

- The CT portion of a PET/CT can be considered acceptable for lesion measurement only in case it is documented that the CT performed as part of the PET/CT is of identical diagnostic quality to a diagnostic CT (with IV and oral contrast).

- Chest X-ray is permitted by the RECIST v.1.1 guidelines to assess lung lesions, but CT of the chest is strongly preferred.

- The same method of assessment, the same technique and, whenever possible, the same reviewer should be used to characterize each identified and reported lesion at baseline and during the study.

- For the purpose of this study, bone metastatic lesions, with the exception of lytic or mixed lytic-blastic bone lesions with soft tissue components seen on CT or MRI (See Section 10.10.1.3), should not be recorded as target or non-target lesions according to RECIST 1.1 criteria, but they will be recorded at baseline and followed during treatment using PCWG3 criteria (See Section 10.10.2).

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10.10.1.2. Overview of Assessment Process

The tumor burden will be documented at baseline. First, a reviewer will determine which lesions are appropriate for repeated quantitative assessment (measurable lesions). From the measurable lesions, the reviewer will choose a set of lesions to be followed quantitatively throughout the trial (target lesions). Target lesions will be selected based on size, reproducibility of measurement, and whether the group of lesions represents the disease distribution. A value for the target tumor burden (sum of diameters) will be calculated. All tumor lesions that are not chosen for quantitative assessment will be documented and followed qualitatively as non-target lesions.

At each follow-up visit, the reviewer will assess the target lesions by making measurements that correspond to measurements made at baseline. The Reviewer will verify the calculated sum of diameters, and the comparison of that sum to the baseline value (for determining partial response) and to the nadir, the smallest value seen until that point (to look for progression). The Reviewer will assess the non-target lesions qualitatively and will search for new lesions. Information about target, non-target, and new lesions will be combined to produce an overall visit response for the subject. After all visit assessments are completed, the reviewer will verify each visit response, and the visit responses will be used to derive information relevant to the endpoints (such as the date of progression and the best overall response).

10.10.1.3. Measurable and Non-Measurable Disease

Eligibility of lesions for quantitative assessment will be determined at baseline, with target lesions selected from the measurable lesions identified. After baseline, target lesions will always be assessed quantitatively, and non-target lesions will be assessed qualitatively, with no further evaluation of measurability according to these definitions.

10.10.1.3.1. Measurable Disease

- A lesion is considered measurable if it has a diameter of ≥10 mm in the axial plane on CT or MRI, assuming that the slice thickness is 5 mm or less. If the slice thickness is greater than 5 mm, the minimum size increases to two times the slice thickness (slice thickness is the total distance between the tops of two adjacent slices, including any gap).

- Record changes in liver, lung, adrenal, and CNS separately

- Malignant lymph nodes are considered measurable if the short axis (greatest dimension perpendicular to the longest diameter) is ≥15 mm at baseline.

- Record changes in pelvic (regional) nodes v extrapelvic (distant/metastatic) nodes separately

If chest X-ray is used, a lesion is considered measurable if it is completely surrounded by aerated lung with clear boundaries, and has a diameter of 20 mm. In the unlikely event that a non-spiral CT scanner is used, the minimum size for measurability is 20 mm.

10.10.1.3.2. Non-Measurable Disease

- A lesion is considered evaluable, but not measurable, if it does not meet the measurability criteria described above, but which is a manifestation of malignancy that can be followed qualitatively as an indicator of disease progression or treatment response.
    - By RECIST v.1.1 definitions, a lymph node that has a short axis diameter of 10-14.9 mm is considered evaluable, but not measurable. Nodes with a short axis diameter of <10 mm are considered normal.
    - Palpable skin nodules that are not discernible using CT or MRI are considered evaluable but not measurable for this study.
- Non-measurable lesions are those the reviewer strongly believes to be malignant, but which do not meet the definitions above.
    - Bone lesions that are lytic or mixed lytic-blastic with soft tissue components seen on CT or MRI, and which meet size criteria above, may be measurable (**Note**: for the purpose of this study, bone lesions with no soft tissue components are excluded from the assessment by RECIST 1.1 and must be assessed with PCWG3 criteria, See Section 10.10.2).
    - Simple cysts are considered benign by default. Cystic metastases may be considered measurable, but solid lesions are preferred for selection as target.
    - Malignant ascites or pleural/pericardial effusions, lymphangitic infiltration of skin or lung, leptomeningeal disease, inflammatory breast disease, and infiltrative disease without clear borders are all examples of disease considered intrinsically non-measurable.
- Previous local treatment: A previously irradiated lesion (or lesion subjected to other local treatment) is nonmeasurable unless it has progressed since completion of previous treatment.

10.10.1.4. Baseline Lesion Selection and Documentation

- Target lesions will be chosen from among the measurable lesions. The lesions chosen should be the largest, most reproducible, and most representative of the overall disease distribution.
- All measurable lesions up to a maximum of 5 lesions total, and 2 lesions per organ, should be selected as target lesions (Note: any remaining measurable lesions will be evaluated as non-target lesions, See below). Paired organs (lungs, kidneys, adrenals, ovaries) are considered one organ. The lymph nodes collectively are considered one organ.

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- Non-nodal target lesions are measured in the longest diameter in the axial plane (on CT or MRI). Malignant lymph nodes are always measured in the short axis diameter (greatest dimension perpendicular to the longest diameter). The appropriate measurements for all target lesions chosen at baseline will be added to produce the sum of diameters.

  The sum of the diameters (long axes of all non-nodal target lesions with the short axes of all target lymph nodes lesions) for all target lesions will be reported as the baseline sum of the diameters. The baseline sum of the diameters will be used as reference by which to characterize the objective tumor response.

- Other disease present at baseline (including remaining measurable lesions not selected as target lesions) should be recorded as 'Non-target lesions.' Multiple nontarget lesions in a single organ can be recorded as one entry during assessment, as non-target lesions are assessed as a whole during follow-up.

10.10.1.5. Assessment of Response for Soft Tissue Disease

The overall response for each visit is a combination of target lesion response, non-target lesion response, and the presence or absence of new lesions.

- **Target lesions measurements** should be performed in the same manner as at baseline. The sum of diameters of all target lesions must be calculated and compared to the baseline value and to the nadir value (the smallest value previously seen during the trial, which may be the same as baseline). The following special rules apply to lesion measurements:

  - If two target lesions coalesce the longest diameter measurement of the coalesced mass is used. If a large target lesion splits, the longest diameter of each fragment is measured, and the sum is used.

  - Measurements for target lesions that become small should continue to be recorded, as long as the Reviewer is confident in the accuracy. If a target lesion is considered to have completely disappeared, 0 mm should be recorded. If it is too small to measure accurately, but is still present, a default value of 5 mm should be recorded.

  - When nodal lesions decrease to <10 mm (normal), the actual measurement should still be recorded.

- **Target lesion response** categories are defined as follows:

  - **CR:** Disappearance of all non-nodal target lesions. Target lymph nodes must reduce to <10 mm in short axis.

  - **PR:** At least a 30% decrease in the sum of diameters of target lesions, compared to the sum at baseline.

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- **PD:** At least a 20% increase in the sum of target lesion measurements, compared to the smallest sum on study (including baseline). Also, the absolute increase in the sum has to be at least 5 mm.

- **SD:** Neither PR nor PD criteria are met.

  - SD can follow PR only in rare cases, when the sum increases by less than 20% from the nadir, but enough that a previously seen 30% decrease from baseline no longer holds.

- **NE:** One or more target lesions cannot be assessed because assessment methods are not comparable to baseline (eg, change of modality), lesion visibility is poor due to image quality or lesion features (except where the lesion is too small to measure), or one or more lesions that were excised or locally irradiated have not reappeared or increased; and the other lesions do not show sufficient growth to qualify for PD on their own.

- **Non-target lesion response** assessment is qualitative. Categories are defined as follows:

  - **CR:** Disappearance of all non-nodal non-target lesions. Non-target lymph nodes must reduce to <10 mm in short axis.

  - **PD:** Unequivocal progression of non-target lesions, evaluated as a whole, such that it is clear that treatment has failed, and disease is progressing, regardless of the status of the target lesions.

  - **Non-CR/Non-PD:** Neither CR nor PD criteria are met.

  - **NE:** One or more non-target lesions cannot be assessed because assessment methods are not comparable to baseline (eg, change of modality), lesion visibility is poor due to image quality or lesion features (except where the lesion is too small to measure), or one or more lesions that were excised or locally irradiated have not reappeared or increased; and the other lesions do not show sufficient growth to qualify for PD on their own.

- **New lesion response** is classified as present or not ("Yes" or "No"). The response is "Yes" if there is at least one lesion that the reviewer considers unequivocal new tumor.

  - Lesions that are equivocal (for example, due to small size) should not be considered new until they are confirmed by later scanning. Once confirmed, they can be retrospectively called new lesions at the time they were first seen.

  - A lesion identified in an area not previously scanned is considered a new lesion.

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- **Overall visit response** is determined by combining the target, non-target, and new lesion responses using a table (Section 10.10.3.2).

If an initial CR or PR is noted, confirmatory scans must be performed at least 4 weeks later.

10.10.1.6. Supplemental Investigations

If CR determination depends solely on a residual lesion that decreased in size but did not disappear completely, it is recommended the residual lesion be investigated with biopsy or fine needle aspirate. If no disease is identified, objective status is CR.

If progression determination depends on a lesion with an increase possibly due to necrosis, the lesion may be investigated with biopsy or fine needle aspirate to clarify status.

10.10.1.7. Determination of Best Overall Response according to RECIST 1.1

The best overall response is the best response recorded from randomization until disease progression (taking as reference for progressive disease the smallest sum on study). For CR and PR, the participant's best response assignment will depend on the achievement of both measurement and confirmation criteria. CR and PR must be confirmed by 2 measurements at least 4 weeks apart. In the case of SD, follow-up measurements must have met the SD criteria at least once after randomization at a minimum interval of 6 weeks.

10.10.2. Assessment of Bone Disease

10.10.2.1. Response Description of the Prostate Cancer Working Group (PCWG) Process for Assessment of Bone Lesions

The rules for evaluation of response and progression based on bone lesions were created by the PCWG and published as part of both PCWG2 and PCWG3 (Scher et al, 2016b). However, for the purpose of the study, all bone lesions (with the exceptions of bone lesions with soft tissue components evaluated according to RECIST 1.1, See Section 10.10.1.5 above) are evaluated according to only the PCWG3 rules, including assessment at screening/baseline and evaluation of response.

10.10.2.2. Imaging Methods

The PCWG rules were designed based on the radionuclide (Tc-99m) bone scintigraphy. Other modalities, including FDG-PET, sodium fluoride PET, bone MRI etc. may have individual advantages, but the PCWG rules were not created with the performance characteristics of these methods in mind and should not be used instead of radionuclide bone scan (ie radionuclide (Tc-99m) bone scintigraphy).

Only bone lesions seen by bone scan may be followed for assessment of bone tumor treatment response. Bone disease seen by CT only (not visible on bone scan) is presumed not to represent active disease and should not be documented as a bone lesion (sclerotic lesions seen on CT may represent healed disease or non- malignant confounders such as bone infarcts or other benign findings).

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10.10.2.3. Documentation of Bone Lesions on Baseline

For this study, bone metastatic lesions – with the exception of lytic bone lesions or mixed lytic-blastic bone lesions with identifiable soft tissue components seen on CT or MRI – will be assessed by bone scan (ie, bone scintigraphy), and followed using the PCWG3 criteria, and not RECIST 1.1.

At baseline, bone lesions seen on bone scan with no soft tissue involvement should not be documented as non-target lesions but reported via the PCWG3 CRF.

10.10.2.4. Assessment of Bone Response at Subsequent Imaging Time Points

At all follow-up timepoints, bone disease will be classified as PD, PDu, Non-PD, NED, or NE. The definitions are summarized in the following table and described in more detail below.

Bone Response	Definition
PD	**Progressive Disease** Within the Flare window (< 12 weeks): 2+2 Rule: at least 2 new lesions compared to baseline attributed to metastatic prostate cancer present at Week 9, followed by an additional 2 new lesions at Week 17 scan. OR Outside the Flare Window (> 12 weeks): Two new lesions attributed to metastatic prostate cancer relative to the week 9 scan confirmed on a subsequent scan at least 6 weeks later.
PDu	**Progressive disease unconfirmed** Temporary marker of possible PD, to be updated to PD or non-PD once a subsequent scan is available. If this is the final visit, the visit response will remain PDu.
Non-PD	**Non- progressive disease** There are <2 new bone lesions since baseline.
NE	**Non- evaluable** The response cannot be evaluated (mixed response eg, some existing lesions disappear while new lesions appear, or a change in scan method since baseline etc).

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Bone Response	Definition
NED	**No evidence of disease** No lesion attributed to metastatic prostate cancer seen on bone scan.

10.10.2.5. Descriptions of Bone Response Categories

No evidence of Disease

No lesions attributed to metastatic prostate cancer seen on bone scan at this visit. Either none were seen at baseline or all completely resolved on subsequent imaging.

Non-progression (Non-PD)

At least one bone lesion attributed to metastatic prostate cancer is present on the scan at this visit but the conditions for progression have not been met because there are not at least two new lesions present since baseline.

Unconfirmed Progressive Disease (PDu)

At least two new lesions attributed to metastatic prostate cancer are present, but an additional scan is required for confirmation. This response category is meant as a placeholder that reflects temporary uncertainty and is updated to PD or non-PD once a subsequent bone scan is available.

Progressive Disease (PD)

Within the Flare window (< 12 weeks): 2+2 Rule: at least 2 new lesions attributed to metastatic prostate cancer present at Week 9, followed by an additional 2 new lesions at Week 17 scan.

OR

Outside the Flare Window (> 12 weeks): Two new lesions attributed to metastatic prostate cancer relative to the week 9 scan confirmed on a subsequent scan at least 6 weeks later.

Confirmation of Progression

Radiographic progression of bone lesions attributed to metastatic prostate cancer is defined as the appearance of ≥2 new bone lesions on radionuclide bone scan. When ≥ 2 new bone lesions are first observed, this is classified as PDu, which marks the possibility of progression that will be resolved by the next scan.

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For new lesions <u>within the flare Window</u> (< 12 weeks)

After a scan classified as PDu within the first 12 weeks of treatment, if the next bone scan outside the flare window shows at least 2 additional new bone lesions attributed to metastatic prostate cancer (in addition to the new lesions seen on the prior scan), the initial progression is considered confirmed and the bone scan response updated to PD. Because this required at least 2 new lesions, followed by another 2 new lesions, this is known as the '2+2' rule.

If the next bone scan outside the flare window does not show at least two additional new bone lesions, attributed to metastatic prostate cancer, the lesions seen on the prior scan within the flare window are considered to be pre-existing lesions that became more visible because of the tumor flare phenomenon.

The bone response at the prior visit is updated to non-PD

The bone lesions seen within the flare window are ignored for the purposes of counting new lesions at later timepoints since they were not new. This may be referred to as rebaselining.

For New Lesions <u>Outside the Flare Window</u> (>12 weeks)

After a scan classified as PDu after the first 12 weeks of treatment, if at least 2 of the new lesions attributed to metastatic prostate cancer seen on that scan persist on the next bone scan performed at least 6 weeks later, this confirms the initial progression (Note: the new bone lesions do not all have to appear at the same time). The prior response is then updated to PD. If the new lesions have disappeared on this later scan, the prior response is updated to non-PD because these lesions are presumed to be non-malignant in nature. No re-baselining of lesions will occur in this scenario.

Superscan

A 'superscan' occurs when there is a diffuse skeletal involvement by tumor such that individual bone lesions are not distinguishable. The bone scan may initially appear normal because the increase bone uptake may be uniform but can be distinguished by the faint or absent activity in the kidneys and urinary tract.

If there is a true superscan at screening/baseline, identifying new bone lesions and determining progression based on bone lesions may be impossible: a participant with a finding of superscan at screening/baseline will be considered not eligible for the study.

If a superscan occurs after baseline, the participant's bone response will be recorded as PD. No subsequent imaging will be required for confirmation, because a superscan is extremely unlikely to be caused by benign conditions or tumor flare.

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Management Following Confirmed PD

If repeat imaging does confirm PD, participants will be discontinued from study intervention, unless still deriving clinical benefit from study intervention, provided that the treating physician has determined that the benefit/risk for doing so is favorable. Please refer to Section 7.1 "Discontinuation of Study Intervention" for details.

Of particular note is the use of the 2+2 rule to distinguish flare from true progression.

The table and figure below provide examples for the determination of Bone Progression:

10.10.2.6. Criteria for Evidence of Bone Progression

Date Progression Detected (Visit) [1]	**Criteria for Progression**	**Criteria to Confirm Progression**	**Criteria to Document Disease Progression**
Week 9	2 or more new lesions compared to baseline bone scan	Timing: at least 6 weeks after progression identified or at week 17 visit [2]	Persistence of at least 2 lesions seen at week 9 AND 2 or more new bone lesions on bone scan compared to week 9 scan
Week 17 or later	2 or more new lesions on bone scan compared to week 9 bone scan	Timing: at least 6 weeks after progression identified or at next imaging time point [2]	Persistence of at least 2 of the lesions identified as new compared to week 9

[1] Progression detected by bone scan at an unscheduled visit during the flare window should follow the confirmation criteria outlined for week 9. Progression detected by bone scan at an unscheduled visit after the flare window should follow the confirmation criteria specified for week 17 or later.

[2] Confirmation must occur at the next available scan. When 3 or more successive unconfirmed PD events exist that are less than 6 weeks apart, but the 1st and 3rd unconfirmed PD events are ≥6 weeks apart, the 1st unconfirmed PD becomes the confirmed date of bone progression. Confirmation of PD must also occur at the next available scan after the flare window in case there is a single new bone lesion appearing at each of two subsequent scans (1+1).

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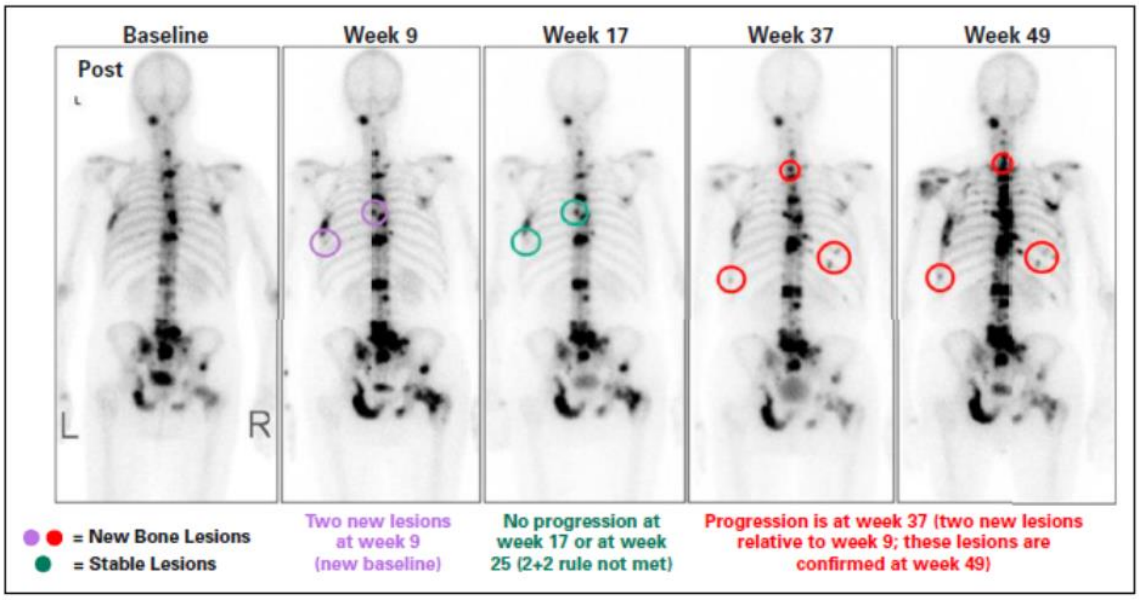
Figure 1. Controlling for Flare by Applying the 2+2 Rule Using the First Post-Treatment Scan as Baseline



In summary, PCWG3 criteria for bone disease consider the following:

- Tumor flare may occur within the first 12 weeks on treatment.
- This implies that the bone scan obtained at week 9 may present a number of bone lesions apparently new that were likely already present at baseline but not detected.
- In this case, the week 9 bone scan will be considered a new baseline for comparison of the subsequent scans.
- Confirmed progression is determined by the presence of at least 2 new bone lesions (which may each occur at different timepoints) in subsequent scans as compared to baseline scan (if there was no flare). During the flare timeframe (< 12 weeks) progression can only be confirmed by presence of 2 additional new lesions on the very next scan (Week 17).

10.10.3. Additional Notes

10.10.3.1. Radiographic Progression and Continued Clinical Benefit from Study Intervention

- If a participant has confirmed radiographic progression according to RECIST 1.1 and/or according to PCWG3 criteria as defined above but is deriving a clinically meaningful benefit from the study intervention as assessed by investigator, the participant will be eligible to continue receiving study intervention, provided that the treating physician has determined that the benefit/risk for doing so is favorable. In this case, if study intervention is continued, tumor imaging should continue as per

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local clinical practice. To ensure that patients are not exposed to unreasonable risks by continued use of the investigational agent despite progression of disease, ensure the following:

- There is an absence of symptoms and signs indicating clinically significant progression of disease,
- No decline in ECOG performance status, and
- Absence of symptomatic rapid disease progression requiring urgent medical intervention (e.g., symptomatic pleural effusion, spinal cord compression).

10.10.3.2. Clinical Progression

Participants requiring discontinuation of treatment due to global deterioration of their clinical status with no objective evidence of disease progression according to RECIST 1.1 nor according to PGWG3 criteria should not be reported as PD on tumor assessment CRFs. This should be indicated on the End of Treatment CRF as the "off treatment" reason due to global deterioration of clinical status. In this case, every effort should be made to document objective progression according to RECIST 1.1 and/or according to PCWG3 criteria even after discontinuation of treatment.

Adapted from Eisenhauer E.A., et al: New response evaluation criteria in solid tumors: Revised RECIST guideline (version 1.1). European Journal of Cancer 45 (2009) 228–247 (Eisenhauer et al, 2009)

Table 12. Objective Response Status at each Evaluation

Target Lesions	Non-target Disease	New Lesions	Objective status
CR	CR	No	CR
CR	Non-CR/Non-PD	No	PR
CR	Indeterminate or Missing	No	PR
PR	Non-CR/Non-PD, Indeterminate, or Missing	No	PR
SD	Non-CR/Non-PD, Indeterminate, or Missing	No	Stable
Indeterminate or Missing	Non-PD	No	Indeterminate
PD	Any	Yes or No	PD
Any	PD	Yes or No	PD
Any	Any	Yes	PD

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The following table will be used for participants with non-target disease when PCWG3 evaluable bone disease is also present:

Table 13. Objective Response Status at each Evaluation for Participants with Non-Target Disease Only

Non-target Disease	New Lesions	Objective status
CR	No	CR
Non-CR/Non-PD	No	Non-CR/Non-PD
Indeterminate	No	Indeterminate
Unequivocal progression	Yes or No	PD
Any	Yes	PD

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10.11. Appendix 11: ECOG Performance Status

Developed by the Eastern Cooperative Oncology Group, Robert L. Comis, MD, Group Chair.

0	Fully active, able to carry on all pre-disease activities without restriction
1	Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, eg, light housework or office work
2	Ambulatory and capable of all selfcare but unable to carry out any work activities. Up and about more than 50% of waking hours
3	Capable of only limited selfcare, confined to bed or chair more than 50% of waking hours
4	Completely disabled. Cannot carry on any selfcare. Totally confined to bed or chair
5	Dead

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10.12. Appendix 12: Protocol Amendment History

The protocol amendment summary of changes table for the current amendment is located directly before the TOC. The protocol amendment summary of changes tables for past amendment(s) can be found below:

Amendment 1 (11 June 2024)

Overall Rationale for the Amendment: The main purpose of Protocol Amendment 1 is to incorporate changes to the protocol based on feedback received from the FDA recommending that the dose of PF-06821497 be adjusted for participants receiving treatment in Arm A (PF-06821497 given in combination with enzalutamide) who permanently discontinue enzalutamide treatment, but continue receiving PF-06821497. Given this feedback the PF-06821497 dose has been adjusted to reflect that the dose will be reduced to 500 mg BID without food instead of 625 mg BID without food.

Description of Change	Brief Rationale	Section # and Name
Substantial Modification(s)		
The PF-06821497 dose was adjusted for participants who receive treatment in Arm A (PF-06821497 given in combination with enzalutamide), and discontinue enzalutamide treatment, but continue PF-06821497 based on FDA feedback.	Given that the PK data for PF-06821497 at 625 mg BID on empty stomach is very limited and its exposure appears to be higher than the target exposure after multiple dosing. Instead, 500 mg BID on empty stomach appears to provide closer PK exposure to target exposure after multiple dosing.	Section 5.3.3 Meals and Dietary Restrictions; and Section 6.6.2 Enzalutamide Dose Modifications
Non-Substantial Modification(s)		
Screening Tumor Biopsy added to SoA if prior histological diagnosis is not available to confirm Inclusion criterion #2.	A line item for Screening Tumor Biopsy was added to ensure clarification of the biopsy collection timing.	Section 1.3 Schedule of Activities, Tables 1 and 2
"HRR mutation status if known" removed from notes section of the medical history on SOA	Text removed from the SOA due to a change in strategy regarding the collection of BRCA/HRR status. Note was added for clarification.	Section 1.3 Schedule of Activities
A note was added to Inclusion Criterion #4 to indicate that participants who are not orchiectomized will also continue to receive a GnRH (gonadotropin releasing	To provide clarity for those patients who are not orchiectomized	Section 5.1 Inclusion Criteria, Criterion #4

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Description of Change	Brief Rationale	Section # and Name
hormone) agonist or antagonist on study.		
Study Objective was clarified to reflect that the PK samples for the pre-dose trough and post-dose plasma concentrations of PF-06821497 are required only at “selected visits”.	Text added to provide clarity.	Section 1.1 Synopsis Section 3.0 OBJECTIVES, ENDPOINTS and ESTIMANDS
Adjusted the text for the 28 day Safety Follow-up period window.	Clarification added to multiple protocol sections that the 28 day Safety Follow-up is to be performed up to Day 35 after the end of treatment.	Section 1.1 Synopsis Section 1.3 Schedule of Activities Section 4.1 Overall Study Design
Text changed to reflect that participants will receive protocol assigned treatment until radiographic progression.	Clarification of text made to avoid confusion.	Section 1.1 Overall Design
Clarified timing of labs in SoA to indicate that the C1 D1 lab samples do not need to be redrawn if collected within 7 days of screening.	Adjustment made to provide clarity.	Section 1.3 Schedule of Activities Tables 1 and 2
Removed duplicated text in section: "An independent E-DMC will monitor the safety of participants on a periodic basis".	Removed to correct unnecessary duplicate text.	Section 4.1 Overall Design
Updated language to clarify the rescreening process for a participant.	For clarification purposes, the text was adjusted and acronym added to Abbreviations listing.	Section 5.4 Screen Failures Section 10.13 Appendix 13 Abbreviations
The term “approximately” vs a “minimum” better describes the intention of how the PK blood samples should be collected.	Language updated for clarification.	Section 8.5.1. Plasma for PK Analysis of PF-06821497
Section corrected to reflect the correct reference to the exclusion criterion #5.	The change was made to correct the typo	Section 6.9.1 Prohibited During the Study
“Clinical assessment of superficial lesions” text removed from this section.	This text is not applicable to this study.	Section 8.2.2.1

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Description of Change	Brief Rationale	Section # and Name
Biomarker collection times for circulating free nucleic acids and Blood samples for serum biomarkers clarified.	The references to the cycles, Days and weeks were unclear and required clarification.	Section 8.8, Table 8

Amendment 2 (13 November 2024)

Overall Rationale for the Amendment:

The main purpose of Protocol Amendment 2 is to incorporate changes to the protocol based on feedback received from the EU health authorities including an adjustment to remove the allowance for participants receiving treatment Arm A (PF-06821497 given in combination with enzalutamide), who discontinue enzalutamide, the option to continue taking mevrometostat monotherapy. This will require participants who discontinue enzalutamide to also discontinue treatment with PF-06821497. Additional language was added to Exclusion Criterion 1 to exclude participants who have active inflammatory gastrointestinal disease, chronic diarrhea, previous gastric resection, or lap-band surgery. Additional revisions to the amendment include clarification to eligibility criteria (to clarify the definition of PSA progression); enhanced text and clarified study intervention text, language regarding liver tests, dose reduction language, and treatment of overdose; simplified language regarding vital signs, ECGs, and PK sampling; clarified language in Appendix 10, Response Evaluation Criteria in Solid Tumors; and made general editorial adjustments.

Substantial Modification(s)		
Description of Change	**Brief Rationale**	**Section # and Name**
Revised protocol language to remove that when patients discontinue enzalutamide, treatment with PF-06821497 will be given alone and their dose will be reduced to 500 mg dose without food. Patients who discontinue enzalutamide will also discontinue PF-06821497.	Decision to remove as there are plans to evaluate monotherapy in other studies.	Sections 3. Meals and Dietary Restrictions; Enzalutamide Dose Modifications

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Added text about active inflammatory gastrointestinal disease, chronic diarrhea, previous gastric resection, lap-band surgery	This text was added to specifically exclude these conditions	Section 5.2 Exclusion Criteria
Non-Substantial Modification(s)		
Added study intervention name and ClinicalTrials.gov ID	Update with current information	Title page, Section 1
Modified text about when to perform laboratory screening tests, PK, PRO-CTCAE, FACT-GP5.	Clarify these points	Section 1.3 Schedule of Activities, Table 1
Modified text describing visit window timeframe; added text about randomization	Make consistent with preceding table	Section 1.3 Schedule of Activities, Table 2
Modified text about hypersensitivity	Emphasize importance of following local label and require discontinuing XTANDI in the event of serious hypersensitivity reactions	Section 2.3.1. Risk Assessment
Added text defining the end of the study	Clarify the definition of the end of the study	Section 4.4 End of Study Definition
Modified text about PSA progression, evidence of disease progression	Clarify this point	Section 5.1 Inclusion Criteria
Added text about neuroendocrine differentiation and other histologic components	Clarify this point	Section 5.1 Inclusion Criteria
Modified text about contraception	Align with EU SMPC for docetaxel	5.3.1. Contraception

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IV administration of dexamethasone added	Add this route of administration as a possibility	Sections 6.1. Study Intervention(s) Administered, Table 3; 6.1.1.3. Docetaxel
Removed citations of SmPCs used as SRSDs	Allow usage of current versions at all times	Section 6.1 Table 3
Modified text about self-administration and clinic administration of oral doses	Clarify this point	Section 6.1.1.1. PF-06821497
Modified text about liver tests	Clarify this topic, remove duplicative material	Section 6.6.4. Modifications Due to Abnormal Liver Tests
Modified text about lymphocyte counts	Add further clinical considerations	Section 6.6.5. Dose Reductions for PF-06821497, Table 6
Modified text about overdose	Specify what is meant by an overdose	Section 6.8. Treatment of Overdose
Removed text requiring physical examination prior to blood draw	Simplify clinical requirement	Section 8.3.1. Vital Signs and Physical Examinations
Removed text requiring ECG prior to blood collection	Simplify clinical requirement	Section 8.3.2. (12-Lead) Electrocardiogram
Modified text describing timing of blood collection for PK	Clarify this point	Section 8.5.1. Plasma for PK analysis of PF-06821497
Added text about sensitivity analyses	Clarify this point	Section 9.3.2 Primary Endpoint(s)/Estimand(s)/ Analysis
Added “following the end of the study globally.”	Clarify this timeframe	Section 10.1.6. Dissemination of Clinical Study Data
Merged Appendices about Country-Specific Requirements, added EU-specific text	Remove duplicative appendix and comply with EU requirements	Section 10.9 Appendix 9: Country-Specific Requirements
Modified text about measurable disease	Clarify these measuring procedures	Section 10.10.1.3.1. Measurable Disease

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Modified text about lesion diameters	Clarify the reporting of these dimensions	Section 10.10.1.4. Baseline Lesion Selection and Documentation
Removed tables	Remove material duplicative of later section	10.10.1.5. Assessment of Response for Soft Tissue Disease
Added statement about PCWG3 for the purpose of this study	Clarify the need to use PCWG3 rules for the purpose of this study	Section 10.10.2.1. Response Description of the Prostate Cancer Working Group (PCWG) Process for Assessment of Bone Lesions
Modified text about documenting bone lesions	Clarify these requirements	Section 10.10.2.3. Documentation of Bone Lesions on Baseline
Added and modified text about definitions of bone response categories	Simplify clinical decisions	Section 10.10.2.4 Assessment of Bone Response at Subsequent Imaging Time Points
Modified text describing bone response categories and the determination of Bone Progression	Clarify these descriptions and requirements	Section 10.10.2.5. Descriptions of Bone Response Categories; Section 10.10.2.6. Criteria for Evidence of Bone Progression
Removed text about categorizing lesions at baseline, recording tumor assessments, objective response status at each evaluation. Modified text describing table “Objective Response Status at each Evaluation for Participants with Non-Target Disease Only”	Remove duplicative text and clarify information about table	Section 10.10.3.2 Clinical Progression
General editorial changes	To correct grammatical errors, to maintain consistency, and/or to increase clarity	Throughout protocol

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Amendment 3 (29 January 2025)

Overall Rationale for the Amendment:

The main purpose of Protocol Amendment 3 is to incorporate changes to the protocol based on feedback received from the EU health authorities primarily related to statistical aspects of the protocol, as well as to add further dose modification instructions for abnormal AST and ALT levels and clarify other details as listed in the table below.

Description of Change	Brief Rationale	Section # and Name
Substantial Modification(s)		
Updated dose modification guidance for Grade 3 serum bilirubin increased and Grade 3 abnormal AST or ALT levels	Revised the dose modification guidelines for Grade 3 serum bilirubin increases to allow for only one dose reduction Revised dose modification guidelines for elevated Grade 3 AST or ALT levels to include the necessary criteria. Added text to refer to guidance in Table 6.	Section 6.6.4 Modifications Due to Abnormal Liver Tests, and Section 6.6.5 Dose Reductions for PF-06821497
Non-substantial Modification(s)		
Clarified differences between participants receiving enzalutamide and participants receiving docetaxel with respect to scheduling of assessments, and added note about taking study drug supplies into account when scheduling assessments	Provides clarity on how to schedule study visits.	Section 1.3 Schedule of Activities, Table 1 and Table 2
Updated the start of LTFU data collection from “last dose of study	Clarified when data collection should begin.	Section 1.1 Synopsis; Section 4.1 Overall Design; Section 7.1.2 Long Term Follow-Up

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Description of Change	Brief Rationale	Section # and Name
intervention" to "Safety Follow-up"		
Added text to specify that imaging assessments should utilize the date of randomization as the reference date for all time points	Ensures visibility of this requirement within the SoA.	Section 1.3 Schedule of Activities, Table 1 and Table 2
Increased visit window for C4 D1 and beyond to +/- 3 days for Arm B participants receiving docetaxel	Provides more flexibility to participants and aligns with the visit window in Cycles 1 to 3.	Section 1.3 Schedule of Activities, Table 2
Added Screening time point for diagnosis of second primary malignancies	Clarified when the activity is to occur.	Section 1.3 Schedule of Activities, Table 1 and Table 2
Modified text about obtaining CD4 counts if lymphocyte count is <500/µL	Clarified this requirement.	Section 1.3 Schedule of Activities, Table 1 and Table 2; Section 10.2 Appendix 2: Clinical Laboratory Tests, Table 10
Adjusted Symptomatic Skeletal Events row to reflect that this assessment will not be performed at Screening	Clarified when the activity is to occur.	Section 1.3 Schedule of Activities, Table 1 and Table 2
Added statement that investigator's reason for treatment choice of docetaxel or enzalutamide be collected prior to	Added to reflect the new variable (reason for treatment choice) in the randomization CRF.	Section 1.3 Schedule of Activities, Table 1 and Table 2; Section 8.1 Administrative and Baseline Procedures

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Description of Change	Brief Rationale	Section # and Name
randomization on C1 D1		
Added statement that approximately 174 sites will participate in this study	Added text per EU Health Authority query response.	Section 4.1 Overall Design
Added text describing the planned subgroup analyses and sensitivity analyses for the primary endpoint	Added text per EU Health Authority query response.	Section 9.3.2 Primary Endpoint(s)/Estimand(s)/Analysis
Added text to specify that subgroup analysis of OS will use the same subgroups specified for the primary endpoint subgroup analyses	Added text per EU Health Authority query response.	Section 9.3.3.1 Key Secondary Endpoint
Added more information about the assessment of other secondary efficacy endpoints	Added text per EU Health Authority query response.	Section 9.3.3.2 Other Secondary Efficacy Endpoints
Added statement about the projected time to reach target number of rPFS events, and removed statement about projected lost to follow-up rate for the OS endpoint	Added text per EU Health Authority query response.	Section 9.5 Sample Size Determination
Adjusted language in exclusion criterion #1 to clarify that participants with a	Added further clarity.	Section 5.2 Exclusion Criteria

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Description of Change	Brief Rationale	Section # and Name
known history of chronic liver diseases or gastrointestinal conditions are excluded		
Removed “atrial fibrillation of any grade” from exclusion criterion #2	Updated to permit enrollment of participants with Grade 1 atrial fibrillation, as participants with Grade 2 and above are already excluded.	Section 5.2 Exclusion Criteria
Removed the statement about discussing cardiac rhythm device/pacemaker use with the medical monitor from exclusion criterion #2	Removed as there are no exceptions that would require discussion with the medical monitor.	Section 5.2 Exclusion Criteria
Provided additional information on the types of prohibited prior treatments in exclusion criterion #5	Added further clarity.	
Adjusted language in exclusion criterion #5a to clarify that first-generation anti-androgen agents must be discontinued prior to first dose of study medication	Clarified these requirements.	Section 5.2 Exclusion Criteria
Modified exclusion criterion #6 to specify that the longer of the two washout periods	Improve efficiency and enable flexibility in enrolling participants and provide clarity when assessing which medications are allowed/disallowed.	Section 5.2 Exclusion Criteria

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Description of Change	Brief Rationale	Section # and Name
(30 days or 5 half-lives) will be used to determine eligibility for participants who have previously received another investigational product, and added text "which does not meet exclusion criterion 5 above)"		
Risk Assessment Section updated to include potential risks for secondary malignancies and alternative facilities	Additional potential risks were identified which include: 1) Secondary Malignancies risk updated to note that this is an important risk and to reference the IB for related data. 2) An Alternative Facilities risk was added to reflect that only routine safety assessments should be performed at an alternative facility.	Section 2.3.1 Risk Assessment
Protocol was updated to clarify when palliative radiotherapy is acceptable	Text updated to note that treatment of bone metastases present at baseline or to control symptoms related to the primary prostate tumor such as lower urinary tract symptoms, hematuria and pain is allowed.	Section 6.9.3 Permitted During the Study
The reference to "radiologists" was removed and replaced with "site staff"	Updated protocol to clarify that the "site staff" responsible for reviewing imaging will be trained on the study.	Section 8.2.2 Assessment of Primary Efficacy Endpoint: rPFS; Section 10.10.1 Assessment of Soft Tissue Disease
Added text to indicate that rescreened participants do not need to repeat screening assessments that	Reduces participant burden.	Section 5.4 Screen Failures

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Description of Change	Brief Rationale	Section # and Name
were completed within 28 days of rescreening		
Removed text about delays to the start of next cycle administration for PF-06821497 and/or enzalutamide when dosing is interrupted due to toxicity	Not applicable for these study interventions as they are continuously dosed.	Section 6.6.1 PF-06821497 Dose Modifications; Section 6.6.5 Dose Reductions for PF-06821497; Section 6.6.6 Dosing Interruptions; Section 6.6.7 Dose Delays
Modified text to increase the permitted time window for making up a missed dose from 4 hours to 6 hours	Adjusted to provide flexibility.	Section 6.1.1.1 PF-06821497
Specified that prednisone, prednisolone, and dexamethasone can be sourced either locally or centrally	Added further clarity.	Section 6.1 Study Intervention(s) Administered, Table 3
Updated instructions for dexamethasone premedication before docetaxel infusion to clarify that this can be done per local practice/standard of care guidelines	Added to provide further clarity.	Section 6.1 Study Intervention(s) Administered, Table 3; Section 6.1.1.3 Docetaxel
Updated list of strong CYP3A4/5 inhibitors	Remove moderate CYP3A4/5 inhibitors from the list of strong inhibitors	Section 6.9.1 Prohibited During the Study
Added China specific list of	Added Section 10.9.3 China to include adjustments to protocol	Section 10.9.3 China

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Description of Change	Brief Rationale	Section # and Name
analysis samples and vendors that will conduct testing and analysis in China	detailed in PACLs dated 18-June-2024 and 31-July-2024.	
Removed administration of prohibited concomitant therapy from list of reasons for discontinuation of study intervention	Updated for consistency with Section 6.9.1 which states that investigator will make judgment on participants using prohibited medications while on study.	Section 1.1 Synopsis, Section 7.1 Discontinuation of Study Intervention
Added text to provide more specific guidance on recording withdrawal of consent in the participant's medical records	Added additional guidance.	Section 7.2.1 Withdrawal of Consent
Added text to specify the approximate total blood volume collected from individual participants	Updated to provide information not previously added.	Section 8.1 Administrative and Baseline Procedures
Added text to state that local laboratory results may be used to satisfy protocol requirements for performing clinical laboratory assessments	Clarifies usage of local laboratories.	Section 8.3.3 Clinical Safety Laboratory Assessments
Added text describing the timing of symptomatic skeletal event	Clarified when the evaluation should occur.	Section 8.3.4 Symptomatic Skeletal Event Evaluation

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Description of Change	Brief Rationale	Section # and Name
evaluation during the study		
Added reference Section 8.7 which contains details about the purpose for genetic testing	Provides link to further information.	Section 10.5 Appendix 5: Genetics
Adjusted units for eGFR to "mL/min/1.73 m^2"	Clarification of eGFR units.	Section 5.2 Exclusion Criteria; Section 10.7.2.1 Adults (18 Years and Above) – 2021 CKD-EPI Equations
General editorial changes	To correct grammatical errors, to maintain consistency, and/or to increase clarity.	Throughout protocol

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10.13. Appendix 13: Abbreviations

The following may be used in the protocol.

Abbreviation	Term
ADT	androgen deprivation therapy
AE	adverse event
AESI	adverse event of special interest
AIDS	acquired immunodeficiency syndrome
AKI	acute kidney injury
ALT	alanine transaminase
AML	acute myelogenous leukemia
ANC	absolute neutrophil count
AR	androgen receptor
AR-V7	androgen receptor splice variant 7
ARSi	androgen receptor signaling inhibition
ASCO	American Society of Clinical Oncology
AST	aspartate aminotransferase
AV	atrioventricular
BICR	blinded independent central review
BID	2 times a day
BP	blood pressure
BPI-SF	brief pain inventory-short form
BRCAm	breast cancer gene mutation
BUN	blood urea nitrogen
ºC	Celsius
C1 D1	cycle 1 day 1
C1 D15	cycle 1 day 15
C2 D1	cycle 2 day 1
C2 D15	cycle 2 day 15
C3 D1	cycle 3 day 1
C4 D1	cycle 4 day 1
C5 D1	cycle 5 day 1
C6 D1	cycle 6 day 1
CBC	complete blood cell count
CD4	T helper cells
cfDNA	circulating free DNA
cfRNA	circulating free RNA
CFR	Code of Federal Regulations
COVID-19	Coronavirus Disease 2019
CHO	Chinese hamster ovary
CHO-K1	Chinese hamster ovary K1
CI	confidence interval
CIOMS	Council of International Organizations of Medical Sciences
CKD	chronic kidney disease

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Abbreviation	Term
CKD-EPI	chronic kidney disease epidemiology collaboration
cm	centimeter
CMH	Cochran-Mantel-Haenszel
CNS	central nervous system
Cox	Cox proportional-Hazards Model
CR	complete response
CRF	case report form
CRO	clinical research organization
CRPC	castration resistant prostate cancer
CSPC	castration sensitive prostate cancer
CSR	clinical study report
CT	computed tomography, clinical trial
CTCAE	common terminology criteria for adverse events
ctDNA	circulating tumor DNA
CTIS	Clinical Trials Information System
CV	coefficient of variation
CVD	cardiovascular disease
CYP	cytochrome P
D1	day 1
D15	day 15
DCT	data collection tool
DDI	drug-drug interaction
DICI	drug induced creatinine increase
DIKI	drug induced kidney injury
DILI	drug induced liver injury
DLBCL	diffuse large B cell lymphoma
DMC	data monitoring committee
DNA	deoxyribonucleic acid
DoR	duration of response
DU	dispensable unit
EC	ethics committee
ECC	emergency contact card
ECG	electrocardiogram
eCrCl	estimated creatinine clearance
ECOG	Eastern Cooperative Oncology Group
eCRF	electronic case report form
E-DMC	external data monitoring committee
EDB	exposure during breastfeeding
EDP	exposure during pregnancy
EDTA	ethylenediaminetetraacetic acid
EFD	embryo fetal development
eGFR	estimated glomerular filtration rate
EMA	European Medicines Agency

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Abbreviation	Term
eSAE	electronic serious adverse event
ePRO	electronic patient reported outcomes
EOT	end of treatment
EQ-5D-5L	European Quality of Life 5-Dimensions 5-Level
ESMO	European Society for Medical Oncology
EU	European Union
EudraCT	European Union Drug Regulating Authorities Clinical Trials European Clinical Trials Database
EZH2	enhancer of zeste homologue-2
EZH2i	enhancer of zeste homologue-2 inhibitor
FACT-GP5	Functional Assessment of Cancer Therapy General Population Item 5
FACT-P	Functional Assessment of Cancer Therapy-Prostate
FAS	full analysis set
FDA	Food and Drug Administration
FDG-PET	fluorodeoxyglucose-positron emission tomography
FFPE	formalin fixed paraffin embedded
FL	follicular lymphoma
FOXA1	Forkhead box A1
GCP	Good Clinical Practice
G-CSF	granulocyte colony-stimulating factor
GGT	gamma-glutamyl transferase
GM-CSF	granulocyte-macrophage colony-stimulating factor
GnRH	gonadotropin releasing hormone
GOF	gain of function
H	hypothesis
HBV	hepatitis b virus
HCV	hepatitis c virus
HGPRT	hypoxanthine-guanine phosphoribosyltransferase
HIV	human immunodeficiency virus
HR	hazard ratio
HCP	health care professional
HGRAC	Human Genetic Resource Administration of China
HRQoL	health related quality of life
HRRm	homologous recombination repair mutation
IB	Investigator’s Brochure
ICD	informed consent document
ICH	International Council of Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
ICU	intensive care unit
ID	identification
IHD	ischemic heart disease
IMP	investigational medicinal product
IND	investigational new drug

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Abbreviation	Term
INR	international normalized ratio
IOCBP	individuals of childbearing potential
IP	internet protocol
IPAL	investigational product accountability log
IPM	investigational product manual
IRB	institutional review board
IRT	interactive response technology
IUD	intrauterine device
IUS	intrauterine system
IV	intravenously
kg	kilogram
LBBB	left bundle branch block
LD	longest diameter
LDH	lactic dehydrogenase
LFT	liver function test
LPLV	last participant last visit
LTFU	long term follow-up
MATE-1	multidrug and toxin extrusion protein 1
mCRPC	metastatic castration-resistant prostate cancer
mCSPC	metastatic castration-sensitive prostate cancer
MDS	myelodysplastic syndrome
MedDRA	medical dictionary for regulatory activities
MQI	medically qualified individual
MRI	magnetic resonance imaging
NA	not applicable
NBF	neutral buffered formalin
NCCN	National Comprehensive Cancer Network
NCI CTCAE	National Cancer Institute Common Terminology Criteria for Adverse Events
NE	non-evaluable
NED	no evidence of disease
NEPC	neuroendocrine prostate cancer
NIMP	non-investigational medicinal product
Non-CR	no complete response
Non-PD	no progressive disease
NRS	numeric rating scale
OR	objective response
ORR	objective response rate
OS	overall survival
PARP	poly adenosine diphosphate-ribose polymerase
PCD	primary completion date
PCP	pneumocystis pneumonia
PCT	physician’s choice of treatment

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Abbreviation	Term
PCWG3	Prostate Cancer Clinical Trials Working Group 3
PD	pharmacodynamics, progressive disease
PDu	progressive disease unconfirmed
PE	physical examination
PET	positron emission tomography
PFS	progression free survival
PFS2	progression free survival on next line of therapy
P-gp	p-glycoprotein
PK	pharmacokinetics
PPI	proton pump inhibitor
PR	pulse rate, partial response
PRC2	polycomb repressor complex 2
PRES	posterior reversible encephalopathy syndrome
PRO	patient reported outcome
PSA	prostate specific antigen
PSSA	Pfizer's Serious Adverse Event Submission Assistant
PT	prothrombin time, preferred term
PVC	premature ventricular contraction
QD	once daily
QTcF	QT corrected for heart rate by Fridericia's cube root formula)
QTL	quality tolerance limit
RB1	retinoblastoma 1
RCT	randomized clinical trial
RDE	Recommended dose for expansion
RECIST	Response Evaluation Criteria in Solid Tumors
RNA	Ribonucleic acid
rPFS	radiographic progression free survival
SAE	serious adverse event
SAP	statistical analysis plan
SARS-CoV2	severe acute respiratory syndrome coronavirus 2
SCLC	small cell lung cancer
Screat	serum creatinine
Scys	serum cystatin C
SD	stable disease
SoA	schedule of activities
SOC	standard of care, system organ class
SOP	standard operating procedure
SOX2	Sex determining region Y-box 2
SmPC	Summary of Product Characteristics
SPECT	single-photon emission computerized tomography
SRSD	Single reference safety document
SSE	symptomatic skeletal event
SSID	single subject identifier

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Abbreviation	Term
SUSAR	suspected unexpected serious adverse reaction
TBD	to be determined
T bili	total bilirubin
TEAE	treatment emergent adverse event
TGI	tumor growth inhibition
T-LBL	T-cell lymphoblastic lymphoma
TP53	tumor protein 53
txt	treatment
ULN	upper limit of normal
US	United States
UVB	ultraviolet B
W	Week
W1	week 1
W1 D1	week 1 day 1
W3	week 3
W5	week 5
W7	week 7
W9	week 9
WBC	white blood cell
WKS	Weeks
WT	wild type

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